

PassReady Prep — NCE Question Bank · Batch 6

Generated: 2026-10-04 · Showing: In review · Total items: 200

How to review

✓ **KEY** marks the correct option. Every option shows why it is right or wrong; check both the key and each distractor.

Mark each item **Approve**, **Revise** (note what to change) or **Retire**. Approved items are published from Admin → NCE Questions.

🔍 **Check** notes mark specific facts or citations the writer was less than certain of — verify those first.

Check: one clearly best answer · factual accuracy (theorists, dates, test properties, code sections) · distractors plausible but defensibly wrong · no answer given away by length or wording · references support the key.

NCE domains (NBCC 2023 content outline)

Professional Practice and Ethics 19 · Intake, Assessment, and Diagnosis 19 · Areas of Clinical Focus 47 · Treatment Planning 14 · Counseling Skills and Interventions 48 · Core Counseling Attributes 13 — of 160 scored items

In this copy

Professional Practice and Ethics **24** · Intake, Assessment, and Diagnosis **24** · Areas of Clinical Focus **59** · Treatment Planning **17** · Counseling Skills and Interventions **60** · Core Counseling Attributes **16**

Professional Practice and Ethics

24 items · NCE weight 12% (19 of 160 scored items)

Q1 of 200 · nce-s-eth-121 · Professional Counseling Orientation and Ethical Practice · ACA Code of Ethics structure · easy · In review

A former client has filed an ethics complaint against a counselor with the ACA Ethics Committee. The counselor wants to read the standards on cooperating with the committee and responding to complaints. Which section of the 2014 ACA Code of Ethics should she consult?

A. Section C, Professional Responsibility

Section C covers competence, advertising, public responsibility and impairment, not the complaint process.

B. Section A, The Counseling Relationship

Section A covers client welfare, consent, boundaries and termination within the counseling relationship.

C. Section I, Resolving Ethical Issues ✓ KEY

Section I addresses ethics complaints, conflicts with law or employers, and cooperation with ethics committees.

D. Section B, Confidentiality and Privacy

Section B covers privacy, exceptions to confidentiality and records, not responding to complaints.

Rationale: The 2014 ACA Code of Ethics has nine sections, A through I. Section I, Resolving Ethical Issues, is the last; it covers knowledge of standards, suspected violations by colleagues, conflicts with law or organizations, and the duty to cooperate with ethics committees. Counselors who receive a complaint are expected to take part in the process rather than resign membership to avoid it.

References: ACA Code of Ethics — Section I: Resolving Ethical Issues (I.3 Cooperation With Ethics Committees)

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor researcher learns that a graduate assistant filled in blank items on 30 returned surveys with the answers participants "probably would have given," so the study would reach its target sample. The data are about to be analyzed. Under Section G of the ACA Code of Ethics, what should the researcher do?

A. Remove or flag the filled-in answers and analyze only the data that participants gave ✓ **KEY**

G.4.a bars distorting or misrepresenting data; staff guesses are fabricated data and cannot be analyzed as real.

B. Keep the filled-in answers but note in the report that staff estimated some of the items

Disclosing the guesses does not make invented responses legitimate data; they still distort the results.

C. Keep the answers, since staff guesses work much like statistical imputation of missing data

Imputation is a transparent statistical method; an assistant writing in guessed answers is fabrication.

D. Let the assistant decide, since the lead researcher is not responsible for staff conduct

The principal researcher is responsible for the ethical conduct of the study, including that of research staff.

Rationale: ACA G.4.a requires counselors to plan, conduct and report research accurately and bars fraudulent research, distorted data and misrepresented results. Answers invented by staff are fabricated data, however well meant. The researcher removes them, handles missing data with an accepted and disclosed method, and addresses the assistant's conduct as the person responsible for the study.

References: ACA Code of Ethics — G.4.a Accurate Results · ACA Code of Ethics — G.1.f Principal Researcher Responsibility

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor educator wants to turn a former master's student's thesis into a journal article. The educator chaired the thesis and plans to rewrite much of the text for publication. Under the ACA Code of Ethics, how should authorship be handled?

A. The educator may be lead author, since rewriting the text is the larger share of the work

The Code gives lead authorship to the student when a work is substantially based on the student's thesis.

B. The student need only be thanked in a footnote, since the thesis has already been graded and filed

A footnote does not credit the person whose thesis the article is substantially based on.

C. The educator may publish alone, as a thesis chair holds joint rights to the student's work

Chairing a thesis gives no right to publish it alone; doing so takes credit for the student's work.

D. It may be published only with the student's permission and with the student as lead author ✓ **KEY**

G.5.f requires the student's permission and lists the student as lead author for work based on a thesis.

Rationale: Under ACA G.5.f, manuscripts or presentations substantially based on a student's course papers, projects, dissertation or thesis are used only with the student's permission and list the student as lead author. G.5.d adds that credit goes to those who made significant contributions, in proportion to those contributions. Faculty power over students makes this protection important.

References: ACA Code of Ethics — G.5.f Student Research · ACA Code of Ethics — G.5.d Contributors

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who holds the National Certified Counselor (NCC) credential and a state license describes herself on her practice website as a "Nationally Licensed Counselor." What is the problem with this description?

A. None, since a national certification and a state license together amount to the same thing

Certification and licensure are different; combining them does not create a national license.

B. It misstates her credential, because NCC is a national certification and not a license ✓ KEY

NCC is a voluntary national certification; licenses are issued by states, so the title misleads the public.

C. It is acceptable only if she also lists the year she first passed the licensing exam

Adding an exam date does not correct a title that names a credential that does not exist.

D. It is acceptable as long as she practices only in the state that issued her license

Limiting practice to one state does not make "nationally licensed" accurate.

Rationale: Counselor licenses are granted by states; there is no national license. The NCC is a voluntary national certification from NBCC. Both the NBCC Code and ACA C.4.a require counselors to represent their credentials accurately, so a counselor lists "NCC" and her state license separately and correctly.

References: NBCC Code of Ethics — Accurate representation of NBCC credentials and qualifications · ACA Code of Ethics — C.4.a Accurate Representation

☐ Approve ☐ Revise ☐ Retire Notes:

While applying for the NCC credential, a counselor inflates the number of supervised postgraduate hours on her application. She reasons that she is not yet an NCC, so the NBCC Code of Ethics does not apply to her. Which statement is accurate?

A. She is right, since the Code binds only counselors who already hold an NBCC credential

The NBCC Code applies to applicants as well as to certified counselors.

B. Only her state licensing board can act, since NBCC has no role in reviewing applications

NBCC reviews its own applications and can act on false information in them.

C. The inflated hours matter only if NBCC chooses to audit her application within the first year

Falsifying an application is a violation whether or not it is caught in an audit.

D. The Code also applies to applicants, so misstating hours can lead to denial or sanctions ✓ KEY

The NBCC Code covers applicants, and dishonesty in the certification process can bring denial or sanctions.

Rationale: The NBCC Code of Ethics applies both to National Certified Counselors and to people applying for NBCC certification. Providing false information about education, supervision or experience undermines the integrity of the credential and can lead NBCC to deny certification or impose sanctions. Honesty with credentialing bodies is part of professional integrity from the start of a career.

References: NBCC Code of Ethics — Application to NCCs and applicants; honesty in the certification process

☐ Approve ☐ Revise ☐ Retire Notes:

A National Certified Counselor who works mainly in substance use treatment wants to add NBCC's specialty certification in addictions counseling to her credentials. Which credential should she pursue?

A. Master Addictions Counselor (MAC) ✓ KEY

The MAC is NBCC's specialty certification in addictions counseling for NCCs.

B. Certified Rehabilitation Counselor

The CRC is a rehabilitation counseling credential from CRCC, not an NBCC addictions specialty.

C. National Certified School Counselor

The NCSC is an NBCC specialty, but it is for school counseling, not addictions.

D. Approved Clinical Supervisor (ACS)

The ACS is a supervision credential, not an addictions specialty.

Rationale: NBCC offers the National Certified Counselor (NCC) as its general credential and specialty certifications built on it, including the Certified Clinical Mental Health Counselor (CCMHC), the National Certified School Counselor (NCSC) and the Master Addictions Counselor (MAC). Specialty credentials require the NCC plus added training and experience in the specialty. Certification is voluntary and separate from state licensure.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: NBCC national certification and specialty certifications

☐ Approve ☐ Revise ☐ Retire Notes:

Q7 of 200 · nce-s-eth-127 · Professional Counseling Orientation and Ethical Practice · counselor impairment and self-care · easy · In review

A community agency counselor carrying 70 cases says she feels drained, has started calling clients "the 2 o'clock no-show" and "the difficult one," and doubts she helps anyone. In Maslach's model of burnout, which dimension do her labels for clients MOST reflect?

A. Emotional exhaustion

Exhaustion is the drained feeling she reports, not the detached labeling of clients.

B. Reduced personal accomplishment

This dimension fits her doubt that she helps anyone, not the labels.

C. Depersonalization ✓ KEY

Depersonalization is a detached, impersonal or cynical stance toward the people one serves.

D. Vicarious traumatization

Vicarious trauma is a shift in core beliefs from trauma exposure, not a burnout dimension.

Rationale: Maslach described burnout as having three dimensions: emotional exhaustion, depersonalization (a detached or cynical response to clients) and reduced personal accomplishment. This counselor shows all three, and her impersonal labels for clients are the clearest sign of depersonalization. Burnout is a warning sign for impairment, which ACA C.2.g asks counselors to monitor in themselves.

References: Gladding (Counseling: A Comprehensive Profession) — Counselor burnout: emotional exhaustion, depersonalization and reduced accomplishment · ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in long-term recovery from alcohol use disorder drank heavily over the weekend after a family loss. On Monday morning she feels hungover and shaky and has seven clients scheduled. Which action BEST follows the ACA Code of Ethics?

A. See the clients as planned, since one weekend of drinking does not make her impaired

She is shaky and hungover now; her current state, not the length of the lapse, puts clients at risk.

B. Cancel or arrange coverage for the day, then seek help before resuming her caseload ✓ KEY

C.2.g directs counselors to stop offering services when impaired and to seek assistance first.

C. See the clients but tell each one about the relapse so they can decide whether to stay

Disclosure does not make impaired services safe and shifts the counselor's burden onto clients.

D. Keep the schedule and talk about the relapse with her own counselor at the next session

Seeking help is right, but it does not justify seeing clients today while impaired.

Rationale: ACA C.2.g asks counselors to monitor themselves for signs of impairment and to refrain from offering services when their physical, mental or emotional state is likely to harm clients. They seek help for problems that reach the level of impairment and, if needed, limit, suspend or end services until it is safe to resume. Arranging coverage protects clients and continuity of care.

References: ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor going through a painful divorce notices that she quickly sides with clients against their spouses and feels irritated when clients consider reconciling. Her supervisor wants to help her manage these reactions. Which approach BEST matches Gelso and Hayes's work on managing countertransference?

A. Avoid any client in marital conflict until her divorce is final and the feelings have faded

Avoidance may be needed briefly if reactions are severe, but it does not build the capacity to manage them.

B. Build self-insight and self-integration, such as through personal therapy and supervision ✓ KEY

Self-insight and self-integration are core countertransference management factors in Gelso and Hayes's model.

C. Disclose her divorce to these clients so they understand the source of her strong reactions

This disclosure serves the counselor's needs and shifts attention away from the client.

D. Suppress her reactions in session and focus only on technique until the feelings subside

Suppression without understanding tends to let countertransference shape the work unnoticed.

Rationale: Gelso and Hayes describe five factors in managing countertransference: self-insight, self-integration, anxiety management, empathy and conceptualizing skills. Personal therapy and supervision help counselors understand where their reactions come from and keep them from acting them out with clients. Unmanaged reactions tied to a counselor's own life stress can become impairment, which C.2.g asks counselors to monitor.

References: Gelso & Hayes (Countertransference) — Countertransference management factors: self-insight, self-integration, anxiety management, empathy, conceptualizing skills · ACA Code of Ethics — C.2.g Impairment

☐ Approve ☐ Revise ☐ Retire Notes:

A 26-year-old client who has been in counseling for four months sends the counselor a friend request on the counselor's personal social media account, saying it would help her feel "more connected." Under the ACA Code of Ethics, what should the counselor do?

A. Accept the request but limit the client's view of personal posts through privacy settings

A limited personal connection is still a personal virtual relationship with a current client.

B. Accept the request, since social media contact is a minor and helpful boundary crossing

The Code does not treat this as an acceptable crossing; it prohibits it with current clients.

C. Ignore the request and say nothing, so the client is not embarrassed by being turned down

Silence leaves the client confused and misses a chance to discuss the boundary and her need.

D. Decline and discuss in session why personal online ties with clients are not permitted ✓ KEY

A.5.e prohibits personal virtual relationships with current clients; talking it through supports the work.

Rationale: ACA A.5.e prohibits counselors from having personal virtual relationships, such as through social media, with current clients. The counselor declines the request and explores in session what the request means to the client, which can be useful clinical material. The counselor's social media policy, reviewed during informed consent, should explain this in advance.

References: ACA Code of Ethics — A.5.e Personal Virtual Relationships With Current Clients · ACA Code of Ethics — H.6.a Virtual Professional Presence

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor has seen a 38-year-old woman individually for a year. She now asks the counselor to start seeing her and her husband together for couples counseling, and the counselor is willing. Under the ACA Code of Ethics, what must happen before the change?

A. Explain the change and its effects, get informed consent, and note the right to refuse it ✓ KEY

A.6.d requires informed consent for a role change, including the right to refuse it and its consequences.

B. Begin joint sessions, since she already consented to treatment with this same counselor

Consent to individual counseling does not cover a change to couples counseling.

C. End individual work first, since a counselor may not shift from individual to couples work

The Code permits this role change when it is handled through informed consent.

D. Have the husband sign a release, since only his consent is needed for the new format

The existing client's informed consent to the change is central; a release for the husband is not enough.

Rationale: ACA A.6.d treats a shift from individual to relationship or family counseling as a role change. The counselor obtains informed consent and explains the client's right to refuse services related to the change, along with any effects, such as how confidentiality and the alliance may shift with a second client. The husband also gives his own informed consent as a new client.

References: ACA Code of Ethics — A.6.d Role Changes in the Professional Relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A client taking an antidepressant prescribed by her primary care physician tells her counselor she feels better and plans to stop the medication this week. She asks whether stopping on her own is safe. What is the counselor's BEST response?

A. Advise her to taper the dose slowly over two weeks to reduce the risk of side effects

Giving dosing instructions is medical advice outside a counselor's scope of practice.

B. Tell her that stopping is fine, since her improved mood shows that she no longer needs the drug

This is a medical judgment the counselor is not trained or licensed to make.

C. Urge her to talk with her prescriber first, and offer to coordinate care with her consent ✓ KEY

This respects the counselor's scope while supporting the client and her care team.

D. Tell her to keep taking the full dose, since stopping would undo the progress so far

Telling her to continue is also a medication decision that belongs to the prescriber.

Rationale: Counselors practice within the boundaries of their education, training and licensure, which do not include prescribing or advising on medication changes. The counselor can share general concerns, encourage the client to consult her prescriber before stopping, and, with consent, coordinate with the physician. Exploring her reasons for wanting to stop remains within the counselor's role.

References: ACA Code of Ethics — C.2.a Boundaries of Competence · ACA Code of Ethics — D.1.c Interdisciplinary Teamwork

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor whose group training and experience are in psychoeducational groups is asked by her agency to lead a long-term psychotherapy group for adults with complex trauma histories. What is her BEST course of action?

A. Get specialized training and supervised experience in that type of group before leading it ✓ KEY

Competence is specific to group type; she builds it through training and supervision first.

B. Lead the group, since basic group leadership skills carry over evenly across all types of groups

ASGW distinguishes group types because each calls for different specialized skills.

C. Lead the group as a psychoeducational class so that it stays within her current training

Changing the format quietly does not meet the needs the group was formed to serve.

D. Accept the role, since an agency assignment makes the group part of her scope of practice

An employer's assignment does not create competence or expand professional boundaries.

Rationale: ASGW identifies distinct group specializations, including task, psychoeducational, counseling and psychotherapy groups, each calling for its own training. ACA C.2.a and C.2.b limit practice to areas of competence and require education, training and supervised experience before practicing in a new specialty area. Long-term trauma psychotherapy groups demand advanced skills in managing intense affect and member safety.

References: ASGW Best Practice Guidelines — Group leader competence and training for the type of group led · ACA Code of Ethics — C.2.b New Specialty Areas of Practice

☐ Approve ☐ Revise ☐ Retire Notes:

A licensure supervisor meets weekly with a supervisee who describes her sessions in detail. The supervisor has never seen or heard any of the supervisee's work. Two clients have recently dropped out without notice. What change would BEST meet the supervisor's obligation to clients?

A. Ask the supervisee to write longer summaries of each session before their weekly supervision meetings

Longer summaries are still self-report and may miss what the supervisee does not notice.

B. Contact the two clients directly to ask why they ended counseling without giving notice

This bypasses the supervisee and does not fix how the supervisor monitors ongoing care.

C. Continue as before, since a supervisee's self-report is the accepted basis for supervision

Self-report alone limits the supervisor's ability to monitor the services clients receive.

D. Add direct review of the work, such as recordings or live observation, with client consent ✓ KEY

Direct review lets the supervisor monitor client welfare and supervisee performance, as F.1.a requires.

Rationale: Under ACA F.1.a, a supervisor's primary obligation is to monitor the services supervisees provide to clients. Self-report misses what the supervisee does not see or chooses not to share, so direct methods such as recordings or live observation give a fuller picture. Clients consent to recording or observation in advance, as B.6.c and B.6.d require.

References: ACA Code of Ethics — F.1.a Client Welfare · ACA Code of Ethics — B.6.c Permission to Record

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in private practice is hired by a school district to consult with teachers on building trauma-informed classrooms. Before the consulting work begins, which step does the ACA Code of Ethics call for?

A. Gather confidential details on the individual students the teachers are most worried about

Student details are not needed for this consultation and raise privacy concerns.

B. Review, in writing and verbally, the rights and responsibilities of her and the consultees ✓ KEY

D.2.b calls for written and verbal review of both parties' rights and responsibilities in formal consultation.

C. Offer individual counseling to any of the teachers who seem stressed during the consultation

Adding a counseling role to a consulting role creates a multiple relationship.

D. Agree to report each teacher's performance to the principal as part of her contract work

This makes the consultant an evaluator and should not be agreed to without clear consent from consultees.

Rationale: ACA D.2.b requires that, when providing formal consultation, counselors review in writing and verbally the rights and responsibilities of both counselor and consultees, using clear language. This clarifies goals, roles, confidentiality and limits before work begins. D.2.a adds that consultants must be competent in the area of consultation and refer when needed.

References: ACA Code of Ethics — D.2.b Informed Consent in Formal Consultation · ACA Code of Ethics — D.2.a Consultant Competency

☐ Approve ☐ Revise ☐ Retire Notes:

Through the 20/20: A Vision for the Future of Counseling initiative, a large group of counseling organizations agreed on a shared definition of counseling to support a unified professional identity. Which statement captures that definition?

- A. Counseling is the treatment of diagnosed mental disorders through talk-based psychotherapy methods alone
The definition is broader than disorder treatment and includes wellness, education and career goals.
- B. Counseling is a brief advice-giving service that helps people without disorders solve everyday practical problems
The definition describes an empowering relationship, not advice-giving for healthy people only.
- C. A relationship that empowers diverse people to reach mental health, wellness, education and career goals ✓ KEY
This matches the consensus 20/20 definition of counseling.
- D. Counseling is the use of psychological tests to classify people and guide them into suitable careers
This resembles early vocational guidance, not the modern consensus definition.

Rationale: The 20/20 initiative, sponsored by ACA and the American Association of State Counseling Boards, produced a consensus definition: counseling is a professional relationship that empowers diverse individuals, families and groups to accomplish mental health, wellness, education and career goals. The definition emphasizes empowerment, diversity and a wellness focus rather than illness alone, which sets counseling apart from neighboring professions.

References: Gladding (Counseling: A Comprehensive Profession) — 20/20: A Vision for the Future of Counseling consensus definition of counseling

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor educator is teaching the history of counselor licensure in the United States. Which statement is accurate?

- A. California was the first state to license professional counselors, and Virginia was the last
The order is reversed: Virginia was first and California was last.
- B. Every state licensed counselors by 1990, soon after CACREP was founded in 1981
CACREP dates to 1981, but licensure did not reach all 50 states until 2009.
- C. Virginia licensed counselors first, in 1976, and California was the last state, in 2009 ✓ KEY
Virginia passed the first counselor licensure law in 1976; California completed the 50 states in 2009.
- D. Counselor licensure began at the federal level and was later handed over to the states
Counselor licensure has been a state matter from the start; there is no federal license.

Rationale: Counselor licensure in the United States is state-based. Virginia enacted the first general counselor licensure law in 1976, and California became the last of the 50 states to license professional counselors in 2009. Differences among state laws are one reason portability efforts such as the Counseling Compact have developed.

References: Gladding (Counseling: A Comprehensive Profession) — History of counselor licensure: Virginia (1976) to California (2009)

☐ Approve ☐ Revise ☐ Retire Notes: _____

A counselor sees that many clients struggle to use a county clinic that offers no materials in Spanish. A local parents' group is already organizing on the issue. The counselor offers the group her data skills and support while its members lead the effort. Which ACA Advocacy Competencies domain does this BEST illustrate?

A. Community collaboration ✓ KEY

She acts with a community group at the school/community level, supporting its own leadership.

B. Systems advocacy

Systems advocacy is the counselor acting on behalf of clients to change a system, not supporting a group's lead.

C. Client/student advocacy

This is acting on behalf of an individual client, not joining a community effort.

D. Social/political advocacy

This is acting in the public arena on legislation and policy, not supporting a local group.

Rationale: The ACA Advocacy Competencies distinguish acting with clients and communities from acting on their behalf, at the client, school/community and public arena levels. Community collaboration means working alongside groups that are addressing a shared concern, offering the counselor's skills while the community leads. Systems advocacy, by contrast, has the counselor taking the lead to change an institution.

References: ACA Advocacy Competencies — School/community level: community collaboration vs. systems advocacy

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who is a white, upper-middle-class man reflects on how his privilege and his client's marginalized status as a low-income Black woman shape power, trust and communication between them in session. In the Multicultural and Social Justice Counseling Competencies, which developmental domain does this MOST directly address?

A. Counselor self-awareness

Self-awareness centers on the counselor's own identity; the focus here is the interaction between them.

B. Client worldview

Client worldview centers on understanding the client's identities and experiences.

C. Counseling and advocacy interventions

This domain concerns intervening at individual through systemic levels, not the in-session dynamic.

D. The counseling relationship ✓ KEY

This domain addresses how privileged and marginalized statuses of both parties shape their relationship.

Rationale: The MSJCC (Ratts et al., 2016) arrange competencies in four developmental domains: counselor self-awareness, client worldview, the counseling relationship, and counseling and advocacy interventions. A quadrant model places counselor and client as privileged or marginalized. The counseling relationship domain asks how those combined statuses affect power, trust and communication in the work.

References: MSJCC (Ratts et al., 2016) — Developmental domains; privileged and marginalized quadrants; the counseling relationship

☐ Approve ☐ Revise ☐ Retire Notes:

A doctoral student preparing to teach and supervise counselors wants to join the ACA division devoted to counselor preparation and clinical supervision, which publishes the journal Counselor Education and Supervision. Which organization is it?

A. NBCC

NBCC is an independent certification body, not an ACA division.

B. ACES ✓ KEY

The Association for Counselor Education and Supervision focuses on counselor preparation and supervision.

C. AARC

The Association for Assessment and Research in Counseling focuses on assessment and research.

D. CACREP

CACREP accredits counseling programs; it is not a membership division.

Rationale: The Association for Counselor Education and Supervision (ACES) is the ACA division for counselor educators and supervisors and publishes Counselor Education and Supervision. It promotes quality in counselor preparation and supervision practice. CACREP accredits programs and NBCC certifies individuals; neither is an ACA division.

References: Gladding (Counseling: A Comprehensive Profession) — ACA divisions: Association for Counselor Education and Supervision

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor who works in vocational rehabilitation with adults who have disabilities wants the national credential specific to rehabilitation counseling. Which body grants that credential?

A. CRCC, which grants the Certified Rehabilitation Counselor (CRC) credential ✓ KEY

The Commission on Rehabilitation Counselor Certification grants the CRC.

B. CACREP, which accredits rehabilitation programs and certifies their graduates

CACREP accredits programs; it does not certify individual counselors.

C. ASCA, which certifies counselors who serve students with disabilities

ASCA is the school counselor association and does not grant a rehabilitation credential.

D. AASCB, which issues a rehabilitation counseling license valid in all states

AASCB is an association of state boards; it issues no licenses, and there is no national license.

Rationale: The Commission on Rehabilitation Counselor Certification (CRCC) grants the Certified Rehabilitation Counselor (CRC) credential and maintains its own code of ethics for CRCs. Certification is a voluntary national credential separate from state licensure. Accreditation bodies such as CACREP review programs, not individuals.

References: Gladding (Counseling: A Comprehensive Profession) — Credentialing: rehabilitation counseling and the CRC credential (CRCC)

☐ Approve ☐ Revise ☐ Retire Notes:

Early in counseling, a 45-year-old client discloses an affair and asks the counselor not to write "anything at all" about her sessions, because her spouse might someday see the record. What is the counselor's BEST response?

A. Agree to keep no record of her sessions, so that her request for privacy is honored

Counselors must keep the documentation needed to provide and continue services.

B. Keep a separate private file at home so that the official agency record holds no session content

A hidden file is less secure and misrepresents what the official record contains.

C. Explain that some records are required, and discuss what will be noted and who may see it ✓ KEY

This meets A.1.b while respecting her concern through openness about content and access.

D. Record every detail of the affair, since full notes protect the counselor in any dispute

Excess personal detail is not needed for care and adds risk to the client's privacy.

Rationale: ACA A.1.b requires counselors to create, safeguard and maintain the documentation needed to provide services, including sufficient and timely notes. The counselor explains this, describes how records are protected and who can access them, and documents what is clinically necessary without needless personal detail. Being open about records supports trust and informed consent.

References: ACA Code of Ethics — A.1.b Records and Documentation · Wiger (Documentation) — Clinically necessary content and minimum necessary detail in progress notes

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling intern's practicum requires her to audio-record sessions for review in supervision. A new client arrives for an intake. Under the ACA Code of Ethics, what must the intern do about the recording?

A. Record the session and tell the client afterward, since supervision benefits the client too

Permission must come before recording, not after the fact.

B. Record without asking, since the recordings are kept within the training program alone

Limited access does not remove the need for the client's permission.

C. Record only the intern's own voice, since that avoids any need for client permission

Session content is still captured, and the client still has a right to decide.

D. Get the client's permission before recording and explain how it will be used and stored ✓ KEY

B.6.c requires client permission before sessions are recorded.

Rationale: ACA B.6.c requires counselors to obtain clients' permission before recording sessions by electronic or other means, and F.1.c requires that clients know the counselor is in training and how supervision affects confidentiality. Informed consent covers who will hear the recording, how it is stored and when it is erased. Clients may decline.

References: ACA Code of Ethics — B.6.c Permission to Record · ACA Code of Ethics — F.1.c Informed Consent and Client Rights

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor providing career counseling by video wants a 34-year-old client to complete a free online "career personality quiz" she found on a popular website. The site gives no information on how the quiz was developed or tested. What is the BEST ethical course?

A. Use the quiz, since online tools are acceptable in distance counseling if the client agrees

Client agreement does not make an untested instrument valid for career decisions.

B. Choose an instrument with documented validity, reliability and norms that fit the client ✓ KEY

E.6.a requires weighing validity, reliability and appropriateness when selecting assessments.

C. Use the quiz but present its results to the client as a firm guide to her best careers

Presenting unknown-quality results as firm guidance misleads the client.

D. Use the quiz, since free career tools carry less risk than formal psychological testing

Career decisions carry real stakes; low cost does not lower the standard for evidence.

Rationale: ACA E.6.a asks counselors to consider validity, reliability, psychometric limits and appropriateness when selecting assessments, whether given in person or online. Many free web quizzes have no technical data, so their results cannot support career decisions. Well-developed interest and values inventories with documented norms, interpreted with the client, serve distance career counseling better.

References: ACA Code of Ethics — E.6.a Appropriateness of Instruments · Sharf (Career Development Theory) — Use of interest and values inventories in career counseling

☐ Approve ☐ Revise ☐ Retire Notes:

Intake, Assessment, and Diagnosis

24 items · NCE weight 12% (19 of 160 scored items)

Q25 of 200 · nce-s-ass-121 · Assessment and Testing · validity types · easy · In review

A state board is building a certification exam for school counselors. A panel of experienced practitioners checks every item against a table of specifications that lists the job tasks and how much weight each should carry. Which type of validity evidence does this process mainly provide?

A. Face validity

Face validity is how the test looks to test takers, not a systematic expert review against a blueprint.

B. Content validity ✓ KEY

Expert judgment of how well items sample a defined domain, guided by a table of specifications, is content validity evidence.

C. Predictive validity

Predictive validity needs a later criterion, such as job performance, correlated with scores.

D. Convergent validity

Convergent validity comes from correlations with other measures of the same construct.

Rationale: Content validity asks whether a test's items adequately and proportionally sample the domain it claims to cover. It is built in during development by writing items to a table of specifications (test blueprint) and having subject-matter experts judge the match. Unlike criterion-related validity, it rests on systematic expert judgment rather than a correlation coefficient.

References: Hays (Assessment) — Validity: content validity, test blueprints and expert review

☐ Approve ☐ Revise ☐ Retire Notes:

Q26 of 200 · nce-s-ass-122 · Research and Program Evaluation · validity types · medium · In review

A college counseling center finds that scores on an admissions test correlate .60 with students' first-year grade point averages. A staff member asks what this validity coefficient means. Which interpretation is accurate?

A. About 60% of students will have their first-year GPA predicted correctly by the test

A correlation is not a hit rate; it does not tell what percentage of individual predictions will be right.

B. About 64% of the differences in first-year GPA are explained by the test scores

Sixty-four percent is the variance NOT shared ($1 - .36$), so this reverses the meaning.

C. The test must have a reliability of .36, since validity is the square of reliability

Validity is limited by reliability, but reliability is not found by squaring a validity coefficient.

D. About 36% of the differences in first-year GPA are shared with the test scores ✓ KEY

Squaring r gives the coefficient of determination: $.60 \times .60 = .36$, or 36% shared variance.

Rationale: A validity coefficient is a correlation between test scores and a criterion. Squaring it gives the coefficient of determination, the proportion of criterion variance shared with (accounted for by) the test: .60 squared is .36, so 36% of the variation in GPA is associated with test scores and 64% is not. A coefficient of .60 is strong for a predictor, yet much of the criterion still depends on other factors.

References: Erford (Research and Evaluation in Counseling) — Correlation: coefficient of determination (r squared) as shared variance · Hays (Assessment) — Criterion-related validity: interpreting validity coefficients

☐ Approve ☐ Revise ☐ Retire Notes:

Researchers give the Career Decision Scale to undecided first-year students before and after a semester-long career planning course. Indecision scores drop for course participants but not for a similar group that did not take the course. The researchers cite this result in support of the scale. What type of evidence is it?

A. Construct validity, because scores change in the way the theory of indecision predicts ✓ KEY

Showing that scores respond to an intervention as the construct predicts is a recognized source of construct validity evidence.

B. Content validity, because the items cover the main reasons students are undecided

Content validity is judged from item coverage, not from score changes after an intervention.

C. Poor test-retest reliability, because scores for one group shifted over the semester

A change that appears only in the group that received the course reflects real change, not instability of the measure.

D. Concurrent validity, because scores are compared with another test given at the same time

No second criterion measure was given; the comparison is between groups over time.

Rationale: Construct validity is supported by many kinds of evidence, including experimental studies showing that scores change in theoretically expected ways. If a measure of career indecision drops after a career intervention but stays stable in a comparison group, that pattern fits what the construct predicts. The stable comparison group also argues against the change being measurement error.

References: Hays (Assessment) — Construct validity: evidence from intervention and group-difference studies · Sharf (Career Development Theory) — Career decision making and indecision: assessment of indecision

☐ Approve ☐ Revise ☐ Retire Notes:

A test developer compares bilingual and monolingual examinees who earned the same total scores on a reading test. On one item, an idiom-based question, bilingual examinees at every total-score level answer correctly far less often than monolingual examinees. What does this finding MOST likely indicate?

A. The bilingual examinees have lower reading ability than the monolingual examinees

The groups were matched on total score, so a general ability difference does not explain the gap on this item.

B. The item discriminates well and should be kept to separate strong and weak readers

Discrimination concerns high versus low scorers overall, not differences between matched groups.

C. The item shows differential functioning and may be biased against bilingual examinees ✓ KEY

When equally able groups perform differently on an item, the item functions differently and should be reviewed for bias.

D. The overall test is invalid for every group, since one of its items has a problem

One flagged item calls for review or removal of that item, not rejection of the whole test.

Rationale: Differential item functioning (DIF) analysis compares groups who are matched on the overall trait the test measures. If matched groups still differ in their odds of answering an item correctly, something other than the target ability, here familiarity with an idiom, is influencing the item. Flagged items are reviewed and revised or dropped, which is a key step in making a test culturally fair.

References: Hays (Assessment) — Test bias: item bias and differential item functioning

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor chooses a nonverbal matrix-reasoning test to estimate the problem-solving ability of a 40-year-old client who recently arrived from Eritrea and speaks little English. A colleague says the test is "culture-free," so cultural factors no longer need to be considered. How should the counselor respond?

A. Nonverbal tests lower language demands but still reflect cultural and schooling experience ✓ KEY

Culture-fair tests reduce, but do not remove, the influence of familiarity with testing, abstract figures, and timed tasks.

B. Nonverbal tests are fully culture-free, so the score can be read like any other client's

No test is culture-free; experience with test formats and schooling still affects performance.

C. Nonverbal tests are unsuitable for adults, so a verbal test given in English is better here

An English verbal test would mainly measure English proficiency for this client.

D. Nonverbal tests are valid only if the client was born in the country where norms were set

Norm fit matters, but birthplace alone does not decide validity, and the test can still be used with care.

Rationale: Culture-fair or culture-reduced tests, such as nonverbal matrix tasks, lessen reliance on language and specific cultural knowledge. They are not culture-free: familiarity with paper-and-pencil formats, abstract designs, formal schooling, and working against time all affect scores. Counselors still consider the client's background and the norm group when interpreting results.

References: Hays (Assessment) — Culturally fair assessment: culture-reduced (culture-fair) versus culture-free tests

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor plans to give a standardized personality inventory to a 33-year-old client who moved from Mexico a year ago, speaks mostly Spanish at home, and is comfortable with conversational English at work. What should the counselor do BEFORE selecting the form and norms to use?

A. Give the English form, since conversational English at work shows enough proficiency

Workplace conversation does not show the reading level and fluency needed for a long inventory.

B. Skip standardized testing, since no inventory can be fairly used with recent immigrants

Many inventories have translated forms and research with diverse groups; careful selection is the answer.

C. Ask a family member to read each item aloud in Spanish while the client gives answers

Informal oral translation by a relative changes standard conditions and may affect privacy and honesty.

D. Assess the client's acculturation and language dominance to guide the form and norms used ✓ KEY

Knowing acculturation and preferred language helps the counselor pick an appropriate version and interpret scores in context.

Rationale: Acculturation and language dominance affect how clients understand items and how well a norm group represents them. Assessing them first, informally or with an acculturation measure, helps the counselor decide whether a translated and validated form is needed and how cautiously to interpret results. The ACA Code asks counselors to consider culture when selecting and interpreting tests.

References: ACA Code of Ethics — Section E: Evaluation, Assessment, and Interpretation — multicultural issues and diversity in assessment · Sue & Sue (Counseling the Culturally Diverse) — Acculturation and culturally appropriate assessment

☐ Approve ☐ Revise ☐ Retire Notes:

With a signed release, a counselor receives hospital records from four years ago for a 36-year-old client. The discharge summary lists borderline personality disorder, but the client says that diagnosis "never fit" and was made during a single crisis admission. How should the counselor use this record?

A. Adopt the prior diagnosis so the client's records stay consistent across providers

Copying a prior diagnosis without assessment can carry forward an error.

B. Treat it as one source of data and test it against a current, independent assessment ✓ KEY

Records inform the picture, but the counselor reaches a diagnosis from their own current evaluation.

C. Set the record aside, since the client disagrees with it and it is four years old

Older records can still hold useful history about symptoms, risk, and prior treatment.

D. Ask the hospital to remove the diagnosis before the counselor completes an assessment

The counselor has no basis to challenge the record before doing their own evaluation.

Rationale: Prior records are valuable collateral data, offering history of symptoms, risk, and treatment response. They are also snapshots taken in a particular context, and a diagnosis made during one crisis admission may not hold over time. Counselors weigh records alongside the interview and other sources rather than adopting or dismissing a prior diagnosis automatically.

References: Hays (Assessment) — Clinical assessment: integrating records and collateral sources with interview data

☐ Approve ☐ Revise ☐ Retire Notes:

Q32 of 200 · nce-s-ass-128 · Professional Counseling Orientation and Ethical Practice · collateral information and records · easy · In review

During intake for depression, a 25-year-old client declines to sign a release that would let the counselor speak with her mother. She is not at imminent risk and is otherwise engaged in the assessment. What should the counselor do?

A. Call the mother anyway, since family input is needed for a complete diagnostic picture

Contacting a third party without consent breaches confidentiality when no exception applies.

B. Pause the assessment until the client agrees to sign the release for her mother

Making services depend on a release pressures the client and is not needed for assessment.

C. Respect her decision, explore her concerns, and continue the assessment without it ✓ KEY

An adult client controls the release of her information; the counselor can still assess well from other data.

D. Contact the mother only for history, since this does not reveal what was said in session

Even confirming that a person is a client discloses confidential information.

Rationale: Collateral contacts with family or other providers require the client's informed, written consent unless a recognized exception to confidentiality applies. When an adult client declines, the counselor respects that choice, may gently explore the reasons, documents the decision, and proceeds with the information available. The client can revisit the release later.

References: ACA Code of Ethics — Section B: Confidentiality and Privacy — respecting client rights; consent to disclose information

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is assessing a 10-year-old for a court-ordered custody evaluation. The father, who is seeking sole custody, describes the child as severely aggressive at home. The mother and the child's teacher describe only mild, occasional outbursts. How should the counselor weigh these reports?

- A. Rely most on the father's account, since a parent describing more problems is more cautious
Severity of a report does not make it more accurate, especially when the informant has a stake in the outcome.
- B. Rely on the teacher's report alone, since school staff have no reason to be inaccurate
Teachers see one setting; a single source is not enough even when it seems neutral.
- C. Average the three reports to get one score that reflects the child's typical behavior
Averaging hides real differences across settings and ignores informant motivation.
- D. Weigh each informant's setting and motives, and gather more data before concluding ✓ KEY
Informant stake and setting both affect reports, so the counselor integrates sources and seeks corroboration.

Rationale: Collateral reports are shaped by the informant's vantage point and by what they stand to gain or lose. In an evaluation tied to custody, a parent's report may be influenced by the dispute, so the counselor weighs it alongside other informants, records, and direct observation. Discrepancies are treated as data to explain, not averaged away.

References: Hays (Assessment) — Collateral sources: informant bias and integrating multiple sources of information

☐ Approve ☐ Revise ☐ Retire Notes:

On the Strong Interest Inventory, a 21-year-old client's Investigative General Occupational Theme has a standard score of 62. She asks whether this means she "got 62 percent." Which explanation is accurate?

- A. It means she marked "Like" on about 62% of the items in the Investigative theme
The theme score is a converted standard score, not the share of items she endorsed.
- B. It means her interests fall at the 62nd percentile compared with other college students
A standard score of 62 on this scale is well above the 62nd percentile.
- C. It is a standard score, a bit over one SD above the mean of a broad sample of adults ✓ KEY
Theme scores have a mean of 50 and SD of 10 in a general reference sample, so 62 is 1.2 SD above the mean.
- D. It means she has about a 62% chance of succeeding in an investigative type of career
Interest scores describe likes and dislikes, not chances of success.

Rationale: The Strong Interest Inventory reports General Occupational Theme scores as standard scores with a mean of 50 and a standard deviation of 10, based on a general reference sample of employed adults. Interpretation is norm-referenced: a score of 62 shows stronger Investigative interests than most people in that sample. It is not a percentage correct, a percentile, or a prediction of success.

References: Hays (Assessment) — Strong Interest Inventory: General Occupational Themes reported as standard scores (mean 50, SD 10)

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old's Strong Interest Inventory profile shows a high score on the Physician occupational scale. He asks, "So this proves I can make it through medical school?" Which response is BEST?

A. Interest scores show similar likes, not ability; grades and test scores speak to ability ✓ **KEY**

Interest inventories compare likes and dislikes with satisfied workers; ability evidence comes from achievement and aptitude data.

B. A high occupational score is a strong sign that he has the ability to finish the training

Interest scores do not measure aptitude or achievement, so they cannot predict academic success.

C. He should take the inventory again, since a single result cannot be trusted for planning

Retesting does not change what the inventory measures; the issue is interest versus ability.

D. A high score means he should drop other options and focus only on applying to medicine

One scale should open exploration, not close off other options.

Rationale: Interest inventories such as the Strong describe how closely a person's likes and dislikes match those of people satisfied in an occupation. They do not measure intelligence, aptitude, or achievement. Career counselors combine interest results with ability evidence such as grades, achievement tests, and admissions test scores before judging the fit of a demanding path.

References: Hays (Assessment) — Career assessment: interest inventories versus aptitude and achievement measures

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-item self-efficacy scale has a reliability coefficient of .50. The developer adds 20 new items of the same kind and quality, for 30 items in all. According to the Spearman-Brown formula, what is the estimated reliability of the longer scale?

A. .67

This is the result for doubling the test ($2 \times .50 / 1.50$), not tripling it.

B. .75 ✓ **KEY**

With $k = 3$: $(3 \times .50) / (1 + 2 \times .50) = 1.50 / 2.00 = .75$.

C. .90

Reliability rises with length but with diminishing returns; .90 overstates the gain.

D. 1.50

Simply multiplying by 3 gives an impossible value; reliability cannot exceed 1.0.

Rationale: The Spearman-Brown formula estimates reliability when test length changes by a factor of k : $r_{\text{new}} = k r / (1 + (k - 1) r)$. Tripling a scale with reliability .50 gives $1.50 / 2.00 = .75$. Adding comparable items raises reliability, but each added block helps less than the one before.

References: Erford (Research and Evaluation in Counseling) — Reliability: Spearman-Brown prophecy formula and test length

☐ Approve ☐ Revise ☐ Retire Notes:

A seventh grader answers 42 of 60 items correctly on a reading test. The score report lists his percentile rank as 85. His parent asks what the two numbers mean. Which statement is accurate?

A. He answered 85% of the items correctly, which placed him at the 70th percentile

This swaps the two numbers; 42 of 60 is 70%, and the percentile rank is 85.

B. He answered 70% of items correctly, so he scored lower than about 85% of his peers

A percentile rank of 85 means he scored higher than about 85% of the norm group, not lower.

C. He answered 85% of the items correctly, and about 70% of his peers did at least as well

The percentage correct is $42 / 60 = 70\%$, and percentile rank refers to students scoring below him.

D. He answered 70% of items correctly and scored higher than about 85% of the norm group ✓ **KEY**

$42 / 60 = .70$, a percentage correct; a percentile rank of 85 means he outscored about 85% of the norm group.

Rationale: Percentage correct ($42 / 60 = 70\%$) is an absolute, criterion-style figure about how much of the content a student mastered. A percentile rank is a norm-referenced figure: the percentage of the norm group scoring below the student. A difficult test can produce a modest percentage correct and a high percentile rank, as here.

References: Hays (Assessment) — Score interpretation: percentile ranks versus percentage correct

☐ Approve ☐ Revise ☐ Retire Notes:

Q38 of 200 · nce-s-ass-134 · Human Growth and Development · screening instruments (PHQ-9, GAD-7, AUDIT, CAGE) · medium · In review

At a pediatric clinic, a parent completes the M-CHAT-R/F, an autism screening checklist, for a 20-month-old. The result falls in the medium-risk range. The parent asks the counselor whether this means the child has autism. What is the BEST response?

A. Yes; a medium-risk result on this checklist is enough to make the autism diagnosis

A screening result flags possible risk; diagnosis requires a comprehensive evaluation.

B. No; screening this young is unreliable, so wait until the child is in school to retest

The tool is designed for toddlers, and waiting would delay early intervention if needed.

C. Not yet; follow-up questions come next, then a full evaluation if concerns remain ✓ **KEY**

A screen that flags concerns leads to follow-up and, if still positive, referral for diagnostic evaluation.

D. No; the result is a false positive, since most toddlers this age show these behaviors

A screen cannot be dismissed without follow-up; doing so could delay early identification.

Rationale: Developmental screeners such as the M-CHAT-R/F identify toddlers who may need further assessment; they do not diagnose. A result that raises concern leads to the structured follow-up questions and, if concern persists, referral for a comprehensive diagnostic evaluation. Early identification matters because early intervention can improve developmental outcomes.

References: Hays (Assessment) — Screening versus diagnostic assessment: positive screens lead to follow-up and comprehensive evaluation · Berk (Development Through the Lifespan) — Early childhood development: autism and the value of early identification

☐ Approve ☐ Revise ☐ Retire Notes:

With about ten minutes left in a first intake session, a counselor has gathered the presenting concern, history, and a risk screen for a 50-year-old client. What is the MOST useful way to close the intake?

A. Summarize the main concerns, check accuracy, and agree on next steps and questions ✓ KEY

A closing summary confirms shared understanding and sets a clear, collaborative plan.

B. Use the remaining time to begin a new topic, such as the client's early childhood

Opening new, deep material at the end leaves no time to process it.

C. Give the client the formal diagnosis and a full treatment plan to sign before leaving

Diagnosis and planning are shared and collaborative; presenting them as finished skips client input.

D. End on time with no summary, since the next session will review all of this material

Ending without a summary misses a chance to confirm understanding and build the alliance.

Rationale: The close of an intake ties the session together. The counselor summarizes key concerns and strengths, checks whether the summary fits the client's view, and agrees on next steps such as scheduling, referrals, or homework, inviting the client's questions. This structure supports the working alliance and makes the plan collaborative.

References: Ivey et al. (Intentional Interviewing) — Five-stage interview structure: summarizing and planning at the end of the session

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client speaks rapidly and is hard to interrupt. She says, "Your plant is green, green is for go, go to the races, horses need water, I should start a water company." How should the counselor record this in the mental status examination?

A. Circumstantiality

Circumstantial speech includes many details but eventually reaches the original point.

B. Flight of ideas ✓ KEY

Rapid jumps from idea to idea, linked by word play or chance associations, describe flight of ideas.

C. Thought blocking

Thought blocking is a sudden halt in the train of thought, not a rapid stream.

D. Perseveration

Perseveration is repeating the same idea or response, not jumping between ideas.

Rationale: Flight of ideas is a thought process finding: an accelerated flow of ideas with abrupt shifts, where connections are based on sounds, word play, or distracting stimuli. It often occurs with pressured speech and is common in manic episodes. Unlike loosened associations, the links between ideas can usually be followed.

References: Hays (Assessment) — Mental status examination: thought process (flight of ideas, circumstantiality, blocking) · DSM-5-TR — Manic episode, Criterion B: flight of ideas or subjective experience that thoughts are racing

☐ Approve ☐ Revise ☐ Retire Notes:

Using the Columbia Suicide Severity Rating Scale (C-SSRS), a counselor asks about recent behavior. A 44-year-old client says that last week he poured a bottle of pills into his hand, intending to die, then put them back on his own before swallowing any. No one else was present. How is this behavior classified?

A. An interrupted attempt, since the act was stopped before any pills were swallowed

An interrupted attempt is stopped by an outside person or circumstance, which did not occur here.

B. Non-suicidal self-injurious behavior, since no physical injury took place at all

He reported intent to die, so the behavior is suicidal, not non-suicidal self-injury.

C. Suicidal ideation only, since the client did not take any action that could harm him

Pouring out pills to take them is a behavior, not only a thought.

D. An aborted or self-interrupted attempt, since he stopped himself before taking any pills ✓ **KEY**

The C-SSRS defines an aborted or self-interrupted attempt as beginning steps toward an attempt and stopping oneself.

Rationale: The C-SSRS separates suicidal behaviors into actual attempts, interrupted attempts (stopped by an outside circumstance), aborted or self-interrupted attempts (the person stops themselves), and preparatory acts such as gathering means or writing a note. Each counts as suicidal behavior and raises concern. Precise classification guides risk formulation and safety planning.

References: C-SSRS — Suicidal behavior section: actual, interrupted, and aborted or self-interrupted attempts; preparatory acts

☐ Approve ☐ Revise ☐ Retire Notes:

A 37-year-old outpatient comes to an agency seeking treatment for alcohol use disorder, which is the focus of today's visit. During the intake, the counselor also confirms a long-standing generalized anxiety disorder. Under DSM-5-TR recording guidance, which diagnosis is generally listed first?

A. Generalized anxiety disorder, because the condition with the earliest onset is listed first

Order is not set by which disorder began first.

B. Whichever diagnosis has the lowest ICD-10-CM code, so that the record is in code order

Diagnoses are not ordered numerically by code.

C. Alcohol use disorder, as the reason for the visit is generally listed first in outpatient care ✓ **KEY**

In outpatient settings, the condition chiefly responsible for the visit is generally listed first.

D. Generalized anxiety disorder, because anxiety disorders come before substance use disorders

DSM-5-TR chapter order does not set the order of a client's diagnoses.

Rationale: When a person has more than one diagnosis, DSM-5-TR asks clinicians to list the principal diagnosis (inpatient) or the reason for visit (outpatient) first. The reason for visit is the condition chiefly responsible for the services received. Additional diagnoses are listed after it, and the other conditions still guide care.

References: DSM-5-TR — Use of the Manual: principal diagnosis and reason for visit

☐ Approve ☐ Revise ☐ Retire Notes:

For the past year, a 27-year-old accountant has had sudden surges of pounding heart, shaking, and breathlessness, but only when asked to present in meetings or to eat with coworkers. He fears they will see him as incompetent and now avoids team lunches. He has had no unexpected attacks. Which diagnosis BEST fits?

A. Social anxiety disorder ✓ KEY

Fear of negative evaluation in social and performance situations, with avoidance for 6 months or more, fits social anxiety disorder.

B. Panic disorder

Panic disorder requires recurrent unexpected attacks; his attacks occur only in feared social situations.

C. Agoraphobia

Agoraphobic fear centers on escape or help being hard to get in places such as crowds or transit.

D. Generalized anxiety disorder

GAD involves broad, hard-to-control worry across many areas, not fear focused on scrutiny.

Rationale: Panic attacks can occur within many disorders. When attacks happen only in social or performance situations and are driven by fear of being judged, the diagnosis is social anxiety disorder rather than panic disorder, which requires recurrent unexpected attacks. The panic attack specifier can be added to note the attacks.

References: DSM-5-TR — Social Anxiety Disorder: criteria and differential diagnosis (panic disorder); panic attack specifier

☐ Approve ☐ Revise ☐ Retire Notes:

An ability test has a standard deviation of 15. Its manual reports a reliability of .84 for 6- and 7-year-olds and .96 for adults. A 7-year-old earns a score of 100. Within which range does the child's true score fall with about 68% confidence?

A. 97 to 103

This uses the adult SEM of 3 ($15 \times .20$), which does not apply to a 7-year-old.

B. 94 to 106 ✓ KEY

$SEM = 15 \times \text{the square root of } (1 - .84) = 15 \times .40 = 6$; 100 plus or minus 6 gives 94 to 106.

C. 88 to 112

Plus or minus 12 is about a 95% band (2 SEMs), not 68%.

D. 84 to 116

This treats $1 - .84$ as 16 points instead of multiplying 15 by its square root.

Rationale: The standard error of measurement is $SD \times \text{the square root of } (1 - r)$. Because reliability often differs by age, the counselor uses the coefficient for the client's own group: $15 \times \text{the square root of } .16 = 15 \times .40 = 6$. One SEM on either side of the obtained score gives a 68% confidence band of 94 to 106.

References: Hays (Assessment) — Standard error of measurement: computation and confidence bands; reliability by age group

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a community mental health center is considering the MMPI-3 for a 45-year-old client with a complex presentation. Which description of the MMPI-3 is accurate?

A. A projective test in which the client describes what they see in ambiguous visual images

The MMPI-3 is an objective self-report inventory, not a projective test.

B. An inventory normed for adolescents aged 14 to 18 that cannot be used with adult clients

Adolescent versions are the MMPI-A and MMPI-A-RF; the MMPI-3 is for adults.

C. A measure of the five-factor model that yields scores for openness and agreeableness

The five-factor domains are measured by the NEO inventories.

D. A true/false self-report inventory for adults that includes scales checking response style ✓ **KEY**

The MMPI-3 is an adult true/false inventory with validity scales for non-responding, inconsistency, over- and underreporting.

Rationale: The MMPI-3 is the current adult version of the MMPI, a broad objective self-report inventory answered true or false. Its validity scales detect non-responding, inconsistent responding, and over- or underreporting before substantive scales are interpreted. Adolescents take the MMPI-A or MMPI-A-RF, and five-factor traits are assessed with instruments such as the NEO-PI-3.

References: Hays (Assessment) — Personality assessment: MMPI family of instruments and validity scales

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor considering a projective technique asks a supervisor what main advantage is usually claimed for projective tests compared with self-report inventories. What is the BEST answer?

A. They have higher reliability because scoring rules leave no room for judgment

Projective tests typically have lower reliability, partly because scoring involves judgment.

B. They are faster to give and score, making them well suited to group screening

Most projective techniques are individually given and time-consuming to score.

C. Their purpose is less obvious, so responses are harder to shape deliberately ✓ **KEY**

Ambiguous tasks make it harder for clients to tell what a "good" answer is, reducing deliberate faking.

D. They yield precise diagnoses that match DSM-5-TR criteria for most disorders

Projective results generate hypotheses; they do not map directly to diagnostic criteria.

Rationale: Projective techniques present ambiguous stimuli, based on the idea that people project their needs and conflicts into their responses. Because the purpose is less transparent, clients have a harder time presenting themselves in a planned way. The trade-offs are lower reliability, scoring that depends on examiner judgment, and weaker validity evidence for many uses.

References: Hays (Assessment) — Projective assessment: projective hypothesis, advantages and limitations

☐ Approve ☐ Revise ☐ Retire Notes:

An outpatient program wants a semi-structured interview that rates the severity of an adult client's problems across medical status, employment, alcohol use, drug use, legal status, family and social relationships, and psychiatric status. Which instrument fits this purpose?

A. Addiction Severity Index ✓ KEY

The ASI is a semi-structured interview rating problem severity across seven life areas.

B. CAGE questionnaire

The CAGE is a four-question alcohol screen, not a multi-area severity interview.

C. CRAFFT screening tool

The CRAFFT is a brief screen developed for adolescents.

D. AUDIT-C screening items

The AUDIT-C is three consumption questions about alcohol only.

Rationale: The Addiction Severity Index (ASI) is a widely used semi-structured interview for adults that assesses seven areas: medical, employment and support, alcohol, drug, legal, family and social, and psychiatric. Its multi-area profile helps with treatment planning and tracking change. Brief screens such as the CAGE, AUDIT-C, and CRAFFT identify possible problems but do not map severity across life areas.

References: Hays (Assessment) — Substance use assessment: Addiction Severity Index and brief screening instruments

☐ Approve ☐ Revise ☐ Retire Notes:

A 48-year-old client with no psychiatric history reports two weeks of little sleep, irritability, racing thoughts, and unusual energy. In the medical section of the biopsychosocial history, she mentions starting a high-dose course of prednisone for a severe asthma flare about three weeks ago. What should the counselor do?

A. Diagnose bipolar I disorder, since her symptoms meet the criteria for a manic episode

Symptoms caused by a medication do not count toward bipolar I disorder.

B. Record the timeline and coordinate with her prescriber, since the drug may be the cause ✓ KEY

Corticosteroids can induce manic-like symptoms; the timing calls for medical consultation.

C. Advise her to stop taking the prednisone right away, since it is causing her symptoms

Medication changes are a prescriber's decision; stopping corticosteroids abruptly can be unsafe.

D. Leave the medication out of the assessment, since it is unrelated to her mental health

Medication history is central to the biopsychosocial assessment and differential diagnosis.

Rationale: The medical part of the biopsychosocial history includes current medications and recent changes, because many medications affect mood and behavior. DSM-5-TR lists corticosteroids among medications that can induce bipolar-type symptoms, and an episode that begins after starting such a drug is diagnosed as substance/medication-induced unless evidence shows otherwise. The counselor documents the timing and coordinates with the prescriber rather than giving medical advice.

References: DSM-5-TR — Substance/Medication-Induced Bipolar and Related Disorder: medications such as corticosteroids

☐ Approve ☐ Revise ☐ Retire Notes:

Areas of Clinical Focus

59 items · NCE weight 29% (47 of 160 scored items)

Q49 of 200 · nce-s-clf-296 · Assessment and Testing · depressive disorders · medium · In review

A 39-year-old met full criteria for a major depressive episode last year. After treatment, she had no significant symptoms for 5 months. For the past 3 weeks she again meets full criteria for a major depressive episode. She has never had manic or hypomanic symptoms. How should the counselor classify her major depressive disorder?

A. Single episode, since a new episode is counted only after a full year of being well
DSM-5-TR does not require a full year; the separating interval is at least 2 consecutive months.

B. Single episode, since the current symptoms are a continuation of last year's episode
Five months without meeting criteria is long enough to count the current episode as a new one.

C. Recurrent, since at least 2 straight months without full criteria split the episodes ✓ **KEY**
Episodes are separate when at least 2 consecutive months pass in which criteria for a major depressive episode are not met.

D. Recurrent, but only after the current episode has lasted at least 6 months in total
The current episode needs only the usual 2-week minimum; there is no 6-month requirement for recurrence.

Rationale: In DSM-5-TR, a major depressive disorder is coded as recurrent when there have been at least two episodes. To count as separate, the episodes must be divided by an interval of at least 2 consecutive months in which criteria for a major depressive episode are not met. Recurrence matters clinically because the risk of further episodes rises with each one, which shapes plans for maintenance treatment.

References: DSM-5-TR — Major depressive disorder: recording procedures, single vs recurrent episode (2-month interval)

☐ Approve ☐ Revise ☐ Retire Notes:

Q50 of 200 · nce-s-clf-297 · Assessment and Testing · depressive disorders · easy · In review

A 63-year-old in a major depressive episode says that nothing lifts her mood, not even visits from her grandchildren. She wakes at 4 a.m. and cannot get back to sleep, feels worst in the mornings, has lost weight without trying, and feels guilty over small past mistakes. Which specifier BEST fits this episode?

A. With melancholic features ✓ **KEY**
Lack of mood reactivity plus early waking, morning worsening, weight loss and excess guilt fits melancholic features.

B. With atypical features
Atypical features require mood reactivity, the opposite of her mood that nothing brightens.

C. With seasonal pattern
Seasonal pattern needs a regular link between episodes and a time of year, which is not described.

D. With anxious distress
Anxious distress centers on tension, restlessness, worry and fear of losing control, not on these features.

Rationale: The melancholic features specifier requires either loss of pleasure in nearly all activities or a lack of reactivity to usually pleasurable events. At least three more features must be present, such as depression that is regularly worse in the morning, waking at least 2 hours early, marked psychomotor change, significant weight loss and excessive guilt. Melancholic presentations are often severe and are a signal to coordinate with a prescriber.

References: DSM-5-TR — Specifiers for depressive disorders: with melancholic features

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old has had two major depressive episodes. Between them, on three separate occasions, she had 3 days of elevated mood, a reduced need for sleep, rapid speech and taking on many new projects. Coworkers noticed the change, but she kept working well. She has never had a longer period of these symptoms. Which diagnosis BEST fits?

- A. Bipolar II disorder, since the hypomanic periods were clearly seen by others
A hypomanic episode requires at least 4 consecutive days; her periods lasted only 3 days.
- B. Cyclothymic disorder, since mood swung between highs and lows for some time
Cyclothymic disorder is ruled out once criteria for a major depressive episode have been met.
- C. Bipolar I disorder, since highs plus depressive episodes meet the main criteria
Bipolar I requires a manic episode of at least a week or one needing hospitalization; none occurred.

D. Other specified bipolar disorder, with short-duration hypomanic episodes ✓ KEY
DSM-5-TR lists 2 to 3 day hypomanic episodes plus major depressive episodes as an other specified presentation.

Rationale: A hypomanic episode in DSM-5-TR lasts at least 4 consecutive days, so 3-day periods do not support bipolar II disorder. When a person has had major depressive episodes along with hypomanic periods of 2 to 3 days, DSM-5-TR lists the presentation as other specified bipolar and related disorder. Recognizing these brief highs matters, because they may change treatment choices for depression.

References: DSM-5-TR — Other specified bipolar and related disorder: short-duration hypomanic episodes (2-3 days) and major depressive episodes

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is training college residence-hall staff to spot early signs of serious mood disorders. According to DSM-5-TR, at about what age does the first mood episode of bipolar I disorder begin, on average?

- A. Around age 8, in middle childhood
Bipolar I can begin in childhood, but onset this early is far below the average.
- B. Around age 18, in late adolescence ✓ KEY**
DSM-5-TR gives a mean age of about 18 for the first manic, hypomanic or depressive episode.
- C. Around age 45, in middle adulthood
First onset in midlife or later occurs but is much less common and calls for a medical workup.
- D. Around age 65, in late adulthood
Late-life first mania is uncommon and often points to a medical or neurological cause.

Rationale: DSM-5-TR reports that the mean age at onset of the first manic, hypomanic or major depressive episode in bipolar I disorder is about 18 years. This places the typical onset squarely in the college years, which is why campus staff benefit from knowing early warning signs. A first manic episode in late life should prompt a search for medical or neurological causes.

References: DSM-5-TR — Bipolar I disorder: development and course, mean age at onset about 18 years

☐ Approve ☐ Revise ☐ Retire Notes:

For 14 months, a 15-year-old has had many periods of high energy, little sleep and talkativeness, along with many periods of low mood and poor focus. None of these periods has met criteria for a hypomanic, manic or major depressive episode, and he has not gone more than 2 months without symptoms. What diagnostic conclusion is BEST supported?

A. No diagnosis yet, since the 2-year duration applies to clients of every age

The 2-year rule is for adults; DSM-5-TR shortens the duration to 1 year for children and adolescents.

B. Cyclothymic disorder, as 1 year suffices for children and adolescents ✓ KEY

In youth, cyclothymic disorder needs at least 1 year of these fluctuating subthreshold symptoms.

C. Bipolar II disorder, since his high-energy periods count as hypomanic ones

His highs never met full hypomanic criteria, and he has had no major depressive episode.

D. Disruptive mood dysregulation disorder, given the frequent mood swings

DMDD is defined by severe temper outbursts and persistent irritability, not alternating highs and lows.

Rationale: Cyclothymic disorder involves numerous periods of hypomanic and depressive symptoms that never meet full criteria for an episode. Adults need at least 2 years of these symptoms, but children and adolescents need only 1 year, and symptoms must not be absent for more than 2 months at a time. Youth with cyclothymic disorder warrant close follow-up because some later develop bipolar I or II disorder.

References: DSM-5-TR — Cyclothymic disorder: Criterion A, at least 1 year in children and adolescents

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old graduate student who grew up in Japan avoids classes and meetings. He is not afraid that others will judge him as weak. Instead, he fears that his gaze, blushing and body odor make other people uncomfortable and offend them. Which cultural concept of distress does this MOST closely match?

A. Ataque de nervios, an episode of loss of control after family stress

Ataque de nervios is a Latin American syndrome of acute outbursts, not a fear of offending others.

B. Shenjing shuairuo, a Chinese syndrome of mental and physical fatigue

Shenjing shuairuo centers on weakness, fatigue and poor focus, not on social offense.

C. Dhat syndrome, an anxiety about the loss of semen and its effects

Dhat syndrome is a South Asian concern about semen loss, unrelated to his social fears.

D. Taijin kyofusho, a fear that one's body or manner offends others ✓ KEY

Taijin kyofusho is a Japanese syndrome related to social anxiety, focused on offending or upsetting others.

Rationale: DSM-5-TR describes taijin kyofusho as a cultural syndrome marked by fear that one's appearance, gaze, facial expressions or body odor are offensive or embarrassing to others. It overlaps with social anxiety disorder, but the fear is of harming others rather than of being judged oneself. Knowing cultural concepts of distress helps counselors understand how clients frame symptoms and avoid misdiagnosis.

References: DSM-5-TR — Glossary of cultural concepts of distress: taijin kyofusho; social anxiety disorder, culture-related issues

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old Cambodian refugee describes sudden attacks of dizziness, a pounding heart, shortness of breath and ringing in her ears. She explains that "wind" is surging up in her body and may burst the vessels in her neck and kill her. Which counselor response is MOST appropriate?

A. Explore what the wind attacks mean to her while assessing for panic and trauma ✓ KEY

Khyâl cap often meets panic attack criteria and is tied to trauma; exploring its meaning builds trust and accuracy.

B. Tell her the wind belief is mistaken and teach a medical model of panic at once

Dismissing her explanatory model damages rapport and ignores how she understands the attacks.

C. Record a delusional belief about her body and refer her for antipsychotic care

A shared cultural explanation of symptoms is not a delusion and does not suggest psychosis.

D. Treat the attacks as culture-bound and not a target for any clinical attention

Cultural framing does not make distress untreatable; the attacks still warrant assessment and care.

Rationale: Khyâl cap, or "wind attacks," is a Cambodian cultural syndrome with panic-like symptoms and the fear that wind rising in the body will cause serious harm. Episodes often meet panic attack criteria and are common among Cambodian refugees with trauma histories. Using tools such as the Cultural Formulation Interview, counselors explore the client's explanation while assessing for panic disorder and PTSD.

References: DSM-5-TR — Glossary of cultural concepts of distress: khyâl cap; panic attack specifier, culture-related issues · Sue & Sue (Counseling the Culturally Diverse) — Culture-bound syndromes and clients' explanatory models of illness

☐ Approve ☐ Revise ☐ Retire Notes:

A 47-year-old warehouse worker had a spinal injury 9 months ago that left him using a wheelchair. Since a month after the injury he has had low mood, tearfulness and worry that do not meet criteria for another disorder. Distress is out of proportion to what his care team expected. Which adjustment disorder specifier applies?

A. Acute, since the symptoms began soon after an identifiable stressor

Acute applies only when the disturbance has lasted less than 6 months.

B. Not applicable, as symptoms past 6 months rule out adjustment disorder

Symptoms may continue when the stressor's consequences endure, as with a lasting disability.

C. Persistent (chronic), since the symptoms have lasted 6 months or more ✓ KEY

Persistent (chronic) is used when the disturbance lasts 6 months or longer, as with enduring consequences.

D. With delayed expression, since full symptoms appeared after a month

Delayed expression is a PTSD specifier, not an adjustment disorder specifier.

Rationale: Adjustment disorder symptoms begin within 3 months of a stressor and do not persist more than 6 months after the stressor or its consequences have ended. When a stressor has lasting consequences, such as a permanent disability, symptoms can continue longer. DSM-5-TR uses the specifier acute for a disturbance lasting under 6 months and persistent (chronic) for one lasting 6 months or longer.

References: DSM-5-TR — Adjustment disorders: Criterion E and acute vs persistent (chronic) specifiers

☐ Approve ☐ Revise ☐ Retire Notes:

A 5-year-old saw her father seriously injured in a fall at home 2 months ago. Since then she has frequent nightmares, tantrums and trouble sleeping, and she refuses to go near the stairs. She shows no other avoidance and no changes in mood or beliefs. How does DSM-5-TR treat her avoidance and mood symptoms for PTSD in children 6 and younger?

A. One symptom of either avoidance or negative mood and thinking can suffice ✓ KEY

For children 6 and younger, one symptom from the combined avoidance or negative cognition and mood list is enough.

B. Two avoidance symptoms are needed, the same rule used for older children

Older children and adults need one avoidance symptom plus two negative cognition or mood symptoms.

C. PTSD cannot be given before age 7, so an adjustment disorder is recorded

DSM-5-TR has a separate set of PTSD criteria for children 6 years and younger.

D. She must put her fear into words before avoidance symptoms are counted

The preschool criteria rely on observed behavior; verbal report of fear is not required.

Rationale: DSM-5-TR gives separate PTSD criteria for children 6 years and younger. Instead of requiring both avoidance and two negative cognition and mood symptoms, the preschool criteria need only one symptom from a combined list of avoidance or negative changes in mood and thinking. The criteria are behavior-based, which suits young children who cannot describe their inner states.

References: DSM-5-TR — PTSD for children 6 years and younger: Criterion C (one avoidance or negative cognition/mood symptom)

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor preparing a staff workshop wants to describe how obsessive-compulsive disorder typically begins and unfolds. Which statement about OCD across the lifespan is accurate according to DSM-5-TR?

A. First onset after age 35 is typical, as most cases begin in midlife

Onset after 35 is unusual; the mean age at onset in the U.S. is about 19.5 years.

B. Females tend to show symptoms earlier than males, often by age 10

The reverse is true; males have an earlier age at onset than females.

C. Untreated cases usually remit for good within the first year or so

Without treatment the course is usually chronic, often waxing and waning; full remission is uncommon.

D. Males tend to have an earlier onset, often during their childhood ✓ KEY

Males have earlier onset, and nearly a quarter of males develop OCD before age 10.

Rationale: DSM-5-TR reports a mean age at onset of OCD of about 19.5 years in the United States, with a quarter of cases starting by age 14. Males have an earlier onset than females, and nearly 25% of males have onset before age 10. Without treatment the course is usually chronic, so early recognition in schools can shorten years of impairment.

References: DSM-5-TR — Obsessive-compulsive disorder: development and course; gender-related issues

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old spends 3 hours a day arranging objects until they feel "right." She is completely convinced that if she stops, her mother will die, and no evidence shakes this belief. She has no hallucinations or other psychotic symptoms, and her thinking is otherwise organized. Which diagnosis BEST fits?

A. Delusional disorder, since her belief is fixed and resists all evidence

Her belief is the content of an obsession tied to compulsions, which DSM-5-TR keeps within OCD.

B. Schizophrenia, since a fixed false belief shows active psychotic illness

Schizophrenia needs two or more psychotic symptoms over time; she has none beyond this OCD belief.

C. OCD with absent insight/delusional beliefs, the OCD insight specifier ✓ **KEY**

When a person is fully convinced OCD beliefs are true, OCD is diagnosed with this specifier, not a psychotic disorder.

D. Obsessive-compulsive personality disorder, given her rigid perfectionism

OCPD involves ego-syntonic perfectionism without true obsessions and compulsions like hers.

Rationale: DSM-5-TR rates insight in OCD as good or fair, poor, or absent/delusional beliefs. A person who is completely convinced that OCD beliefs are true still receives an OCD diagnosis with the absent insight/delusional beliefs specifier, rather than a psychotic disorder. Treatment remains exposure and response prevention and medication, though poor insight can make engagement harder.

References: DSM-5-TR — Obsessive-compulsive disorder: insight specifiers; differential diagnosis with psychotic disorders

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old who has used heroin daily for a year last used about 18 hours ago. In session they keep yawning, have a runny nose and watery eyes, show goosebumps and dilated pupils, and report muscle aches and diarrhea. Which condition do these signs MOST suggest?

A. Opioid intoxication

Opioid intoxication brings drowsiness, slurred speech and constricted, not dilated, pupils.

B. Opioid withdrawal ✓ **KEY**

Yawning, tearing, runny nose, goosebumps, dilated pupils, aches and diarrhea are classic opioid withdrawal signs.

C. Stimulant intoxication

Stimulants can dilate pupils, but they bring euphoria or agitation, not yawning, aches and diarrhea.

D. Cannabis withdrawal

Cannabis withdrawal centers on irritability, sleep and appetite problems, not these autonomic signs.

Rationale: DSM-5-TR opioid withdrawal includes dysphoric mood, nausea or vomiting, muscle aches, tearing or runny nose, dilated pupils, goosebumps or sweating, diarrhea, yawning, fever and insomnia. With short-acting opioids such as heroin, symptoms usually begin within 6 to 12 hours of the last dose. Opioid intoxication, by contrast, causes pupillary constriction and drowsiness, so the pupils are a useful clue.

References: DSM-5-TR — Opioid withdrawal: Criterion B signs and symptoms; opioid intoxication: pupillary constriction

☐ Approve ☐ Revise ☐ Retire Notes:

A college counselor is reviewing DSM-5-TR intoxication criteria with interns so they can recognize recent substance use during walk-in sessions. Which set of physical signs is listed for cannabis intoxication?

A. Slurred speech, unsteady gait, nystagmus and poor attention or memory

These signs describe alcohol or sedative intoxication, not cannabis intoxication.

B. Constricted pupils, drowsiness, slurred speech and impaired memory

Pupillary constriction with drowsiness is the pattern of opioid intoxication.

C. Dilated pupils, chills, nausea, weight loss and psychomotor agitation

These signs fit stimulant intoxication, such as cocaine or amphetamine use.

D. Red eyes, increased appetite, dry mouth and a rapid heart rate ✓ **KEY**

Conjunctival injection, increased appetite, dry mouth and tachycardia are the listed cannabis signs.

Rationale: DSM-5-TR cannabis intoxication requires problematic behavioral or psychological changes plus two or more signs within 2 hours of use: conjunctival injection (red eyes), increased appetite, dry mouth and tachycardia. Knowing each substance's pattern of signs helps counselors tell recent use from other causes of a change in presentation. Constricted pupils point to opioids, while dilated pupils with agitation point to stimulants.

References: DSM-5-TR — Cannabis intoxication: Criterion C physical signs

☐ Approve ☐ Revise ☐ Retire Notes:

A mother says her 16-year-old son, diagnosed with ADHD combined presentation at age 7, "must have outgrown it," since he no longer runs and climbs everywhere. He still loses assignments, misses deadlines and drifts off in class, and he says he feels fidgety inside. How should the counselor respond?

A. Explain that hyperactivity often fades to restlessness while inattention lasts ✓ **KEY**

In adolescence, overt hyperactivity often becomes inner restlessness, while inattention tends to persist.

B. Agree that ADHD has resolved, as fewer motor symptoms mean it is no longer present

His ongoing inattention and impairment show the disorder is still active.

C. Suggest a new diagnosis is needed, since ADHD cannot shift between presentations

ADHD presentations can change over time; a new diagnosis is not required.

D. Advise her that adolescent inattention is a typical phase that needs no support

His symptoms cause real academic impairment and continue a lifelong pattern that warrants support.

Rationale: DSM-5-TR notes that motor hyperactivity is often most visible in preschool and early school years and becomes less obvious in adolescence. In teens and adults it may show as fidgeting, impatience or an inner feeling of restlessness, while inattention and poor planning often persist. A shift from combined to predominantly inattentive presentation is common and does not mean the disorder has resolved.

References: DSM-5-TR — ADHD: development and course; changes in presentation across development · Berk (Development Through the Lifespan) — ADHD in adolescence and adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old with average intellectual ability reads and writes at grade level. However, despite a year of targeted tutoring, he cannot recall basic math facts, counts on his fingers to add small numbers, and makes frequent calculation errors. Which diagnosis BEST fits?

A. Intellectual developmental disorder, mild severity

His intellectual ability is average and his deficits are limited to one academic area.

B. Specific learning disorder, impairment in mathematics ✓ KEY

Persistent trouble with number sense, math facts and calculation despite help fits this specifier.

C. Specific learning disorder, impairment in written expression

His writing is at grade level; the impairment is in math skills.

D. ADHD, with a predominantly inattentive presentation

No broad inattention is described; his difficulty is specific to mathematics.

Rationale: Specific learning disorder requires persistent difficulty in a key academic skill for at least 6 months despite targeted help, with skills well below age expectations. DSM-5-TR specifies the affected domain: reading, written expression or mathematics. Impairment in mathematics includes trouble with number sense, memorizing arithmetic facts, accurate calculation and math reasoning, a pattern sometimes called dyscalculia.

References: DSM-5-TR — Specific learning disorder: with impairment in mathematics; dyscalculia

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old woman from Puerto Rico whose husband died 3 months ago tells the counselor that some evenings she hears him call her name and feels he is near. She finds this comforting, says it is common in her family, and is otherwise managing daily life. What should the counselor do?

A. Refer her urgently for antipsychotic medication to treat the hallucinations

An isolated, comforting experience of this kind does not point to psychosis needing medication.

B. Record schizophrenia, since hearing voices is a core feature of the disorder

Schizophrenia needs several symptoms over at least 6 months with decline; none is described.

C. Explore what the experience means to her, as it alone is not a sign of psychosis ✓ KEY

Hearing a deceased loved one is common in grief and accepted in many cultures, so meaning matters most.

D. Ask her to stop discussing the voice so that she can accept that he is gone

Shutting down the topic dismisses her experience and her culture and may harm rapport.

Rationale: DSM-5-TR cautions that hallucinations with religious content or perceptions of a deceased loved one can be a normal part of experience in some cultures and in grief. Before thinking of psychosis, counselors consider the person's culture, the distress and impairment involved, and whether other symptoms are present. Here, a comforting, culturally familiar experience with intact functioning calls for respectful exploration, not a psychotic disorder diagnosis.

References: DSM-5-TR — Schizophrenia: culture-related diagnostic issues; hallucinations in culturally sanctioned contexts · Sue & Sue (Counseling the Culturally Diverse) — Counseling Latinx clients: cultural values and spiritual beliefs

☐ Approve ☐ Revise ☐ Retire Notes:

A 21-year-old meets criteria for schizophreniform disorder. The clinician is deciding whether to add the "with good prognostic features" specifier, which requires at least two of four listed features. Which finding counts as one of those features?

A. Prominent psychosis began within 4 weeks of the first change in functioning ✓ KEY

Rapid onset of psychosis within 4 weeks of the first noticeable change is one of the four features.

B. A long, slow period of social withdrawal and decline came before the psychosis

A long, insidious prodrome is linked to poorer outcome, not to good prognostic features.

C. Blunted or flat affect was present through most of the active-phase symptoms

The specifier requires the absence of blunted or flat affect, not its presence.

D. A first-degree relative has been treated for schizophrenia for several years

Family history is a risk factor, not one of the listed good prognostic features.

Rationale: The "with good prognostic features" specifier for schizophreniform disorder requires at least two of: onset of prominent psychotic symptoms within 4 weeks of the first noticeable change in usual behavior, confusion or perplexity, good premorbid social and occupational functioning, and the absence of blunted or flat affect. These features predict a better chance of recovery. A slow onset with negative symptoms points toward a poorer outcome.

References: DSM-5-TR — Schizophreniform disorder: with good prognostic features specifier

☐ Approve ☐ Revise ☐ Retire Notes:

A 36-year-old who came to the U.S. as a political refugee 2 years ago is guarded with the counselor, asks who will see his records, and says he does not trust officials. He speaks limited English and has faced discrimination at work. A colleague suggests paranoid personality disorder. What is the BEST response?

A. Agree, since mistrust of others is the core feature of paranoid personality

Mistrust that fits his life circumstances is not a pervasive, unwarranted personality pattern.

B. Agree, but add a delusional disorder diagnosis for his beliefs about officials

His wariness is grounded in real experiences and is not a fixed false belief.

C. Delay any rapport-building until his mistrust has been fully assessed

Building trust is part of assessment; delaying it would likely deepen his guardedness.

D. Consider that his guardedness may reflect refugee history and discrimination ✓ KEY

DSM-5-TR warns that wariness tied to refugee status or discrimination can be mislabeled as paranoid.

Rationale: DSM-5-TR notes that behaviors shaped by sociocultural context, such as guardedness in immigrants, political refugees or members of minority groups who face discrimination, can be wrongly labeled paranoid. Language barriers and unfamiliarity with systems can add to this wariness. Counselors weigh the person's history and context before diagnosing a personality disorder and focus on building trust.

References: DSM-5-TR — Paranoid personality disorder: culture-related diagnostic issues · Sue & Sue (Counseling the Culturally Diverse) — Cultural mistrust and healthy cultural paranoia in clients of color

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old had a stroke affecting the frontal lobe 8 months ago. His wife reports that, unlike before, he now makes rude comments to strangers, spends money impulsively and loses his temper over small things. He had steady work and relationships for decades. Which diagnosis BEST fits?

A. Antisocial personality disorder, given his new impulsive and rude behavior

Antisocial PD requires a pattern since age 15 with conduct disorder before 15; his change is recent.

B. Borderline personality disorder, given his anger and impulsive spending

A personality disorder must trace back to adolescence or early adulthood, not begin in midlife.

C. Personality change due to another medical condition, the stroke ✓ KEY

A persistent change from his prior personality caused by a medical condition fits this diagnosis.

D. Intermittent explosive disorder, given his outbursts over small things

IED is not diagnosed when outbursts are better explained by the effects of a medical condition.

Rationale: A personality disorder is an enduring pattern whose onset can be traced to adolescence or early adulthood and that is not due to a medical condition. When a persistent change from a person's prior personality is the direct result of a medical condition, such as a stroke or brain injury, DSM-5-TR diagnoses personality change due to another medical condition. Subtypes include disinhibited, aggressive and labile types.

References: DSM-5-TR — General personality disorder criteria (onset, exclusions); personality change due to another medical condition

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old meets criteria for bulimia nervosa. Her weight is in the normal range. Over the past 3 months she has averaged about 10 episodes of self-induced vomiting per week after binges. Based on DSM-5-TR, what is the current severity level?

A. Moderate

Moderate covers 4 to 7 compensatory episodes per week.

B. Severe ✓ KEY

Severe covers 8 to 13 compensatory episodes per week, which fits her average of 10.

C. Mild

Mild covers 1 to 3 compensatory episodes per week.

D. Extreme

Extreme begins at 14 or more compensatory episodes per week.

Rationale: Bulimia nervosa severity in DSM-5-TR is based on how often inappropriate compensatory behaviors such as vomiting occur. Mild is 1 to 3 episodes per week, moderate 4 to 7, severe 8 to 13 and extreme 14 or more. Severity may be raised to reflect other symptoms and functional disability, and frequent purging raises medical risk.

References: DSM-5-TR — Bulimia nervosa: severity specifiers based on compensatory behaviors per week

☐ Approve ☐ Revise ☐ Retire Notes:

For 2 months, an 11-year-old has brought food back up into her mouth without effort shortly after most meals, then rechews and swallows it or spits it out. She has no nausea, a full medical workup found no gastrointestinal cause, and she has no concerns about weight or shape. Which diagnosis BEST fits?

A. Rumination disorder ✓ **KEY**

Repeated regurgitation for at least a month, not due to a medical condition, fits rumination disorder.

B. Bulimia nervosa

Bulimia involves binges, purging to prevent weight gain and undue focus on weight or shape.

C. Anorexia nervosa

Anorexia requires restriction leading to low weight and fear of gaining weight.

D. Pica

Pica is the persistent eating of nonfood substances, not regurgitation of food.

Rationale: Rumination disorder involves repeated regurgitation of food over at least 1 month, with the food rechewed, reswallowed or spit out. The behavior is not due to a gastrointestinal or other medical condition and does not occur only during another eating disorder. Because it can lead to weight loss and social avoidance at meals, counselors coordinate with medical providers and may use habit reversal methods such as diaphragmatic breathing.

References: DSM-5-TR — Rumination disorder: Criteria A-C

☐ Approve ☐ Revise ☐ Retire Notes:

A 42-year-old is referred for growing irritability, trouble planning and organizing at work, and new jerky, dance-like movements of his hands and face. His mother and his grandfather developed similar problems in midlife. Which neurocognitive disorder do these features MOST suggest?

A. NCD due to Alzheimer's disease

Alzheimer's usually begins later with memory loss and lacks early choreiform movements.

B. Vascular neurocognitive disorder

Vascular NCD follows cerebrovascular events, often in a stepwise course, without chorea.

C. NCD with Lewy bodies

Lewy body NCD features fluctuating cognition, visual hallucinations and parkinsonism, not chorea.

D. NCD due to Huntington's disease ✓ **KEY**

Midlife onset, chorea, executive decline, irritability and a dominant family pattern point to Huntington's.

Rationale: Huntington's disease is an inherited, autosomal dominant condition, so a parent with the illness gives a child a 50% chance of carrying the gene. DSM-5-TR notes an average age at diagnosis of about 40 years, with insidious onset and gradual progression. Early signs often include irritability, depression, anxiety and executive dysfunction, with choreiform movements as the hallmark motor feature.

References: DSM-5-TR — Major or mild neurocognitive disorder due to Huntington's disease

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old with 30 years of heavy daily drinking has been sober for 4 months. He cannot learn new information, forgetting a conversation within minutes, and he fills gaps with detailed but invented stories. His attention and other skills are largely intact, and he needs help to manage his daily life. Which diagnosis BEST fits?

A. Alcohol withdrawal delirium, since the memory gaps followed his drinking

Delirium involves disturbed attention with acute onset; his deficits are stable months into sobriety.

B. Alcohol-induced major NCD, amnestic-confabulatory type, from heavy use ✓ **KEY**

Persistent, severe new-learning loss with confabulation after long heavy drinking fits this specifier.

C. Major NCD due to Alzheimer's disease, given the profound memory loss

The link to long heavy drinking and the confabulation point to an alcohol-induced cause.

D. Alcohol use disorder in early remission, with expected memory lapses

Remission status does not explain a severe, lasting amnestic disorder that impairs independence.

Rationale: DSM-5-TR includes an amnestic-confabulatory type for alcohol-induced major neurocognitive disorder, a presentation also known as Korsakoff syndrome. It reflects thiamine deficiency linked to chronic heavy drinking and features severe loss of new learning with confabulation, while other abilities may be relatively spared. Because deficits persist beyond intoxication and withdrawal, care often involves supported living and memory aids.

References: DSM-5-TR — Substance/medication-induced major NCD: alcohol, amnestic-confabulatory type

☐ Approve ☐ Revise ☐ Retire Notes:

Over only 4 months, a 61-year-old has gone from managing her own business to needing help dressing. Her family notes sudden, brief jerks of her arms and a new unsteady gait. She has no history of stroke, head injury or substance use. Which neurocognitive disorder do these features MOST suggest?

A. NCD due to Alzheimer's disease, typical of early-onset cases

Alzheimer's progresses gradually over years, not to severe impairment within months.

B. Mild neurocognitive disorder, since symptoms began only recently

She now needs help with daily tasks, so her impairment is major, not mild.

C. NCD due to prion disease, given rapid decline with myoclonus ✓ **KEY**

Very rapid progression with myoclonus and ataxia is the hallmark of prion disease.

D. Frontotemporal NCD, since her decline began in her early 60s

Frontotemporal NCD leads with behavior or language change and progresses more slowly.

Rationale: DSM-5-TR describes neurocognitive disorder due to prion disease, such as Creutzfeldt-Jakob disease, as having an insidious onset with commonly rapid progression. Motor features such as myoclonus or ataxia, or biomarker evidence, support the diagnosis. A decline over months rather than years is a key clue that sets it apart from Alzheimer's and frontotemporal NCD.

References: DSM-5-TR — Major or mild neurocognitive disorder due to prion disease

☐ Approve ☐ Revise ☐ Retire Notes:

The wife of a 67-year-old client reports that several times a month, late in the night, he shouts and punches while asleep. When she wakes him he is alert at once and describes a dream of fighting off an attacker. He recently struck her by accident. Which sleep disorder do these features MOST suggest?

A. REM sleep behavior disorder ✓ KEY

Acting out vivid dreams with full alertness on waking fits REM sleep behavior disorder.

B. Non-REM sleep terror type

Sleep terrors occur early in the night, with confusion on waking and little dream recall.

C. Nightmare disorder

Nightmares cause distressing dreams, but the person does not act them out physically.

D. Non-REM sleepwalking type

Sleepwalking occurs in non-REM sleep with confusion and amnesia, not vivid dream recall.

Rationale: REM sleep behavior disorder involves vocalizing and complex movements during REM sleep, often later in the night, with the person acting out dreams. On waking, the person is fully alert and can describe the dream. It is most common in older men and often precedes synucleinopathies such as Parkinson's disease or Lewy body disease, so referral for sleep and neurological evaluation and bedroom safety planning are warranted.

References: DSM-5-TR — Rapid eye movement sleep behavior disorder: criteria and development and course

☐ Approve ☐ Revise ☐ Retire Notes:

A 74-year-old says that over recent years her sleep has become lighter, she wakes more often during the night, and she gets sleepy by 8:30 p.m. and wakes near 5 a.m. She feels rested and functions well in the daytime. She asks whether something is wrong. Which response BEST reflects lifespan research?

A. Her pattern shows insomnia disorder and needs treatment to restore youthful sleep

Insomnia disorder requires distress or daytime impairment, which she does not have.

B. Older adults need much less sleep, so she should stay up later to sleep deeper

Sleep need changes only modestly with age, and staying up late does not restore deep sleep.

C. Lighter, broken sleep with earlier timing is common in aging and not harmful alone ✓ KEY

Aging brings less deep sleep, more waking and an earlier sleep phase; good daytime function is reassuring.

D. Waking early and sleeping lightly at her age signals early neurocognitive disorder

These changes are typical of normal aging and do not by themselves suggest cognitive decline.

Rationale: Normal aging brings less deep slow-wave sleep, more frequent nighttime waking and a shift toward earlier sleep and wake times. These changes are not a disorder when the person feels rested and functions well during the day. Counselors watch instead for daytime impairment, loud snoring or breathing pauses, and mood or medication effects that suggest a treatable sleep problem.

References: Berk (Development Through the Lifespan) — Late adulthood physical development: changes in sleep · DSM-5-TR — Circadian rhythm sleep-wake disorders: advanced sleep phase type, development and course

☐ Approve ☐ Revise ☐ Retire Notes:

A 19-year-old with severe intellectual developmental disorder repeatedly bangs his head and bites his hand, with the same rhythmic pattern many times a day since early childhood. Staff ask whether the DSM-5-TR proposed nonsuicidal self-injury disorder fits. Which conclusion is BEST supported?

- A. NSSI disorder fits, since he injures himself on at least 5 days each year
Frequency alone is not enough; the proposed criteria exclude repetitive stereotypies in this context.
- B. NSSI disorder fits, since the injuries are deliberate and lack suicidal intent
The behavior is better explained by a stereotypic pattern than by NSSI driven by an expected outcome.
- C. No disorder can be given, since self-injury is expected with his disability
Self-injurious stereotypies can be diagnosed and need intervention; they are not simply expected.
- D. Stereotypic movement disorder, with self-injurious behavior, fits better ✓ KEY
Repetitive, purposeless self-injury from early development in this context fits stereotypic movement disorder.

Rationale: The DSM-5-TR proposed criteria for nonsuicidal self-injury disorder state that the behavior is not better explained by another condition, including repetitive stereotypies in a person with an intellectual developmental disorder. Stereotypic movement disorder involves repetitive, seemingly driven and purposeless motor behavior beginning early in development, and it can be specified as with self-injurious behavior. Accurate diagnosis guides safety measures and behavioral treatment.

References: DSM-5-TR — Conditions for further study: nonsuicidal self-injury disorder, Criterion F; stereotypic movement disorder

☐ Approve ☐ Revise ☐ Retire Notes:

A 17-year-old in DBT says she cut herself again over the weekend but "has no idea why it happened." Her counselor wants to understand, step by step, the events, thoughts, feelings and urges that led to the cutting and what followed it. Which DBT technique BEST fits this goal?

- A. Radical acceptance, letting go of the fight against what has happened
Radical acceptance is a distress tolerance skill, not a method for mapping causes of a behavior.
- B. Behavior chain analysis of the links before and after the self-injury ✓ KEY
Chain analysis traces the prompting event, links and consequences of a target behavior in detail.
- C. The DEAR MAN skill, a script for making clear requests of other people
DEAR MAN is an interpersonal effectiveness skill for asking for what one wants.
- D. Opposite action, acting against the urge tied to an unjustified emotion
Opposite action changes emotions; it does not identify the chain that led to the cutting.

Rationale: In DBT, a behavior chain analysis is a detailed review of a problem behavior such as self-injury. Counselor and client identify vulnerability factors, the prompting event, each link of thoughts, feelings, sensations and actions, and the short- and long-term consequences. The analysis then points to where new skills can break the chain next time.

References: Linehan (DBT Skills Training Manual) — Behavior chain analysis of problem behavior

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old stopped taking paroxetine on her own 3 days ago because she "felt better." She now reports dizziness, nausea, irritability, flu-like aches and brief "electric shock" sensations in her head. She fears her depression is back. What should the counselor do?

A. Explain this may be discontinuation syndrome and urge a call to her prescriber ✓ KEY

Abruptly stopping paroxetine often causes these symptoms; the prescriber can guide restarting or tapering.

B. Tell her that the symptoms confirm relapse and that she should restart the pill

The physical symptoms point to discontinuation, and dosing decisions belong to the prescriber.

C. Suggest she wait it out, as stopping an antidepressant has no physical effects

Antidepressant discontinuation can cause real, distressing physical symptoms.

D. Advise taking an over-the-counter sleep aid until the strange sensations pass

Counselors do not recommend medications, and this would not address the cause.

Rationale: DSM-5-TR describes antidepressant discontinuation syndrome, which can follow abrupt stopping or sharp reduction of an antidepressant taken for at least a month. Symptoms such as dizziness, nausea, irritability, flu-like aches and electric shock sensations usually begin within 2 to 4 days and are more common with short half-life drugs like paroxetine. Counselors help clients tell this from relapse and refer them to the prescriber for a taper plan.

References: DSM-5-TR — Medication-induced movement disorders and other adverse effects: antidepressant discontinuation syndrome

☐ Approve ☐ Revise ☐ Retire Notes:

A client with schizophrenia began a new antipsychotic 10 days ago. In session he cannot sit still, shifts his legs constantly and paces the room. He says he feels "jittery inside" and cannot stop moving, and he denies new worries. What do these signs MOST likely reflect?

A. Tardive dyskinesia from long-term antipsychotic use

Tardive dyskinesia appears after months or years and involves involuntary mouth and face movements.

B. Akathisia, an acute side effect of the new antipsychotic ✓ KEY

Inner restlessness with pacing soon after starting an antipsychotic is the picture of acute akathisia.

C. Worsening psychotic agitation that calls for a higher dose

Mistaking akathisia for agitation and raising the dose can worsen it; the prescriber should assess.

D. Generalized anxiety disorder emerging with the illness

GAD needs 6 months of worry; he denies new worries and his restlessness began with the drug.

Rationale: Medication-induced acute akathisia is a subjective sense of restlessness with observable movements such as fidgeting, rocking or pacing, appearing within a few weeks of starting or raising an antipsychotic. It is often mistaken for anxiety or psychotic agitation, and it is distressing enough to drive nonadherence and has been linked to suicidal thinking. Counselors who notice it should prompt a timely review with the prescriber.

References: DSM-5-TR — Medication-induced movement disorders: medication-induced acute akathisia

☐ Approve ☐ Revise ☐ Retire Notes:

A 71-year-old cares at home for her husband, who has advanced Alzheimer's disease and no longer knows her name. She says, "He is right here, yet the man I married is gone. I don't know whether to grieve or keep hoping." Her friends tell her she cannot grieve someone who is still alive. Which concept BEST describes her loss?

A. Ambiguous loss, as he is present in body but absent in mind and identity ✓ KEY

Boss's ambiguous loss describes a loved one who is physically present but psychologically absent, which blocks closure.

B. Delayed grief, as she has pushed her feelings away and will grieve later

She is not suppressing feelings; she is openly describing a loss that has no clear end point.

C. Prolonged grief disorder, as her yearning has lasted for a very long time

Prolonged grief disorder requires a death at least 12 months earlier; her husband is still living.

D. Exaggerated grief, as her distress has grown into a separate disorder

Nothing suggests a psychiatric disorder; her confusion fits the unclear nature of the loss itself.

Rationale: Pauline Boss described ambiguous loss as a loss without clear facts or closure. In one type, a person is physically present but psychologically absent, as with advanced dementia; in the other, a person is physically missing but kept psychologically present, as with a missing soldier. Because the loss is unclear, others often fail to recognize it, and counselors help clients hold both the loss and the ongoing relationship rather than push for resolution.

References: Boss (Ambiguous Loss) — Psychological absence with physical presence: dementia caregiving · Worden (Grief Counseling) — Grief in caregivers and nonfinite loss

☐ Approve ☐ Revise ☐ Retire Notes:

A hospice plans to require six sessions of grief counseling for every adult family member after each patient's death, whether or not they want it. The program director asks a counselor for advice. Based on research on bereavement interventions, what should the counselor recommend?

A. Keep the plan, since early counseling for every mourner prevents later grief problems

Research has not shown that routine counseling for all bereaved people prevents later problems.

B. Replace counseling with a single group debriefing held in the week after each death

Single-session debriefing is not supported and does not target the people most likely to struggle.

C. Offer care to those at high risk or seeking help, as routine care shows little gain ✓ KEY

Interventions show the clearest benefit for high-risk mourners and those with complicated grief.

D. Delay any contact for one full year, since grief must run its course before help

Mourners at high risk or in distress can benefit from help well before a year has passed.

Rationale: Worden reviews three approaches to bereavement services: offering help to everyone, offering it to those judged at high risk, and waiting until problems appear or people ask. Outcome reviews find little benefit from routine intervention for people with normal grief, but better results for high-risk mourners and those with complicated grief. Programs therefore screen for risk and make help easy to reach rather than requiring it for all.

References: Worden (Grief Counseling) — Is grief counseling effective? Universal vs targeted (high-risk) bereavement intervention

☐ Approve ☐ Revise ☐ Retire Notes:

Within an hour of learning that his business partner embezzled their savings, a 50-year-old arrives at a walk-in clinic pacing, sobbing and unable to follow simple questions. According to James and Gilliland, which basic crisis intervention model is MOST suited to this stage of his crisis?

A. Cognitive model, to dispute his faulty thoughts about the theft right away

The cognitive model fits once a client is stable enough to examine and change his thinking.

B. Equilibrium model, to calm him and restore a basic level of stable coping ✓ KEY

The equilibrium model fits early intervention, when a person is out of control and cannot make choices.

C. Psychosocial transition model, to review how his past shaped this event

The psychosocial transition model draws on inner and outer resources after the client has stabilized.

D. Developmental model, to place the loss within his midlife stage tasks

This is not one of the three basic crisis models, and it does not fit the acute stage of his crisis.

Rationale: James and Gilliland describe three basic crisis intervention models. The equilibrium model aims to restore a person in disequilibrium to a steady state and is most appropriate early, when the person is out of control and disoriented. The cognitive and psychosocial transition models are better suited after stabilization, when the client can work on thoughts or on inner and outer resources.

References: James & Gilliland (Crisis Intervention Strategies) — Basic crisis intervention models: equilibrium, cognitive and psychosocial transition

☐ Approve ☐ Revise ☐ Retire Notes:

At a family assistance center after a bridge collapse, a responder finds a young man sitting alone, staring blankly and rocking. He does not respond when asked his name. Which Psychological First Aid core action should the responder use NEXT?

A. Information gathering, by asking him to describe the collapse in full detail

PFA does not press survivors for a detailed account, and he cannot yet engage in an interview.

B. Linkage with services, by handing him a list of local mental health clinics

Referral material is of little use to a survivor who cannot yet attend to what is said.

C. Information on coping, by teaching him a set of steps for managing stress

Coping education comes later; he must first be calm and oriented enough to take it in.

D. Stabilization, by calmly helping him orient to where he is and who is near ✓ KEY

Stabilization is for survivors who are overwhelmed or disoriented, such as those glassy-eyed and unresponsive.

Rationale: The PFA Field Operations Guide lists eight core actions, including contact and engagement, safety and comfort, stabilization, information gathering, practical assistance, connection with social supports, information on coping, and linkage with collaborative services. Stabilization is used when survivors are emotionally overwhelmed or disoriented, with signs such as a vacant stare, no response to questions, or rocking. The responder stays close, speaks calmly and helps the person orient before moving to other actions.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — Core Action 3: Stabilization of emotionally overwhelmed survivors

☐ Approve ☐ Revise ☐ Retire Notes:

A week after a shooting at a community festival, a town council asks a counselor what to expect for residents' mental health over the coming months. Which response is MOST accurate?

A. Most will have distress that fades with time, while a smaller group needs care ✓ **KEY**

Most survivors recover with support and time; a minority develop lasting problems that need treatment.

B. Most will develop PTSD and should start trauma therapy as soon as possible

Only a minority of people exposed to a disaster develop PTSD; resilience is the common course.

C. Few will be affected, since distress after a single event tends to be rare

Early distress is common and expected; it is the lasting impairment that affects a minority.

D. Symptoms are unlikely to appear until a year later, so services can wait

Reactions often begin right away, and early practical and emotional support is helpful.

Rationale: Psychological First Aid rests on the finding that most people exposed to disasters and mass violence have early distress that eases over time, especially with practical help and social support. A smaller group develops lasting problems such as PTSD, depression or substance misuse. Communities therefore offer broad, supportive help early and screen and follow up so that those who need treatment are linked to it.

References: Psychological First Aid Field Operations Guide (NCTSN/NCPTSD) — Basic objectives of PFA: expected recovery and natural resilience after disaster

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old client whose husband has hit her says she has decided to stay for now. She has no income of her own, fears he will become more violent if she leaves, and wants to keep the family together. What is the BEST response from the counselor?

A. Tell her the counselor cannot keep working with her unless she agrees to leave

Making care depend on leaving is coercive and cuts her off from support when she needs it most.

B. Respect her choice and build a safety plan for staying, with resources to use ✓ **KEY**

Honoring her autonomy while planning for safety where she is keeps her engaged and safer.

C. Begin couples sessions with her husband so that the two of them can resolve it

Couples work while violence is ongoing can raise her risk and implies shared blame for the abuse.

D. Explore how her own behavior may be setting off his anger before each incident

This blames the victim; responsibility for violence lies with the person who uses it.

Rationale: People stay in abusive relationships for many reasons, including economic dependence, fear of escalation, children and hope for change. Pressure to leave can damage trust and does not match the client's own read of her risk. Counselors respect her decisions, assess danger, and build a safety plan that fits her current situation while making shelters, advocates and legal options known.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: reasons for staying and safety planning with the survivor

☐ Approve ☐ Revise ☐ Retire Notes:

A court refers a 35-year-old man after he was arrested for assaulting his partner. He says, "I just have a temper. Put me in an anger management class and I'll be fine." Which referral is MOST appropriate?

- A. A general anger management class focused on spotting triggers and calming down
Anger management treats violence as a loss of control and misses the pattern of power and control.
- B. Conjoint couples counseling so both partners can learn to argue more safely
Couples work is not recommended with ongoing violence and may place the partner at greater risk.
- C. A specialized program for people who abuse partners, stressing accountability ✓ KEY
Batterer intervention programs address beliefs about power and control and hold the person responsible.
- D. Individual insight therapy on how his childhood shaped his feelings of anger
Exploring childhood alone does not address the controlling behavior or his responsibility for it.

Rationale: Partner abuse is a pattern of power and control, not only a problem of anger. Generic anger management can let the person frame the violence as a temper issue and blame the partner for provoking it. Specialized intervention programs confront controlling beliefs, focus on accountability and are coordinated with the courts and with advocates for the victim's safety.

References: James & Gilliland (Crisis Intervention Strategies) — Intimate partner violence: intervention with abusers vs anger management

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old woman says her girlfriend tracks her phone and has threatened to tell the client's conservative family and employer that she is a lesbian if she ever ends the relationship. The client is not out to either. How should the counselor view this threat?

- A. As ordinary conflict, since there has not been any physical violence so far
Abuse includes emotional and controlling tactics; physical violence is not required.
- B. As mutual fighting, since partner abuse is unusual between two women
IPV occurs in same-sex relationships at rates similar to or higher than in other couples.
- C. As a privacy issue alone, to be handled by legal advice rather than safety work
The threat is used to trap her in the relationship and calls for safety planning, not only legal advice.
- D. As a control tactic, using the threat of outing to keep her from leaving ✓ KEY
Threatening to out a partner is a form of coercive control specific to LGBTQ+ relationships.

Rationale: Intimate partner violence in same-sex relationships follows the same pattern of power and control seen in other relationships, with some added tactics. Threatening to reveal a partner's sexual orientation or gender identity to family, employers or others uses stigma to keep the partner from leaving or seeking help. Counselors recognize this as abuse, assess safety, and connect clients with LGBTQ+-affirming advocacy services.

References: James & Gilliland (Crisis Intervention Strategies) — Partner violence in same-sex relationships: outing as a control tactic

☐ Approve ☐ Revise ☐ Retire Notes:

A primary care clinic asks a counselor why it should screen adult patients for adverse childhood experiences (ACEs), such as abuse, neglect and household dysfunction. Which finding from the ACE Study BEST supports screening?

A. Only sexual abuse predicts later health problems, so other ACEs add little

Many types of adversity, not only sexual abuse, are linked to later health and mental health risks.

B. As the number of ACEs rises, so does the risk of many health problems ✓ **KEY**

The ACE Study found a graded, dose-response link between ACE count and later health risks.

C. ACEs are rare among middle-class adults, so screening finds few cases

The original study sample was largely middle class, and ACEs were common in it.

D. ACE effects fade by early adulthood, so their health impact is short

The study linked childhood adversity to health outcomes decades later in adulthood.

Rationale: The CDC-Kaiser ACE Study surveyed a large, mostly middle-class adult sample and found that childhood adversity was common. It showed a graded relationship: the more types of adversity a person reported, the higher the risk of outcomes such as depression, substance misuse, suicide attempts and chronic disease. Trauma-informed care uses this finding to justify routine, sensitive screening and support.

References: SAMHSA TIP 57 — Trauma-informed care: the Adverse Childhood Experiences Study and its graded findings

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-year-old is well fed and has no injuries. Her father, however, calls her "worthless" daily, threatens to give her dog away when she cries, and locks her in a closet when angry. The counselor notes that she has become withdrawn and fearful. Which DSM-5-TR condition BEST fits what she is experiencing?

A. Child neglect, since her father fails to meet her emotional needs

Neglect is a failure to provide basic needs; these are active, harmful acts by the parent.

B. Parent-child relational problem, as their communication is poor

This label fits strained relationships; it does not capture deliberate threats and confinement.

C. Child psychological abuse, from his threats, insults and confinement ✓ **KEY**

Nonaccidental verbal or symbolic acts that cause significant psychological harm define this condition.

D. Child physical abuse, since being shut in a closet harms the body

Physical abuse involves nonaccidental physical injury; confinement here is a form of psychological abuse.

Rationale: DSM-5-TR defines child psychological abuse as nonaccidental verbal or symbolic acts by a parent or caregiver that result, or could reasonably result, in significant psychological harm. Examples include berating, disparaging or humiliating the child, threatening to harm or abandon loved people or pets, and confining the child, such as in a closet. The condition is coded as confirmed or suspected and also triggers the counselor's mandated reporting duty.

References: DSM-5-TR — Other Conditions That May Be a Focus of Clinical Attention: Child Psychological Abuse

☐ Approve ☐ Revise ☐ Retire Notes:

Two weeks after a counselor reported a 12-year-old's disclosure of sexual abuse by her stepfather, the girl says she "made it all up." Her mother, who is upset about the family's money problems since he moved out, sits in the waiting room. What should the counselor do?

A. Tell CPS about the recantation and keep supporting her without pressure ✓ KEY

Recanting is common under family pressure; CPS needs the new information and she needs steady support.

B. Withdraw the report, since the child has now said that it did not happen

A recantation does not undo reasonable suspicion, and the counselor cannot simply withdraw a report.

C. Confront her with the details of her first account to show it was true

Pressing her adds to her distress and makes the counselor an investigator, which is not the role.

D. Ask the mother to help decide which version of events should be believed

The mother may be a source of pressure, and judging the truth belongs to the investigators.

Rationale: Children often recant abuse disclosures, especially when a nonoffending parent is unsupportive, the family faces losses after the report, or the child feels blamed for the fallout. A recantation is new information for CPS but does not by itself cancel the reasonable suspicion that triggered the report. The counselor shares it with CPS as required, avoids investigative questioning, and stays a calm, consistent support for the child.

References: State law — Child abuse reporting: reasonable suspicion standard and sharing new information with CPS

☐ Approve ☐ Revise ☐ Retire Notes:

A 45-year-old bank teller knows his branch will close within a year. He is planning for the change, takes charge of his decisions and believes he can learn new skills, but he has not looked into any other kinds of work and says he "wouldn't know where to start." In Savickas's model of career adaptability, which dimension is MOST in need of support?

A. Concern, his sense that his work future is worth planning for now

He is already planning for the change, which shows concern for his future.

B. Control, his sense that the career choices ahead belong to him

He takes charge of his decisions, which reflects a sense of control.

C. Confidence, his belief that he can master the tasks ahead of him

He believes he can learn new skills, so self-efficacy is not the main gap.

D. Curiosity, his exploration of possible selves and options at work ✓ KEY

He has not explored any options, which points to a gap in curiosity.

Rationale: Savickas's career construction theory describes career adaptability in four dimensions: concern about the future, control over decisions, curiosity that drives exploration of self and options, and confidence to pursue goals. Matching help to the weak dimension makes intervention efficient. For a client low in curiosity, the counselor uses information-seeking tasks, job shadowing and exploration of transferable skills.

References: Sharf (Career Development Theory) — Savickas's career construction theory: concern, control, curiosity and confidence

☐ Approve ☐ Revise ☐ Retire Notes:

Since her parents divorced, a 12-year-old cooks dinner, gets her two younger brothers to bed, and listens late at night as her depressed mother confides her money worries and loneliness. Teachers say the girl seems tired and "older than her years." Which family concept BEST describes this pattern?

A. Emotional cutoff, as the girl pulls away from her family to manage

Cutoff is distancing to manage fusion; she is drawn closer into an adult role, not away.

B. Triangulation, as she is pulled into the conflict between two parents

Triangulation draws a third person into a dyad's conflict; here she has taken over a parent's role.

C. Parentification, as the girl takes on a parent's tasks and burdens ✓ KEY

Parentification places adult caregiving and emotional duties on a child at the cost of her needs.

D. Differentiation, as she grows into a mature and self-reliant person

Differentiation is a healthy balance of self and connection, not a child carrying adult burdens.

Rationale: Boszormenyi-Nagy's contextual therapy describes parentification as a child taking on parental roles, whether practical caregiving or serving as a parent's emotional confidant. Some responsibility can build skills, but chronic parentification without acknowledgment drains the child and creates an imbalance of give and take across generations. Counselors restore generational boundaries, link the parent with adult supports, and acknowledge the child's contributions.

References: Goldenberg (Family Therapy) — Contextual therapy (Boszormenyi-Nagy): parentification and relational ethics

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor notices that many clients who finished college during a deep national recession describe the same pattern years later: lower starting pay, delayed home buying and lasting worry about job security. In the lifespan perspective, which type of influence BEST explains this shared pattern?

A. Normative age-graded influences, shared by everyone at a given age

Age-graded influences are tied to age itself, such as puberty or school entry, not to an era.

B. Nonnormative influences, unusual events that happen to only a few

Nonnormative events are irregular and individual, not shared by a whole cohort.

C. Critical periods, set windows when a skill must be acquired or lost

Critical periods concern biological readiness windows, not economic conditions of an era.

D. History-graded influences, forces tied to a period in history ✓ KEY

Economic downturns, wars and epidemics are history-graded influences that shape a whole cohort.

Rationale: Baltes's lifespan perspective, as presented by Berk, sorts influences into normative age-graded, normative history-graded and nonnormative. History-graded influences are forces unique to an era, such as recessions, epidemics, wars and technological change, which explain why people born at the same time share certain patterns. Counselors who recognize cohort effects can frame clients' struggles as partly structural rather than purely personal.

References: Berk (Development Through the Lifespan) — Lifespan perspective: age-graded, history-graded and nonnormative influences

☐ Approve ☐ Revise ☐ Retire Notes:

Two months after his parents separated, a 5-year-old tells his preschool teacher, "Daddy left because I was too loud." His parents are puzzled, since they never said anything like that. Which explanation should the counselor offer them?

A. Young children often blame themselves for divorce and need clear reassurance ✓ KEY

Preschoolers' egocentric thinking often leads them to see themselves as the cause of a breakup.

B. His remark shows that he has overheard the parents arguing about his behavior

Self-blame is common in young children without any such cue; it need not reflect what he heard.

C. Self-blame at his age points to a depressive disorder that needs prompt treatment

Self-blame alone is a common, expected reaction at this age and is not a sign of a disorder.

D. He is trying to bring his parents back together by drawing attention to himself

Reunion fantasies are common, but this statement reflects self-blame rather than a plan.

Rationale: Berk notes that preschool and early school-age children often blame themselves for their parents' divorce and may fear being abandoned, reflecting their egocentric reasoning. Parents can help by explaining in simple terms that the decision was between the adults, that it was not caused by the child, and that both parents will keep caring for him. Predictable routines and contact with both parents also support adjustment.

References: Berk (Development Through the Lifespan) — Divorce: young children's self-blame and fears of abandonment

☐ Approve ☐ Revise ☐ Retire Notes:

A 27-year-old who became a mother at 16 is now finishing her associate degree among classmates nine years younger. She is proud of her progress but says she feels "out of step" with both her old friends, who are starting families, and her classmates. Which concept BEST frames her experience?

A. Identity foreclosure, as she committed to roles without exploration

Foreclosure is commitment without exploration; she is actively exploring new roles now.

B. Off-time transitions, as her life events differ from the social clock ✓ KEY

Early parenthood and later schooling are off-time relative to age norms, which can bring strain.

C. A non-event transition, as an event she expected has not taken place

Non-events are expected events that fail to happen; her events did happen, just at unusual times.

D. Role confusion, as she lacks a clear sense of her values and her goals

She is proud of her progress and has clear goals; her strain comes from timing, not confusion.

Rationale: Neugarten described the social clock, the age-graded expectations for when major life events such as finishing school, marrying and having children should occur. Events that occur well before or after these norms are off-time and can bring stress, fewer peers in the same position and less social support. Counselors help clients find peers who share their path and value the strengths gained from a nontraditional sequence.

References: Berk (Development Through the Lifespan) — Social clock: on-time and off-time life events in adulthood

☐ Approve ☐ Revise ☐ Retire Notes:

A 78-year-old retired farmer with arthritis has cut his large garden down to two raised beds of his favorite vegetables, spends more time tending them, and now uses a kneeling stool and long-handled tools. He says gardening is still the best part of his week. Which concept BEST describes how he is adapting?

A. Disengagement theory, as he withdraws from roles as his body declines

He has not withdrawn; he has reshaped a valued activity to keep doing it well.

B. Activity theory, as he keeps as busy as he was in his middle-aged years

He has reduced his garden on purpose rather than keeping his earlier activity level.

C. Selective optimization with compensation, as he narrows and adapts ✓ **KEY**

He selects key goals, optimizes effort on them, and compensates for losses with aids.

D. Gerotranscendence, as he turns from activity toward cosmic reflection

Gerotranscendence involves a shift toward spiritual reflection, not adapting a hobby.

Rationale: Baltes's selective optimization with compensation describes how older adults age well despite losses. They select a smaller set of valued goals, optimize their resources by investing effort in those goals, and compensate for declines with new strategies or aids. The model helps counselors support clients in keeping meaningful activities rather than giving them up.

References: Berk (Development Through the Lifespan) — Late adulthood: selective optimization with compensation (Baltes)

☐ Approve ☐ Revise ☐ Retire Notes:

An 84-year-old former dancer has lost much of her mobility. She tells her counselor, "I spent years upset about what my body could not do. Now I put my energy into teaching young dancers by video and into my friendships." In Peck's view of late adulthood, which task has she MOST clearly resolved?

A. Ego differentiation, by finding worth in roles beyond her career

Ego differentiation concerns finding worth beyond the work role; her shift centers on physical decline.

B. Ego transcendence, by facing death through her legacy to others

Ego transcendence concerns facing mortality; her statement is about moving beyond bodily limits.

C. Ego integrity, by finding peace with the full course of her life

Integrity is Erikson's broad stage; Peck split it into more specific tasks.

D. Body transcendence, by stressing mental and social powers over loss ✓ **KEY**

She has moved past preoccupation with physical limits to emphasize cognitive and social strengths.

Rationale: Robert Peck expanded Erikson's ego integrity stage into three tasks. Ego differentiation means finding worth beyond the work role, body transcendence means compensating for physical decline by emphasizing cognitive, emotional and social powers, and ego transcendence means facing mortality by investing in a future beyond oneself. This client has moved from body preoccupation to body transcendence.

References: Berk (Development Through the Lifespan) — Late adulthood: Peck's tasks of ego integrity (ego differentiation, body transcendence, ego transcendence)

☐ Approve ☐ Revise ☐ Retire Notes:

A 33-year-old with lupus has joint pain and fatigue. She worries about flares before big events and keeps a symptom diary that her rheumatologist asked for, but her worry fits her medical situation, and she spends little time on health concerns otherwise. Her partner asks whether she has somatic symptom disorder. Which response is MOST accurate?

A. Yes, because she has physical symptoms that disrupt her daily life

Distressing symptoms alone do not meet criteria; excessive thoughts, feelings or behaviors are also required.

B. No, because her worry and actions are in proportion to her illness ✓ **KEY**

Criterion B requires disproportionate thoughts, high health anxiety or excessive time and energy.

C. No, because the diagnosis cannot apply when a medical illness exists

Somatic symptom disorder can occur alongside a diagnosed medical condition.

D. Yes, because keeping a symptom diary is a form of excess health checking

Her diary was requested by her physician and is a reasonable part of managing lupus.

Rationale: Somatic symptom disorder requires distressing somatic symptoms plus excessive thoughts, feelings or behaviors about them: disproportionate thoughts about seriousness, persistently high health anxiety, or excessive time and energy devoted to them, usually lasting more than 6 months. A medical condition does not rule out the diagnosis, but proportionate concern and recommended self-monitoring do not meet Criterion B. Mislabeling realistic coping can invalidate clients with chronic illness.

References: DSM-5-TR — Somatic symptom disorder: Criterion B excessive thoughts, feelings or behaviors

☐ Approve ☐ Revise ☐ Retire Notes:

A 41-year-old engineer who uses a wheelchair says coworkers often call her "so inspiring" for simply coming to work, and a stranger at a café praised her for "getting out." She feels uneasy but wonders whether she is being ungrateful. How should the counselor understand these comments?

A. As kind support she should accept, since the remarks are meant well

Good intent does not remove the message that her ordinary life is surprising.

B. As a sign of her own low self-worth that is shaping how she hears praise

This locates the problem in her rather than in the assumptions behind the comments.

C. As overt discrimination that calls for a formal complaint at her job

The remarks are subtle and well meant; they fit microaggressions rather than overt discrimination.

D. As ableist microaggressions that cast everyday life as heroic ✓ **KEY**

Praising routine activities implies low expectations of people with disabilities.

Rationale: Microaggressions are brief, often unintended messages that communicate slights toward members of marginalized groups. Treating routine activities of a person with a disability as remarkable implies that a full life with a disability is unexpected. Counselors validate the client's unease, help her name the pattern, and support her in choosing whether and how to respond.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling persons with disabilities: ableism and disability microaggressions

☐ Approve ☐ Revise ☐ Retire Notes:

A 70-year-old gay man facing heart surgery has been estranged from his siblings for decades. He says three close friends have been "my real family for forty years" and fears they will be shut out of medical decisions. What should the counselor do FIRST?

A. Urge him to reconcile with his siblings, who are his legal next of kin

This dismisses the supports he has chosen and pushes contact he has not asked for.

B. Honor his chosen family and help him plan for a health care proxy ✓ **KEY**

Recognizing chosen family and helping with advance directives protects his wishes and supports.

C. Explain that hospitals tend to let close friends decide in emergencies

Without legal documents, friends may be excluded from decisions; this reassurance is misleading.

D. Explore whether his distance from his siblings reflects his own shame

This assumes pathology and sidesteps his urgent, practical concern about his care.

Rationale: Many LGBTQ+ older adults rely on chosen family, often because of past rejection by family of origin. These bonds can lack legal standing, so a partner or close friends may be excluded from medical decisions unless documents such as a health care proxy are in place. Counselors affirm chosen family as real family and help clients plan so their wishes are honored.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ+ individuals: older adults and families of choice

☐ Approve ☐ Revise ☐ Retire Notes:

A 32-year-old Black lesbian says she hears racist remarks at LGBTQ+ social events and anti-gay sermons at the Black church where she grew up. She says, "I have to leave part of myself at the door wherever I go." Which approach BEST reflects the Multicultural and Social Justice Counseling Competencies?

A. Address her stress as racial and sexual stress in separate parts of treatment

Splitting her identities misses how they combine to shape her experience in each setting.

B. Ask her to choose which identity matters more so that goals stay in focus

Forcing a ranking of identities repeats the very split she describes as painful.

C. Explore how her intersecting identities shape the stress she faces in each place ✓ **KEY**

An intersectional lens sees her identities, privileges and oppressions as interacting.

D. Encourage her to stop attending both settings so she can avoid more stress

Advising withdrawal cuts her off from communities and supports she may value.

Rationale: The MSJCC build on intersectionality, the idea that identities such as race, sexual orientation and gender combine to shape a person's experiences of privilege and oppression. A client who belongs to more than one marginalized group may face rejection in each community that shares one of her identities. Counselors explore how these identities interact and help the client find or build spaces where she can be whole.

References: MSJCC (Ratts et al., 2016) — Intersectionality of identities; client worldview and counseling relationship domains

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a resettlement agency is designing an intake for adult refugees from several countries. Many arrived after years in camps, and all now face new jobs, schools and language. Which framework BEST guides a full assessment of their stressors?

A. Explore stressors before, during and after migration across their journey ✓ KEY

Premigration, migration and postmigration stressors each add to refugee distress.

B. Focus on the original trauma, since resettlement stress tends to be minor

Postmigration stressors such as poverty and isolation strongly affect refugee well-being.

C. Rate each client's English skill, since language ability predicts all of it

Language matters but is one stressor among many and does not capture trauma or losses.

D. Focus on present symptoms only, since the past cannot be changed in therapy

Ignoring earlier stages misses trauma and losses that shape present distress.

Rationale: Counseling literature on immigrants and refugees assesses stressors across three phases. Premigration stressors include war, persecution and loss; migration stressors include dangerous travel and long stays in camps; postmigration stressors include poverty, discrimination, language barriers and loss of status. Assessing all three phases gives a fuller picture of distress and strengths and guides practical as well as clinical help.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling immigrants and refugees: premigration, migration and postmigration stressors

☐ Approve ☐ Revise ☐ Retire Notes:

A 24-year-old devout Catholic spends up to three hours a day repeating prayers until they feel "perfect" and confesses the same minor thoughts several times a week. Her priest has told her this is not required and urged her to stop, but she fears she is damned if she does. How should the counselor view this pattern?

A. As religious obsessions and compulsions beyond what her faith asks ✓ KEY

Her rituals exceed her faith's norms, cause distress, and take over an hour a day, fitting OCD.

B. As devout practice to respect, since it is part of her religious life

Her own faith leader views the rituals as excessive, so they are not normative religious practice.

C. As a spiritual problem only, to be addressed by her priest, not therapy

The pattern fits a treatable disorder; working with her priest can help but does not replace care.

D. As a delusion, since her fear of damnation is a fixed false belief

Her fear arises within her faith and she can see it as excessive; it is not a delusion.

Rationale: DSM-5-TR notes that OCD themes can be religious, and it advises against an OCD diagnosis for culturally prescribed rituals unless they exceed cultural norms, occur at times others in the culture see as inappropriate, and interfere with functioning. Here the client's rituals take more than an hour a day, cause distress and go beyond what her own priest expects. With her consent, collaborating with her priest can support exposure and response prevention.

References: DSM-5-TR — Obsessive-compulsive disorder: religious themes; culture-related diagnostic issues and differential diagnosis

☐ Approve ☐ Revise ☐ Retire Notes:

A 30-year-old left the tight-knit religious community she was raised in. Her family now limits contact, and her former friends will not speak to her. She says she does not regret leaving but feels lost without the rituals, friendships and sense of purpose she once had. Which focus is BEST for counseling?

A. Celebrate her escape and help her reject the beliefs she was taught

Taking a side against her former faith imposes the counselor's view on her journey.

B. Process her losses and help her build new sources of meaning and support ✓ KEY

Leaving a faith community can bring grief over relationships, identity and meaning that need care.

C. Help her plan a return, since her distress shows she made the wrong choice

Distress over losses does not mean the decision was wrong; she does not regret leaving.

D. Avoid the topic of religion, since it lies outside the scope of counseling

Spiritual and religious concerns are a valid focus of counseling, not outside its scope.

Rationale: Leaving a religious community can bring layered losses: relationships, rituals, identity, a shared worldview and sometimes contact with family. Clients may feel relief and grief at once. Counselors stay neutral about the faith itself, help the client mourn what was lost, and support her in building new communities, values and sources of meaning.

References: Gladding (Counseling: A Comprehensive Profession) — Spirituality in counseling: spiritual and religious issues as a focus of counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor with a strong Christian faith is working with a client who has recently found peace through Buddhist meditation and wants to deepen that practice to manage anxiety. The counselor feels an urge to suggest prayer from her own tradition instead. According to the ACA Code of Ethics, what should the counselor do?

A. Support the client's chosen practice and keep from imposing her own values ✓ KEY

Counselors respect clients' beliefs and avoid imposing their own values on clients.

B. Refer the client to a Buddhist counselor, since their beliefs differ

Referral based only on the counselor's values is discouraged; differing beliefs are not a reason to refer.

C. Share her own faith as one more option, so that the client can compare

Promoting her own faith risks imposing her values on a client who has chosen a path.

D. Tell the client about her beliefs so the client can decide whether to stay

Disclosing her faith shifts the focus to the counselor and is not needed to serve the client.

Rationale: ACA Code A.4.b directs counselors to be aware of their own values, attitudes and beliefs and to avoid imposing them on clients. A.11.b adds that counselors do not refer clients based solely on the counselor's personal values. The counselor supports the client's chosen spiritual practice as a coping resource and seeks supervision if her reactions persist.

References: ACA Code of Ethics — A.4.b Personal Values; A.11.b Values Within Termination and Referral

☐ Approve ☐ Revise ☐ Retire Notes:

A middle school counselor is writing a unit of classroom lessons to build students' readiness for high school and future careers, such as goal setting, self-discipline and belief in their own ability to succeed. Which ASCA resource should the counselor use to select the lesson outcomes?

A. ASCA Student Standards: Mindsets & Behaviors for Student Success ✓ KEY

These standards set the mindsets and behaviors school counseling lessons aim to build.

B. ASCA Ethical Standards for School Counselors and their sections

The ethical standards guide counselor conduct, not student learning outcomes.

C. The ASCA position statement on the role of the school counselor

Position statements describe counselor roles, not lesson outcomes for students.

D. The ASCA program assessment used to rate school counseling plans

The program assessment evaluates the program as a whole, not student outcomes.

Rationale: The ASCA National Model uses the ASCA Student Standards: Mindsets & Behaviors for Student Success to define the knowledge, attitudes and skills students need for college and career readiness. School counselors choose standards across the academic, career and social/emotional domains to set lesson objectives. This keeps classroom lessons tied to measurable student outcomes.

References: ASCA National Model — ASCA Student Standards: Mindsets & Behaviors for Student Success; college and career readiness

☐ Approve ☐ Revise ☐ Retire Notes:

After job shadowing at a clinic and taking a health science elective, a 15-year-old tells her school counselor that she wants "some kind of health care career" but has not chosen a specific job or training path. In Super's exploration stage, which task is she working on?

A. Specifying, by narrowing her preference to one particular occupation

Specifying is choosing a specific occupation, which she has not yet done.

B. Implementing, by finishing training and taking up an entry-level job

Implementing involves completing training and entering work, a later task.

C. Stabilizing, by settling into a chosen job and confirming the choice

Stabilizing belongs to the establishment stage of early adulthood.

D. Crystallizing, by forming a general idea of the field she may pursue ✓ KEY

Crystallizing is forming a tentative, general career preference, which fits her statement.

Rationale: In Super's life-span, life-space theory, the exploration stage includes the tasks of crystallizing, specifying and implementing a career choice. Crystallizing means forming a general preference based on interests, abilities and values, as this student has done. School counselors support it with exploration activities and then help students specify choices and plan the training that leads to them.

References: Sharf (Career Development Theory) — Super's life stages: exploration tasks of crystallization, specification and implementation

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old Army veteran in counseling for PTSD mentions that a community physician recently prescribed alprazolam for his nightmares and anxiety, and that he now takes it most nights with a few beers. What is the counselor's BEST response?

A. Tell him to stop the medication at once, since it is unsafe for PTSD

Advising a sudden stop is outside the counselor's scope and can be dangerous with daily use.

B. Leave medication issues to the physician and focus on trauma work

Ignoring a risky combination fails the client; coordinating care is part of the counselor's role.

C. With consent, share concerns with the prescriber and screen his alcohol use ✓ **KEY**

Guidelines advise against benzodiazepines for PTSD, and mixing them with alcohol adds risk.

D. Suggest he ask for a higher dose so that his nightmares are fully controlled

Recommending doses is outside scope, and benzodiazepines are not advised for PTSD.

Rationale: The VA/DoD clinical practice guideline for PTSD recommends against benzodiazepines, which lack evidence for PTSD, may interfere with trauma-focused therapy and carry risks of dependence. Combined with alcohol they also raise the risk of oversedation and overdose. The counselor does not change medications but, with the client's consent, coordinates with the prescriber and assesses alcohol use.

References: VA/DoD CPG — Management of PTSD and Acute Stress Disorder: recommendation against benzodiazepines

☐ Approve ☐ Revise ☐ Retire Notes:

Treatment Planning

17 items · NCE weight 9% (14 of 160 scored items)

Q108 of 200 · nce-s-trt-086 · Counseling and Helping Relationships · discharge and aftercare planning · medium · In review

A 37-year-old client has recovered from her second major depressive episode after 14 sessions of cognitive behavioral therapy. As discharge nears, she says, "What if the depression sneaks up on me again?" Which aftercare step BEST addresses her concern?

- A. Reassure her that people who finish CBT seldom have another depressive episode afterward
This is inaccurate; depression often recurs, especially after more than one episode, and false reassurance leaves her unprepared.
- B. Keep weekly sessions going without an end date so that any return of symptoms is caught
Open-ended weekly care is not clinically indicated once goals are met and can undercut her confidence in her own skills.
- C. Write a plan with her that lists early warning signs and the steps she will take if they appear ✓ KEY
Preparing for setbacks with a written plan of personal warning signs and coping steps is a core relapse prevention task before termination.
- D. Ask her to contact the agency only if she begins to have thoughts of suicide after discharge
Waiting for suicidal thoughts sets the bar for returning far too high; earlier warning signs should prompt action.

Rationale: In cognitive behavioral therapy, relapse prevention is built into the final phase of treatment. The counselor helps the client anticipate setbacks, identify her own early warning signs and write down the skills and supports she will use, including when to seek a booster session. This turns the fear of recurrence into a concrete plan she can act on.

References: J. Beck (CBT: Basics and Beyond) — Termination and relapse prevention: preparing for setbacks and planning for early warning signs

☐ Approve ☐ Revise ☐ Retire Notes:

Q109 of 200 · nce-s-trt-087 · Counseling and Helping Relationships · discharge and aftercare planning · easy · In review

A 24-year-old client in treatment for anxiety has missed three sessions in a row and has not answered phone calls. Agency policy allows the case to be closed. What should the counselor do before closing the case?

- A. Send a caring outreach letter that invites return and lists referral and crisis resources ✓ KEY
Outreach that leaves the door open and offers other resources avoids abandonment and supports continuity of care.
- B. Close the case without contact, since the client has chosen to stop coming to counseling
Closing with no outreach risks abandonment; the reason for the absences is unknown and the client may still need help.
- C. Keep the case open with weekly calls until the client agrees to come back for a final session
Repeated pursuit can feel intrusive and ignores the client's choice; one clear outreach with resources is the standard.
- D. Ask the client's emergency contact to find out why the client has stopped coming to sessions
Contacting a third party about attendance without consent breaches confidentiality when there is no emergency.

Rationale: When a client stops attending, the counselor should try to reach them, invite a return or a closing session, and offer referrals and crisis resources before the case is closed. The ACA Code asks counselors to provide pretermination counseling and recommend other providers when needed, so that ending services does not amount to abandonment. The outreach attempt and closing summary are documented in the record.

References: ACA Code of Ethics — A.11.c Appropriate Termination · Wiger (Documentation) — Termination documentation: discharge reason, outreach attempts and referrals offered

☐ Approve ☐ Revise ☐ Retire Notes:

While building a safety plan, a 32-year-old client names her older brother as the only person she would call for help in a crisis. She adds that he works nights and seldom answers his phone after 10 p.m., when her urges are strongest. What should the counselor do NEXT?

A. Accept the brother as her contact, since she trusts him and chose him on her own

Her trust matters, but leaving a known barrier unaddressed makes this step likely to fail when she needs it most.

B. Move straight to the step on making her home safer, since this step will not work

Skipping the step gives up on social support; the barrier can be solved by adding contacts who are reachable.

C. Tell her to call 911 instead of family members whenever her urges begin late at night

Emergency services belong in the plan for imminent danger, but they do not replace the social support step.

D. Explore this barrier with her and identify other people she could reach late at night ✓ **KEY**

Safety planning includes checking how likely each step is to be used and problem-solving obstacles, such as an unreachable contact.

Rationale: In the Safety Planning Intervention, the counselor asks how likely the client is to use each step and works through any obstacles. A contact who cannot be reached at the hours of highest risk is a barrier that should be solved, for example by adding other people or a crisis line for those hours. A plan that fits the client's real life is more likely to be used.

References: Stanley-Brown SPI — Assessing the likelihood of using each step and problem-solving barriers to use

☐ Approve ☐ Revise ☐ Retire Notes:

A 66-year-old widower completing a Stanley-Brown safety plan says, "When the dark thoughts start, I walk to the diner and chat with the regulars about baseball. I don't tell them how I feel, but it takes my mind off things." In which step of the safety plan does this strategy BEST belong?

A. People I can ask for help

This step lists people the client tells about the crisis and asks for help; he does not disclose his distress at the diner.

B. Social settings for distraction ✓ **KEY**

Going to a social setting to take his mind off things, without disclosing the crisis, fits the distraction step.

C. Internal coping strategies on my own

Internal strategies are things he does on his own without contacting others, such as a hobby at home.

D. Warning signs that a crisis is coming

Warning signs are the thoughts, moods and situations that signal a crisis, not the actions taken in response.

Rationale: The Stanley-Brown plan moves from warning signs, to internal coping strategies, to people and social settings that provide distraction, to people the client can ask for help, to professionals and crisis services, and finally to making the environment safe. The distraction step uses social contact without disclosing the crisis, while the help step involves telling someone that he is struggling. His diner visits fit the distraction step.

References: Stanley-Brown SPI — Step 3, people and social settings that provide distraction, versus Step 4, people whom I can ask for help

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor-in-training lists this objective for a 45-year-old client with depression: "Counselor will provide cognitive behavioral therapy weekly for 12 weeks." Why is this entry a weak treatment objective?

A. It names a treatment approach that has too little evidence for adults with depression

CBT is a well-supported treatment for depression; the problem lies in how the objective is written.

B. It describes the counselor's intervention, not a measurable change by the client ✓ **KEY**

Objectives state what the client will do or achieve; the counselor's planned service belongs under interventions.

C. It sets a 12-week time frame, which is too long for an objective in a treatment plan

A defined time frame is a strength, and 12 weeks is a reasonable span for this kind of objective.

D. It leaves out the diagnosis, which must be repeated within each treatment objective

The diagnosis appears elsewhere in the plan; objectives do not need to restate it.

Rationale: A treatment plan separates goals, objectives and interventions. Objectives are measurable, client-centered steps, such as a target score or behavior by a set date, while interventions describe what the counselor will do to help the client get there. Writing the counselor's service as an objective leaves no way to judge the client's progress.

References: Wiger (Documentation) — Treatment plans: measurable client objectives distinguished from clinician interventions

☐ Approve ☐ Revise ☐ Retire Notes:

A supervisor reviews this objective for a 16-year-old referred for angry outbursts at school: "Client will gain insight into the causes of his anger by the end of the semester." Which revision BEST makes the objective measurable?

A. Client will develop a much deeper understanding of his anger by the end of the semester

"Deeper understanding" is still an internal state that cannot be observed or counted.

B. Client will feel calmer and more in control at school by the end of the fall semester

Feeling calmer is a worthy aim but is vague and gives no way to observe or measure change.

C. Client will attend all of his weekly counseling sessions through the end of the semester

Attendance is measurable but tracks participation, not a change in the problem behavior.

D. Client will name three anger triggers and use a coping skill for each in session by week 4 ✓ **KEY**

This states observable client behaviors, a number to count and a deadline, so progress can be checked.

Rationale: Measurable objectives use observable action verbs such as name, list, use or reduce, rather than internal states such as understand, gain insight or feel. They also specify how much and by when. Turning "insight" into naming triggers and showing coping skills lets the counselor and client see whether the objective has been met.

References: Wiger (Documentation) — Writing behavioral objectives: observable action verbs, criteria and target dates

☐ Approve ☐ Revise ☐ Retire Notes:

A 29-year-old client with moderate opioid use disorder is stable on buprenorphine, has no medical or psychiatric complications and wants to stay in recovery. However, he shares an apartment with two people who use heroin daily, and he has nowhere else to live. Which assessment dimension MOST supports a more structured setting?

A. Recovery or living environment ✓ KEY

Living with people who use daily, with no other housing, is a serious environmental risk that can justify more structure.

B. Withdrawal potential

He is stable on buprenorphine, so acute withdrawal is not driving the placement decision.

C. Biomedical conditions

The stem states that he has no medical complications.

D. Readiness to change

He wants to stay in recovery, so readiness is a strength here rather than a concern.

Rationale: Placement decisions rest on a multidimensional assessment rather than diagnosis alone. Here every dimension looks favorable except his living environment, where daily exposure to active use and the lack of other housing threaten his recovery. That single dimension can support a more structured option, such as a residential setting or recovery housing combined with continued medication.

References: ASAM Criteria — Multidimensional assessment: recovery/living environment as a basis for level-of-care decisions

☐ Approve ☐ Revise ☐ Retire Notes: _____

On day 12 of a residential substance use program, a 40-year-old client has no withdrawal symptoms, a stable home with a sober partner and strong coping skills, and meets criteria for outpatient care. The program requires every client to complete 30 days. What does level-of-care practice support?

A. Keep her in residential care for all 30 days, since full program completion predicts success

Length of stay should follow the client's progress, not a fixed program length.

B. Discharge her with no follow-up care, since she has done so well in the first two weeks

Moving down a level is appropriate, but ending all care removes needed support early in recovery.

C. Transfer her to outpatient care now, based on reassessment of her current needs and progress ✓ KEY

Ongoing reassessment should move clients along the continuum when they meet criteria for a different level.

D. Extend her stay to 60 days so she can build even stronger coping skills before going home

Extending care she no longer needs is over-placement and disrupts her home and work life without benefit.

Rationale: Level-of-care decisions are outcome-driven rather than time-driven. Clients are reassessed during treatment and moved to the least intensive level that is safe and effective once they meet its criteria, instead of staying for a preset number of days. Continuing care at the next level keeps support in place during the transition.

References: ASAM Criteria — Continued service and transfer criteria: variable length of service based on progress rather than fixed program length

☐ Approve ☐ Revise ☐ Retire Notes: _____

The parents of a 6-year-old girl describe daily defiance, frequent tantrums and hitting her younger brother. Her teacher reports similar behavior at school. Which intervention has the strongest evidence as a first-line approach for this age group?

A. Parent training that teaches praise, clear commands and consistent consequences ✓ KEY

Behavioral parent training programs are the first-line, best-supported treatment for conduct problems in young children.

B. Weekly individual insight-oriented therapy so the girl can explore her feelings

Insight-oriented individual work with a young child lacks the evidence that parent-focused behavioral programs have.

C. A referral for stimulant medication as the first step to bring her behavior down

Medication is not a first-line treatment for conduct problems themselves, and no ADHD diagnosis is described.

D. A peer group with other aggressive children so they can learn from one another

Grouping aggressive children together can reinforce problem behavior and is not the recommended first step.

Rationale: For children aged about 3 to 11 with oppositional or conduct problems, parent training programs have the strongest research base. Parents learn to use positive attention, praise, clear instructions and consistent, non-harsh consequences, which change the patterns that keep the behavior going. Medication is not recommended as routine treatment for conduct problems.

References: NICE guidelines — Antisocial behaviour and conduct disorders in children and young people (CG158): parent training programmes for ages 3 to 11

☐ Approve ☐ Revise ☐ Retire Notes:

A 48-year-old client with generalized anxiety disorder has not improved after psychoeducation and a guided self-help program. Under NICE stepped-care guidance, which pair of psychological treatments is recommended as the high-intensity option?

A. Eye movement desensitization and reprocessing or psychodynamic therapy

Neither EMDR nor psychodynamic therapy is a recommended high-intensity treatment for GAD in this guidance.

B. Individual cognitive behavioral therapy or applied relaxation training ✓ KEY

NICE names individual CBT and applied relaxation as the high-intensity psychological options for GAD.

C. Exposure and response prevention or interpersonal psychotherapy alone

ERP is a treatment for OCD, and IPT is not a recommended high-intensity intervention for GAD.

D. Nondirective supportive counseling or a large psychoeducational group

Psychoeducational groups are a low-intensity step, and supportive counseling is not a recommended GAD treatment.

Rationale: NICE uses stepped care for generalized anxiety disorder: identification and psychoeducation first, then low-intensity interventions such as guided self-help or psychoeducational groups, and then high-intensity options. At the high-intensity step, the recommended psychological treatments are individual CBT or applied relaxation, with medication as an alternative based on client preference. Treatment intensity is stepped up only when a lower step has not helped.

References: NICE guidelines — Generalised anxiety disorder and panic disorder in adults (CG113): stepped care, step 3 high-intensity CBT or applied relaxation

☐ Approve ☐ Revise ☐ Retire Notes:

A 51-year-old client with mild alcohol use disorder and no withdrawal history wants to cut down to two drinks on weekend evenings. Her counselor believes abstinence would be safer. During treatment planning, what is the MOST collaborative way to proceed?

A. Write abstinence as the goal, since the counselor's expertise should guide what goes in the plan

Imposing a goal the client does not hold tends to raise resistance and lower engagement.

B. Delay planning until the client accepts that abstinence is the only realistic goal for her

Withholding planning until she agrees is confrontational and risks losing her from treatment.

C. Adopt her reduction goal, share concerns openly, and agree to review progress on it together ✓ **KEY**

Starting from the client's goal, giving honest input and planning to reassess respects her autonomy while keeping safety in view.

D. Let her write the plan entirely on her own so the counselor does not influence her choices

Collaboration means shared work; the counselor should still offer expertise and help shape measurable steps.

Rationale: Collaborative planning treats the client as the decision maker about her own goals while the counselor contributes information and concerns. In motivational interviewing, the counselor can offer a view with permission, then negotiate a plan that starts where the client is and includes a way to check how it is working. If moderation proves difficult, the goal can be revisited together.

References: Miller & Rollnick (MI) — Planning: negotiating change goals, offering information with permission and honoring client autonomy

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor uses the Outcome Rating Scale (ORS) at the start and the Session Rating Scale (SRS) at the end of each session. A 22-year-old client's ORS scores are flat, and after session three his SRS score drops well below his earlier ratings. What should the counselor do?

A. Invite his feedback about what did not fit in the session and adjust the approach with him ✓ **KEY**

The SRS taps the alliance; a drop calls for an open conversation about goals, methods and fit.

B. Increase session frequency, since the drop in the SRS shows that his symptoms are worsening

The SRS measures the client's view of the session and alliance, not his symptoms.

C. Set the SRS aside and focus on the ORS, since only the ORS is a true measure of outcome

Alliance feedback is useful because alliance problems predict dropout and poor outcomes.

D. Replace both scales with a longer symptom inventory that gives more reliable scores

Swapping measures avoids the signal; the drop should be explored with the client directly.

Rationale: The ORS is a brief measure of how the client is doing personally, interpersonally, socially and overall, while the SRS asks how the session went in terms of the relationship, goals and topics, approach and overall fit. A falling SRS score is an early alliance warning. Discussing it openly and adjusting the work is the purpose of this kind of client feedback system.

References: Lambert (Outcome Monitoring) — Client feedback systems: Outcome Rating Scale and Session Rating Scale, alliance feedback

☐ Approve ☐ Revise ☐ Retire Notes:

A community mental health center plans to adopt the Outcome Questionnaire-45 (OQ-45) to track client progress. Which three areas of functioning does the OQ-45 assess?

A. Mood, anxiety and substance use

These are symptom areas but not the three OQ-45 subscales.

B. Insight, motivation and alliance

These are process variables, not the outcome domains the OQ-45 measures.

C. Cognition, memory and attention

These are neuropsychological domains, not OQ-45 subscales.

D. Symptoms, relationships, social role ✓ KEY

The OQ-45 has Symptom Distress, Interpersonal Relations and Social Role subscales.

Rationale: The OQ-45 is a 45-item self-report measure designed for repeated use during therapy. Its three subscales are Symptom Distress, Interpersonal Relations and Social Role performance, and higher scores mean more distress. Its sensitivity to change makes it useful for session-by-session monitoring.

References: Lambert (Outcome Monitoring) — Outcome Questionnaire-45: Symptom Distress, Interpersonal Relations and Social Role subscales

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client stopped gambling six weeks ago. He is working hard to stay away from casinos and to handle urges as they come. According to the Transtheoretical Model, which processes of change are MOST emphasized at his stage?

A. Consciousness raising and dramatic relief to build his awareness of the problem

These experiential processes are most used early, in moving from precontemplation to contemplation.

B. Stimulus control and counterconditioning to manage cues and replace the behavior ✓ KEY

In the action stage, behavioral processes such as stimulus control and counterconditioning are emphasized.

C. Environmental reevaluation to weigh how his gambling has affected his family

Environmental reevaluation is linked mainly with contemplation, when people are weighing change.

D. Self-reevaluation to picture who he would be without gambling in his life

Self-reevaluation is most prominent in the shift from contemplation to preparation.

Rationale: Having changed his behavior within the past six months, he is in the action stage. Research on the Transtheoretical Model links the action and maintenance stages with behavioral processes: stimulus control, counterconditioning, contingency management and helping relationships. Experiential processes such as consciousness raising, dramatic relief and reevaluation are emphasized in earlier stages.

References: Transtheoretical Model — Integration of stages and processes of change: behavioral processes in action and maintenance

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor at a HIPAA-covered agency is sending a client's assessment and treatment summary to the psychiatrist who will now manage the client's medication. A colleague says HIPAA's minimum necessary standard requires removing most details first. Which statement is accurate?

A. Minimum necessary applies to every disclosure, so only a diagnosis and medication may be sent

The standard has exceptions, and disclosures to a provider for treatment are one of them.

B. Minimum necessary bars any sharing of mental health records unless a court orders the release

HIPAA permits sharing for treatment; a court order is not needed for coordination between providers.

C. Minimum necessary does not apply to disclosures made to another provider for treatment purposes ✓ **KEY**

The Privacy Rule exempts disclosures to, or requests by, a health care provider for treatment from this standard.

D. Minimum necessary applies only to paper records, so electronic summaries can be sent in full

The standard covers protected health information in any form, paper or electronic.

Rationale: Under the HIPAA Privacy Rule, the minimum necessary standard limits uses and disclosures for purposes such as payment and operations, but it does not apply to disclosures to a health care provider for treatment. Providers can share what is clinically relevant to coordinate care. Psychotherapy notes kept separate from the record still require the client's authorization, and state law or agency policy may set stricter rules.

References: HIPAA — Privacy Rule minimum necessary standard (45 CFR 164.502(b)): exception for disclosures to a provider for treatment

☐ Approve ☐ Revise ☐ Retire Notes:

In a trial of a group treatment for depression, the treatment group's mean posttest score is 21 and the control group's mean is 27 on a measure where higher scores mean more severe symptoms. The pooled standard deviation is 8. What is Cohen's d, and how large is the effect?

A. 0.75, between medium and large ✓ **KEY**

$d = (27 - 21) / 8 = 0.75$, which falls between the medium (0.5) and large (0.8) benchmarks.

B. 1.33, a very large effect

This divides the SD by the mean difference (8 / 6) instead of the reverse.

C. 6.0, a very large effect

Six is the raw mean difference; d divides it by the standard deviation.

D. 0.29, a small effect

This divides the difference by a mean (6 / 21) instead of by the standard deviation.

Rationale: Cohen's d is the difference between two group means divided by the pooled standard deviation, which expresses the effect in standard deviation units. Here $(27 - 21) / 8 = 0.75$. By Cohen's conventions (about 0.2 small, 0.5 medium, 0.8 large), this is a medium-to-large effect.

References: Erford (Research and Evaluation in Counseling) — Effect size: computing Cohen's d from group means and pooled standard deviation

☐ Approve ☐ Revise ☐ Retire Notes:

A small pilot study had a power of .60 to detect a medium effect. It found $p = .24$, and the authors concluded that the new intervention does not work. If the intervention truly has a medium effect, what error may they have made, and how likely was it?

A. A Type I error, with probability .05

A Type I error is rejecting a true null; these authors did not reject the null.

B. A Type II error, with probability .60

.60 is the power, the chance of correctly detecting the effect, not the chance of missing it.

C. A Type I error, with probability .40

The error is failing to detect a real effect, which is Type II, not Type I.

D. A Type II error, with probability .40 ✓ KEY

Missing a real effect is a Type II error; $\beta = 1 - \text{power} = 1 - .60 = .40$.

Rationale: A Type II error is failing to reject a false null hypothesis, that is, missing an effect that really exists. Its probability, β , equals 1 minus power, so a study with power of .60 has a .40 chance of missing a true medium effect. Underpowered outcome studies can therefore lead to the mistaken conclusion that an effective treatment does not work.

References: Erford (Research and Evaluation in Counseling) — Hypothesis testing: Type II error, β and statistical power ($1 - \beta$)

☐ Approve ☐ Revise ☐ Retire Notes:

Counseling Skills and Interventions

60 items · NCE weight 30% (48 of 160 scored items)

Q125 of 200 · nce-s-cou-301 · Counseling and Helping Relationships · reality therapy and choice theory · medium · In review

A mother asks a reality therapist to "make" her 16-year-old stop gaming late and start studying. She says she has tried yelling, grounding him and taking away his phone, and nothing works. Which axiom of Glasser's choice theory should guide the counselor's response?

- A.** Past events with the son determine the present, so they must be resolved first
Glasser held that the past shapes us but that needs can be met only in the present, so therapy focuses on now.
- B.** Firm rewards and punishments from parents are what teens need to choose well
Rewards and punishments are external control psychology, which choice theory says harms relationships.
- C.** The only person whose behavior we can control is our own, not her son's ✓ **KEY**
This is Glasser's first axiom; the mother can change her own choices, which may in turn improve the relationship.
- D.** A teen's feelings drive his acts, so the counselor should change his mood first
Choice theory holds that we change feelings indirectly by changing acting and thinking, not the reverse.

Rationale: Glasser's ten axioms of choice theory begin with the idea that the only behavior we can control is our own; all we can give another person is information. Efforts to force others through criticism, threats or punishment reflect external control psychology and damage relationships. A reality therapist would help the mother evaluate what she is doing and choose behaviors more likely to connect with her son.

References: Corey (Theory & Practice) — Reality Therapy: choice theory and Glasser's axioms; external control psychology

☐ Approve ☐ Revise ☐ Retire Notes:

Q126 of 200 · nce-s-cou-302 · Counseling and Helping Relationships · reality therapy and choice theory · hard · In review

A 45-year-old accountant seeks help for tension headaches and worry that have lasted two years. He wants to talk mainly about his symptoms and a harsh father who died a decade ago. Following Glasser's choice theory, where would a reality therapist MOST likely focus first?

- A.** On which important relationship in his life now is unsatisfying ✓ **KEY**
Glasser held that long-lasting problems are relationship problems and that the problem relationship is part of present life.
- B.** On a detailed history of each headache and the worry that comes with it
Reality therapists give little attention to symptoms, seeing them as chosen ways of coping with unmet needs.
- C.** On working through his anger at his late father to gain full insight
Insight into the past is not the aim; Glasser focused on present behavior and present relationships.
- D.** On teaching relaxation so the headaches ease before other work begins
Symptom relief skills are not the reality therapy focus; the aim is meeting needs through better choices.

Rationale: Among Glasser's axioms are that all long-lasting psychological problems are relationship problems and that the problem relationship is always part of the person's present life. Reality therapists therefore spend little time on symptoms or the distant past. They look for the current relationship in which the client's needs for love and belonging or power are not being met.

References: Corey (Theory & Practice) — Reality Therapy: choice theory axioms; focus on present relationships rather than symptoms or the past

☐ Approve ☐ Revise ☐ Retire Notes:

A client says, "I've been so depressed since the layoff." Her reality therapist replies, "It sounds like you've been depressing a lot lately. What are you doing while you're depressing?" What is the main purpose of describing her mood with an active verb?

A. To show the client that her low mood is not real and will pass soon

Glasser does not deny the pain; he reframes it as part of a behavior the person is choosing.

B. To confront her gently so she feels guilty about staying in her mood

Reality therapy avoids blame and criticism; the verb form is meant to empower, not shame.

C. To find the early experience that first taught her to feel this way

Searching for early causes is not part of reality therapy, which stays with present choices.

D. To stress that total behavior is chosen and can be changed by acting ✓ **KEY**

Glasser used verbs such as "depressing" to show that the feeling is part of chosen total behavior that she can change.

Rationale: Choice theory holds that all behavior is total behavior (acting, thinking, feeling and physiology) and that it is chosen. Glasser described feelings with verbs such as "depressing" or "angering" to underline that people are active in what they experience. Because acting and thinking are the parts we directly control, changing them is the route to feeling better.

References: Corey (Theory & Practice) — Reality Therapy: total behavior and the use of verbs (e.g., "depressing")

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old Black woman who uses a wheelchair describes being passed over for promotion. Her feminist counselor helps her explore how race, disability, gender and class combine to shape this experience in ways that a focus on gender alone would miss. Which concept is the counselor drawing on?

A. Androgyny

Androgyny refers to a flexible blend of gender-typed traits, not to overlapping social identities.

B. Intersectionality ✓ **KEY**

Intersectionality examines how multiple identities and systems of oppression interact to shape a person's experience.

C. Gender-role analysis

Gender-role analysis focuses on gender messages, while this counselor looks at several identities together.

D. Relabeling

Relabeling changes the language used for a trait or symptom; it does not analyze overlapping identities.

Rationale: Contemporary feminist therapy is multicultural and intersectional. It recognizes that gender is experienced together with race, class, disability, sexual orientation and other identities, and that oppression operates across these at once. Exploring these intersections helps the client locate distress in its full social context rather than in personal failing.

References: Corey (Theory & Practice) — Feminist Therapy: multicultural and intersectional perspectives · Sue & Sue (Counseling the Culturally Diverse) — Multiple identities and intersectionality in counseling

☐ Approve ☐ Revise ☐ Retire Notes:

A graduate student says she feels "crazy" for being upset by a professor's repeated sexist jokes. Her feminist counselor briefly shares that she, too, has faced sexism at work and found it hard to name at first. What is the PRIMARY purpose of this disclosure in feminist therapy?

A. To shift the session toward the counselor's own story so she can model coping

Feminist self-disclosure is brief and for the client's benefit; it does not shift the focus to the counselor.

B. To lessen the power gap and show that her reaction is a shared, valid one ✓ **KEY**

Purposeful disclosure equalizes the relationship and normalizes the client's reaction as a response to sexism.

C. To persuade the client to file a formal complaint against the professor

Feminist counselors respect client choice; disclosure is not used to push a particular action.

D. To set up transference so the client can work through early relationships

Fostering transference is a psychodynamic aim, not the purpose of feminist self-disclosure.

Rationale: Feminist therapists use self-disclosure purposefully to reduce the power differential, model openness and help clients see that their reactions are understandable responses to social conditions. Disclosures are brief, relevant and offered for the client's benefit. Used this way, they support the egalitarian relationship at the heart of feminist practice.

References: Corey (Theory & Practice) — Feminist Therapy: techniques (therapist self-disclosure) and the egalitarian relationship

☐ Approve ☐ Revise ☐ Retire Notes:

Which therapy goal is MOST consistent with a feminist approach to counseling?

A. Helping the client adjust to the roles that are expected of her in her setting

Feminist therapy questions adjustment to restrictive roles rather than making it the goal.

B. Resolving unconscious conflicts that took root in early childhood experiences

Resolving unconscious conflict is a psychoanalytic goal, not the defining aim of feminist therapy.

C. Eliminating symptoms quickly so the client can return to her usual routine

Symptom relief alone ignores the social context that feminist therapy places at the center.

D. Empowering the client and fostering change in oppressive social conditions ✓ **KEY**

Feminist therapy aims at personal empowerment and at social transformation, not adjustment to the status quo.

Rationale: Feminist therapy seeks both individual and social change. Goals include empowerment, self-definition, valuing the client's own perspective and challenging oppressive conditions, often through advocacy or social action. Because "the personal is political," adjustment to unjust norms is not considered a healthy outcome.

References: Corey (Theory & Practice) — Feminist Therapy: therapeutic goals (empowerment and social transformation)

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old farmworker comes to a rural clinic two days after a flood destroyed his home. Using Roberts' seven-stage model, the counselor has completed a lethality assessment and established rapport. What should the counselor do NEXT?

A. Identify the major problems, including the event that set off the crisis ✓ KEY

Stage 3 of Roberts' model is identifying the major problems and the precipitating event.

B. Develop and put into action a step-by-step plan for the coming week

Formulating an action plan is stage 6, after problems, feelings and alternatives are explored.

C. Brainstorm alternatives and coping options the client has not yet tried

Generating and exploring alternatives is stage 5 and comes after problems and feelings are addressed.

D. Schedule follow-up contact to check on how he is doing in a few weeks

Follow-up is the seventh and final stage of Roberts' model.

Rationale: Roberts' seven stages are: (1) assess lethality and psychosocial needs, (2) establish rapport, (3) identify major problems or precipitants, (4) deal with feelings and emotions, (5) generate and explore alternatives, (6) develop and carry out an action plan, and (7) follow up. Defining the problems that triggered the crisis gives focus to the work on feelings and options that follows.

References: Gladding (Counseling: A Comprehensive Profession) — Crisis counseling: Roberts' seven-stage crisis intervention model

☐ Approve ☐ Revise ☐ Retire Notes:

A crisis counselor trained in James and Gilliland's six-step model is working with a teen who ran away after a family fight. A supervisor asks when assessment should take place in this model. Which answer is accurate?

A. Only at the start, during the step of defining the problem

Assessment is not limited to the first step; the client's state can change at any point.

B. Only at the end, when the counselor obtains a commitment

Waiting until the end would miss changes in safety and functioning during the session.

C. Continuously, across all six steps of the intervention ✓ KEY

James and Gilliland place assessment as an ongoing process that runs through every step.

D. At set intervals, after each pair of the six steps is done

The model does not schedule assessment at fixed points; it is ongoing throughout.

Rationale: James and Gilliland's model has three listening steps (define the problem, ensure safety, provide support) and three acting steps (examine alternatives, make plans, obtain commitment). Assessment is not a separate step; it is pervasive and continuous throughout. The counselor keeps judging the client's emotional, behavioral and cognitive functioning to decide how directive to be.

References: James & Gilliland (Crisis Intervention Strategies) — Six-step model of crisis intervention: assessing throughout the process

☐ Approve ☐ Revise ☐ Retire Notes:

Modern crisis theory traces back to a psychiatrist's study of grief reactions among survivors and bereaved relatives after the 1942 Cocoanut Grove nightclub fire in Boston. Who conducted this study?

A. Albert Roberts

Roberts developed the seven-stage crisis intervention model decades later.

B. Elisabeth Kübler-Ross

Kübler-Ross described stages of dying in the late 1960s, not grief after the fire.

C. Gerald Caplan

Caplan built on this work to develop preventive psychiatry and crisis theory.

D. Erich Lindemann ✓ KEY

Lindemann's study of grief after the Cocoanut Grove fire is a founding work of crisis theory.

Rationale: Erich Lindemann studied the acute grief of people affected by the Cocoanut Grove fire and described normal grief reactions and how brief help could aid recovery. Gerald Caplan, his colleague, extended this into a theory of crisis and preventive community mental health. Together their work is the foundation of modern crisis intervention.

References: James & Gilliland (Crisis Intervention Strategies) — History of crisis intervention: Lindemann and Caplan

☐ Approve ☐ Revise ☐ Retire Notes:

During a lifestyle assessment, a 30-year-old client says, "People will take advantage of you if you let them," and later, "Unless I'm the best at something, I'm nothing." Her Adlerian counselor notes these as faulty convictions to explore. What are these called?

A. Basic mistakes ✓ KEY

Mosak described basic mistakes, such as overgeneralization and denial of one's worth, as self-defeating lifestyle beliefs.

B. Feelings of inferiority

Inferiority feelings are normal and motivate striving; these statements are specific faulty beliefs.

C. Family constellation

The family constellation is the family system and birth order explored in the assessment, not the beliefs.

D. Social interest

Social interest is the sense of belonging and concern for others, which these beliefs work against.

Rationale: Adlerians summarize the lifestyle assessment by identifying basic mistakes: self-defeating convictions in the client's private logic. Mosak grouped them as overgeneralizations, false or impossible goals of security, misperceptions of life and its demands, minimization or denial of one's worth, and faulty values. Naming them helps the client gain insight and choose new beliefs.

References: Corey (Theory & Practice) — Adlerian Therapy: lifestyle assessment and basic mistakes (Mosak)

☐ Approve ☐ Revise ☐ Retire Notes:

An Adlerian counselor sees progress when a withdrawn retiree begins tutoring neighborhood children and checks in on an ill friend. Adler saw this growing sense of belonging and concern for others as the key measure of mental health. What did he call it?

A. Fictional finalism

Fictional finalism is an imagined central goal that guides behavior, not concern for others.

B. Style of life

Style of life is a person's characteristic way of moving toward goals, healthy or not.

C. Social interest ✓ KEY

Social interest (Gemeinschaftsgefühl) is the sense of community feeling that Adler saw as the core of health.

D. Striving for superiority

Striving for superiority is the drive to overcome inferiority, which can be useful or useless.

Rationale: Adler's concept of social interest, or Gemeinschaftsgefühl, refers to a sense of belonging to and contributing to the human community. He viewed it as the main criterion of psychological health: the more social interest a person shows, the healthier they are. Adlerian counseling aims to increase it, so new contributions to others are a sign of progress.

References: Corey (Theory & Practice) — Adlerian Therapy: social interest and community feeling

☐ Approve ☐ Revise ☐ Retire Notes:

A counseling student is asked to describe the outcome Carl Rogers hoped clients would move toward, which he called becoming a "fully functioning person." Which description is MOST accurate?

A. Living by clear rules learned from parents and keeping strong control of feelings

Living by introjected rules reflects conditions of worth, which Rogers saw as blocking growth.

B. Growing more open to experience and trusting one's own inner, organismic valuing ✓ KEY

Rogers described the fully functioning person as open to experience, living in the moment and self-trusting.

C. Gaining insight into repressed memories and resolving conflicts from childhood

Insight into repressed material is a psychoanalytic aim, not Rogers' description of growth.

D. Disputing irrational beliefs and adopting a more rational philosophy of living

Disputing irrational beliefs is the REBT route to change, not the person-centered ideal.

Rationale: Rogers described the fully functioning person as open to experience, living existentially in the present, trusting their own organism and feeling free to choose. As clients receive congruence, unconditional positive regard and empathy, they rely less on conditions of worth and more on their own valuing process. Person-centered therapy trusts this actualizing tendency rather than directing the client toward set goals.

References: Corey (Theory & Practice) — Person-Centered Therapy: the fully functioning person and goals of therapy

☐ Approve ☐ Revise ☐ Retire Notes:

A 58-year-old former dancer with a progressive spinal condition can no longer perform or teach. She says her life now has meaning because of the courage and humor with which she faces her illness. In Frankl's logotherapy, which source of meaning is she drawing on?

A. Attitudinal values ✓ KEY

Frankl held that the stance a person takes toward unavoidable suffering is itself a source of meaning.

B. Creative values

Creative values come from what one gives through work or deeds, which she can no longer do.

C. Experiential values

Experiential values come from what one receives, such as love, nature or art, not from facing suffering.

D. Will to power

Frankl contrasted his will to meaning with Adler's will to power; it is not a source of meaning.

Rationale: Frankl described three main paths to meaning: creative values (what we give to the world), experiential values (what we take from the world, such as love or beauty) and attitudinal values (the stance we take toward suffering we cannot change). Attitudinal values mean that meaning is possible even when the other two paths are closed. This idea comes from his experience in the concentration camps.

References: Corey (Theory & Practice) — Existential Therapy: Frankl's logotherapy and the search for meaning

☐ Approve ☐ Revise ☐ Retire Notes:

A 50-year-old client says he feels a nagging regret that he never pursued music and has spent decades doing what others expected. His existential counselor sees this feeling as meaningful rather than as a symptom. How would the counselor MOST likely understand it?

A. As neurotic guilt from breaking his parents' rules, which needs to be reduced

Guilt over breaking rules is not the concern here; his regret is about unlived potential.

B. As a sign of depression that should be treated before any deeper exploration

Existential counselors do not first reduce such feelings to a disorder; they explore what they mean.

C. As a cognitive distortion that can be disputed with evidence from his life

Treating the regret as a distortion to dispute reflects a cognitive approach, not an existential one.

D. As existential guilt over unlived potential, a call toward a more authentic life ✓ KEY

Existential guilt arises from failing to realize one's possibilities and can motivate authentic choices.

Rationale: Existential writers such as May and Yalom distinguish existential guilt from neurotic guilt. Existential guilt is the awareness that one has not lived up to one's potential or has lived inauthentically, often by following others' expectations. Rather than removing it, the counselor helps the client hear it as a call to take responsibility and make more authentic choices now.

References: Corey (Theory & Practice) — Existential Therapy: authenticity, existential anxiety and existential guilt

☐ Approve ☐ Revise ☐ Retire Notes:

Each time her gestalt counselor asks how she feels about her mother's illness, a 27-year-old client makes a joke, talks in vague generalities or changes the subject to her job. Which contact boundary disturbance does this BEST illustrate?

A. Confluence

Confluence is a blurring of the boundary between self and others, not a turning away from contact.

B. Retroflection

Retroflection is doing to oneself what one would like to do to others, such as turning anger inward.

C. Deflection ✓ KEY

Deflection avoids sustained contact through humor, vagueness, abstraction or changing the subject.

D. Introjection

Introjection is swallowing others' values whole without examining them.

Rationale: Gestalt therapy describes several ways people interrupt contact: introjection, projection, retroflection, deflection and confluence. Deflection is a way of diluting contact by being vague, overly polite, joking or shifting focus so that experience does not fully land. The counselor helps the client notice the pattern and stay with what is being avoided.

References: Corey (Theory & Practice) — Gestalt Therapy: contact and resistances to contact (deflection)

☐ Approve ☐ Revise ☐ Retire Notes:

A client describes an inner voice that says, "You should be working harder, you lazy slob," and another that whines, "I'll do it tomorrow, I'm too tired." Her gestalt counselor has her give each voice a chair and speak as it. Perls called these two parts of the self:

A. Id and superego

The id and superego are Freudian structures, not the gestalt terms for this split.

B. Top dog and underdog ✓ KEY

Perls named the demanding, critical part the top dog and the passive, excuse-making part the underdog.

C. Parent and Child

Parent and Child are ego states in transactional analysis, not Perls' terms.

D. Figure and ground

Figure and ground describe what stands out in awareness, not two conflicting parts of the self.

Rationale: Perls described a common internal split between the top dog, which is righteous, demanding and full of "shoulds," and the underdog, which resists through excuses, helplessness and delay. The two-chair technique lets the client voice each side fully. Bringing both into awareness helps the client integrate them rather than stay stuck in the conflict.

References: Corey (Theory & Practice) — Gestalt Therapy: top dog and underdog; the empty-chair (two-chair) technique

☐ Approve ☐ Revise ☐ Retire Notes:

A client insists, "I must never make a mistake at work." His REBT counselor asks, "Where has holding on to that demand gotten you? How has it helped you at work and with your stress?" Which type of disputing is the counselor using?

A. Empirical disputing

Empirical disputing asks for evidence that the belief is true, such as "Where is the proof?"

B. Pragmatic disputing ✓ KEY

Pragmatic (functional) disputing asks whether the belief helps or hurts the client in reaching goals.

C. Logical disputing

Logical disputing asks whether the demand follows logically from a preference.

D. Imaginal disputing

Rational emotive imagery works with images and emotions, not questions about usefulness.

Rationale: Ellis and Dryden describe three main kinds of disputing. Empirical disputing asks whether there is evidence for the belief, logical disputing asks whether it makes sense, and pragmatic or functional disputing asks whether holding the belief helps the client. Showing a client that a demand leads to stress and poorer work often motivates change when other arguments do not.

References: Ellis & Dryden (REBT) — Disputing irrational beliefs: empirical, logical and pragmatic arguments

☐ Approve ☐ Revise ☐ Retire Notes:

After her fiancé ended their engagement, a 31-year-old client stays in bed, tells herself she is unlovable and says she wants her REBT counselor to "make the pain go away completely." What is the MOST appropriate emotional goal from an REBT view?

A. Feeling sad and disappointed rather than depressed and self-condemning ✓ KEY

REBT aims to replace unhealthy negative emotions with healthy ones such as sadness, not to remove all pain.

B. Feeling calm and indifferent about the end of the relationship by now

Indifference to a real loss is not a REBT goal; caring about the loss is healthy.

C. Feeling cheerful and positive by focusing only on what she gained

Forced positivity ignores the loss; REBT does not aim to replace pain with cheer.

D. Feeling angry at her fiancé so her energy is turned outward instead

Swapping one unhealthy emotion for another does not reflect the REBT model of healthy emotions.

Rationale: REBT distinguishes unhealthy negative emotions, such as depression, anxiety, shame and rage, from healthy negative emotions, such as sadness, concern, disappointment and annoyance. Healthy negative emotions fit real losses and come from rational preferences. The goal is not to feel nothing, but to feel appropriately bad without self-damning beliefs.

References: Ellis & Dryden (REBT) — Healthy versus unhealthy negative emotions

☐ Approve ☐ Revise ☐ Retire Notes:

A single parent of three lists debts, conflict with an ex-partner and a child's school troubles, and says she does not know where to start. Her solution-focused counselor helps her pick one small, doable step for the week. Which SFBT assumption BEST explains this choice?

A. Each problem must be fully understood before any solution can be tried

SFBT holds that solutions need not be tied to a full understanding of problems.

B. Large, lasting change requires first working through all the past causes

SFBT is future-focused and does not work through past causes as a first step.

C. The counselor, as expert, should pick the goal that matters most of all

SFBT views the client as the expert who chooses goals; the counselor does not decide.

D. A small change can set off a ripple effect leading to larger changes ✓ **KEY**

De Shazer held that small changes build on each other and can spread to other areas of life.

Rationale: Solution-focused brief therapy assumes that change is constant and that a small change in one area can lead to larger changes elsewhere. Small, concrete goals are easier to reach, build hope and confidence, and can shift how the whole system works. The client, as the expert on their own life, chooses which step to try.

References: Corey (Theory & Practice) — Solution-Focused Brief Therapy: key assumptions (small change leads to bigger change)

☐ Approve ☐ Revise ☐ Retire Notes:

A client who sees himself as "a coward" describes, step by step, how he spoke up for a coworker who was being mocked. His narrative counselor then asks, "What does it say about what you value that you took that stand?" In White's terms, this second question explores the:

A. Landscape of action

The landscape of action covers events, sequence and time, which the first account already described.

B. Dominant story

The dominant story is the problem-saturated "coward" account, not the new identity meaning.

C. Landscape of identity ✓ **KEY**

Landscape of identity questions ask what an event shows about values, hopes, intentions and character.

D. Externalizing conversation

Externalizing separates the person from the problem; this question builds meaning from an event.

Rationale: In re-authoring conversations, Michael White moves between two landscapes. Landscape of action questions explore events, circumstances and sequence, while landscape of identity questions explore what those events reveal about the person's values, purposes and commitments. Moving back and forth thickens an alternative story that can stand against the dominant one.

References: White (Maps of Narrative Practice) — Ch. 2, re-authoring conversations: landscape of action and landscape of identity

☐ Approve ☐ Revise ☐ Retire Notes:

Which pair of therapists is MOST closely associated with founding narrative therapy and writing Narrative Means to Therapeutic Ends?

A. Michael White and David Epston ✓ KEY

White (Australia) and Epston (New Zealand) developed narrative therapy and co-wrote this 1990 book.

B. Steve de Shazer and Insoo Kim Berg

De Shazer and Berg developed solution-focused brief therapy.

C. Salvador Minuchin and Jay Haley

Minuchin is linked to structural and Haley to strategic family therapy.

D. Albert Ellis and Aaron Beck

Ellis founded REBT and Beck founded cognitive therapy.

Rationale: Michael White and David Epston developed narrative therapy in the 1980s, drawing on ideas about stories, power and knowledge. Their book *Narrative Means to Therapeutic Ends* introduced practices such as externalizing the problem and therapeutic letters. White later described his methods in *Maps of Narrative Practice*.

References: Corey (Theory & Practice) — Narrative Therapy: key figures (White and Epston) · White (Maps of Narrative Practice) — Introduction: development of narrative practice

☐ Approve ☐ Revise ☐ Retire Notes:

A 40-year-old client who gambles online says, "I've got to do something about this. I can't keep going on like this." The counselor wants to recognize and reflect the change talk. Using the DARN-CAT framework, which type of change talk is this?

A. Desire

Desire is stated as wanting or wishing, such as "I want to stop."

B. Ability

Ability statements express capacity, such as "I could cut back."

C. Activation

Activation signals readiness to move, such as "I'm willing to try," which is mobilizing talk.

D. Need ✓ KEY

Need statements express urgency or necessity, such as "I've got to" or "I can't keep going."

Rationale: Miller and Rollnick divide change talk into preparatory talk (Desire, Ability, Reasons, Need) and mobilizing talk (Commitment, Activation, Taking steps). Need language expresses an imperative for change without saying why or what will be done. Hearing and reflecting each form helps the counselor strengthen the client's own motivation.

References: Miller & Rollnick (MI) — Evoking: preparatory change talk (DARN), including need

☐ Approve ☐ Revise ☐ Retire Notes:

A client who smokes says calmly, "I've tried to quit five times, and it never sticks. Honestly, smoking is the one thing that gets me through my shifts." The client remains warm and engaged with the counselor. In MI terms, these statements are BEST described as:

A. Discord in the working alliance

Discord is friction in the relationship, such as arguing with the counselor, which is absent here.

B. Sustain talk favoring the status quo ✓ KEY

Sustain talk is the client's own speech in favor of not changing, such as low ability or reasons to keep smoking.

C. Preparatory change talk about ability

Ability change talk expresses capacity to change; these statements argue against it.

D. Denial requiring direct confrontation

MI does not label ambivalence as denial, and confronting it tends to increase sustain talk.

Rationale: In current MI, what was once called resistance is split into sustain talk and discord. Sustain talk is the client's speech about the target behavior that favors staying the same, while discord is tension in the relationship between client and counselor. Both are normal parts of ambivalence, and the counselor responds with reflections rather than argument.

References: Miller & Rollnick (MI) — Responding to sustain talk and discord

☐ Approve ☐ Revise ☐ Retire Notes:

A client in DBT wants to turn down a close friend's request to borrow her car again, but her main goal is to keep the friendship warm. Her therapist suggests she stay gentle, show interest in the friend's needs, validate them and use an easy manner. Which interpersonal effectiveness skill set is this?

A. DEAR MAN

DEAR MAN targets objectives effectiveness, getting what one wants or saying no firmly.

B. FAST

FAST targets self-respect effectiveness: being fair, not over-apologizing, sticking to values, truthful.

C. GIVE ✓ KEY

GIVE (gentle, interested, validate, easy manner) targets relationship effectiveness.

D. TIPP

TIPP is a distress tolerance skill for lowering extreme arousal, not an interpersonal skill.

Rationale: Linehan's interpersonal effectiveness module teaches three skill sets matched to a person's priority. DEAR MAN serves objectives effectiveness, GIVE serves relationship effectiveness and FAST serves self-respect effectiveness. When keeping the relationship matters most, the client leans on GIVE while still saying no.

References: Linehan (DBT Skills Training Manual) — Interpersonal effectiveness: GIVE skills for relationship effectiveness

☐ Approve ☐ Revise ☐ Retire Notes:

A 22-year-old in a comprehensive DBT program calls her individual therapist on a Friday night when urges to self-harm rise, and the therapist briefly coaches her through a distress tolerance skill. What is the MAIN function of this phone coaching mode?

A. Helping her use skills in the real-life moments where she needs them ✓ KEY

Phone coaching serves generalization, helping clients apply skills in daily life when urges arise.

B. Giving her an open line for long talks to process the whole week

DBT coaching calls are brief and skills-focused, not open-ended therapy sessions.

C. Helping the therapist stay motivated and avoid burnout with clients

Supporting therapist motivation is the role of the DBT consultation team, not phone coaching.

D. Teaching her the four skills modules for the first time in sequence

New skills are taught in skills training groups; coaching helps apply skills already learned.

Rationale: Comprehensive DBT has four modes: individual therapy, group skills training, between-session phone coaching and a therapist consultation team. Skills group teaches skills, while phone coaching helps the client generalize them to the real situations where they are needed. Calls are brief and focused on which skill to use now.

References: Linehan (DBT Skills Training Manual) — Overview of DBT: treatment modes and the function of between-session skills coaching

☐ Approve ☐ Revise ☐ Retire Notes:

Which clinician adapted Carl Rogers' ideas to work with children, set out eight basic principles of nondirective play therapy and wrote the case study Dibs in Search of Self?

A. Melanie Klein

Klein used play as a substitute for free association within a psychoanalytic approach.

B. Anna Freud

Anna Freud used play to build a relationship within child psychoanalysis.

C. Virginia Axline ✓ KEY

Axline founded nondirective play therapy based on Rogers and wrote Dibs in Search of Self.

D. Terry Kottman

Kottman developed Adlerian play therapy, not nondirective play therapy.

Rationale: Virginia Axline, a student of Carl Rogers, applied person-centered ideas to children in nondirective play therapy. Her eight principles include building a warm relationship, accepting the child as they are and letting the child lead. Garry Landreth later developed this work into child-centered play therapy.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: play therapy approaches (Axline's nondirective play therapy)

☐ Approve ☐ Revise ☐ Retire Notes:

In a child-centered play therapy session, a 7-year-old finishes a clay figure after several tries, holds it up and asks, "Isn't it good?" Which counselor response BEST fits this approach?

A. "That's really good! You're a great artist."

Evaluative praise teaches the child to rely on adult approval rather than self-evaluation.

B. "Next time you could try adding some more color."

Suggestions direct the play and imply the work is not yet good enough.

C. "Why did you decide to make that figure today?"

Why questions ask the child to explain and can feel evaluative.

D. "You kept at it until it looked how you wanted." ✓ **KEY**

This esteem-building response notes the child's effort and lets the child judge the work.

Rationale: Child-centered play therapists avoid praise and judgment because these place the source of evaluation in the adult. Esteem-building responses describe the child's effort, persistence or decisions, such as "You figured it out." These help the child develop an internal sense of competence and self-direction.

References: Gladding (Counseling: A Comprehensive Profession) — Counseling children: child-centered play therapy responses (esteem building versus praise)

☐ Approve ☐ Revise ☐ Retire Notes:

A community agency becoming trauma-informed hires staff with their own lived experience of trauma and recovery to share hope and mutual support with clients. Which of SAMHSA's six trauma-informed principles does this step MOST directly reflect?

A. Safety

Safety concerns physical and emotional security, not the use of people with lived experience.

B. Peer support ✓ **KEY**

Peer support uses people with lived experience to build trust, hope and recovery.

C. Trustworthiness and transparency

This principle concerns clear, open decisions and consistent boundaries, not peer staff.

D. Cultural, historical and gender issues

This principle addresses bias, culture and historical trauma, not lived-experience roles.

Rationale: SAMHSA's trauma-informed approach rests on six principles: safety; trustworthiness and transparency; peer support; collaboration and mutuality; empowerment, voice and choice; and cultural, historical and gender issues. Peer support draws on people with lived experience of trauma to promote hope, trust and recovery. Many agencies put this principle into practice by hiring peer specialists.

References: Gladding (Counseling: A Comprehensive Profession) — Trauma and crisis counseling: principles of trauma-informed care (SAMHSA)

☐ Approve ☐ Revise ☐ Retire Notes:

A veteran with PTSD repeatedly recounts his worst combat memory aloud in session, in the present tense, and listens to a recording of it at home. He also slowly approaches crowded stores he has been avoiding. Which evidence-based treatment is he receiving?

A. Prolonged exposure ✓ **KEY**

Prolonged exposure combines repeated imaginal exposure with recorded homework and in vivo exposure.

B. Cognitive processing therapy

CPT centers on stuck points and written work rather than repeated imaginal and in vivo exposure.

C. Eye movement desensitization

EMDR pairs brief memory focus with bilateral stimulation and does not use recordings or in vivo work.

D. Seeking Safety

Seeking Safety is present-focused coping work that does not use detailed exposure to the memory.

Rationale: Prolonged exposure, developed by Edna Foa, reduces PTSD by helping clients face trauma memories and avoided situations until fear lessens. Its core parts are imaginal exposure (recounting the memory, often recorded for home practice), in vivo exposure to safe but avoided situations, and processing. VA/DoD guidelines strongly recommend it along with CPT and EMDR.

References: VA/DoD CPG — PTSD: recommended trauma-focused psychotherapies (prolonged exposure, CPT, EMDR)

☐ Approve ☐ Revise ☐ Retire Notes:

A nursing student will not ask her preceptor questions. In CBT she identifies the thought, "If I ask for help, people will see I'm not competent." Her counselor notes that this conditional rule links a deeper view of herself to her daily thoughts. In J. Beck's model, this is:

A. An automatic thought

Automatic thoughts are quick, situation-specific thoughts, not general if-then rules.

B. An intermediate belief ✓ **KEY**

Intermediate beliefs are attitudes, rules and assumptions, often in if-then form, that link core beliefs to thoughts.

C. A core belief

A core belief is an absolute, global view such as "I am incompetent," not a conditional rule.

D. A cognitive distortion

Distortions are errors in thinking patterns, not a level in the belief structure.

Rationale: J. Beck describes three levels of cognition. Core beliefs are global and absolute; intermediate beliefs are the attitudes, rules and assumptions that grow from them, often stated in if-then form; and automatic thoughts are the situation-specific thoughts that pop up. Identifying intermediate beliefs helps explain why the same kinds of automatic thoughts keep recurring.

References: J. Beck (CBT: Basics and Beyond) — Identifying and modifying intermediate beliefs (rules, attitudes and assumptions)

☐ Approve ☐ Revise ☐ Retire Notes:

A 62-year-old client pauses while describing her first week of retirement. The counselor nods, says "Mm-hm," and then repeats one key word she used: "Restless?" In Ivey's basic listening sequence, what is the counselor doing?

A. Paraphrasing

A paraphrase restates the main content of what the client said in fresh words; a single word and a nod do not.

B. Encouraging ✓ KEY

Encouragers such as head nods, "uh-huh" and repeating a key word prompt the client to keep talking and expand.

C. Summarizing

Summarizing pulls together several themes over a longer stretch of the session, not one word.

D. Interpreting

Interpreting offers a new frame of reference; repeating the client's own word adds no new meaning.

Rationale: Ivey places encouraging in the basic listening sequence along with questioning, paraphrasing, reflecting feeling and summarizing. Encouragers include nods, minimal verbal prompts and the repetition of key words, and they invite the client to elaborate on what matters to them. Repeating a key word often leads the client to explore the feeling or meaning behind it.

References: Ivey et al. (Intentional Interviewing) — Encouraging, paraphrasing and summarizing: encouragers and key-word restatement

☐ Approve ☐ Revise ☐ Retire Notes:

During an intake, a client has spoken at length and in general terms about "always" feeling tired. The counselor now needs to know whether the client has started any new medication in the past month. Which statement BEST describes the role of a closed question here?

A. It should be avoided, since closed questions damage rapport in intake sessions

Closed questions are a normal part of intake; overuse, not use, is what harms rapport.

B. It should be rephrased as a "why" question so the client explains their habits

"Why" questions can sound accusatory and would not efficiently obtain the specific fact needed.

C. It should replace open questions for the rest of the interview to save time

Relying only on closed questions puts the counselor in charge of content and limits the client's story.

D. It fits well, because closed questions gather specific facts and narrow focus ✓ KEY

Closed questions are useful for obtaining specific information and focusing a broad or wandering account.

Rationale: Open questions invite clients to elaborate, while closed questions can usually be answered in a few words. Closed questions are appropriate for gathering specific facts, such as recent medication changes, and for bringing focus to a diffuse account. The skill lies in balancing the two so that the client keeps ownership of the story.

References: Ivey et al. (Intentional Interviewing) — Questions: uses of open and closed questions

☐ Approve ☐ Revise ☐ Retire Notes:

A supervisor reviewing a trainee's recordings notices that clients often mention family and faith, yet sessions keep returning to work stress. The trainee responds warmly whenever work comes up and gives brief, flat replies to everything else. Which concept from Ivey's microskills BEST explains this pattern?

A. Selective attention, since clients tend to talk about what the counselor attends to ✓ KEY

Ivey notes that counselors reinforce certain topics by attending to them, and clients then talk more about those topics.

B. Resistance, since the clients are avoiding the material that matters most to them

The clients do raise family and faith; the narrowing comes from the trainee's responses, not client avoidance.

C. Transference, since the clients are repeating early relationship patterns in session

Nothing suggests the clients are reliving past relationships with the trainee.

D. Mirroring, since the trainee is matching the clients' body language and pace well

Mirroring nonverbal behavior builds rapport; it does not explain why the topics narrow.

Rationale: Ivey describes selective attention as the tendency of counselors to listen and respond more to some topics than others, often without awareness. Because clients follow the counselor's lead, they end up talking mostly about what the counselor reinforces. Reviewing recordings helps trainees notice what they attend to and what they ignore.

References: Ivey et al. (Intentional Interviewing) — Attending behavior: selective attention and verbal tracking

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor says to a young adult client, "It sounds like moving back home has left you feeling both grateful and a little trapped. Am I hearing that right?" What is the main purpose of the final question?

A. To shift the session toward a new topic now that the feeling has been named

The question invites the client to stay with the reflection, not to move on to new material.

B. To test whether the client is being honest about how they feel at home

The question checks the counselor's accuracy, not the client's honesty.

C. To check the accuracy of the reflection and invite the client to correct it ✓ KEY

A checkout (perception check) lets the client confirm, refine or correct the counselor's understanding.

D. To signal that the counselor is unsure and wants the client to lead instead

The checkout is a deliberate part of good reflection, not a sign of counselor uncertainty.

Rationale: Ivey recommends ending many paraphrases and reflections with a checkout, such as "Is that close?" or "Did I get that right?" The checkout keeps the reflection tentative, gives the client power to correct the counselor and often leads to a more precise description of the experience. It also signals that the client, not the counselor, is the expert on their feelings.

References: Ivey et al. (Intentional Interviewing) — Reflecting feelings and paraphrasing: the checkout (perception check)

☐ Approve ☐ Revise ☐ Retire Notes:

A client says calmly, "I was a little irritated when my roommate ate my leftovers again." The counselor responds, "You were furious with him." What is the MAIN weakness of this reflection?

A. It overstates the intensity of the feeling the client expressed ✓ **KEY**

"Furious" is far stronger than "a little irritated," so the reflection misses the client's actual level of feeling.

B. It reflects a feeling rather than the content of what happened

Reflecting feeling is a legitimate skill; the problem is the inaccurate intensity, not the choice to reflect affect.

C. It uses the past tense instead of focusing on the present moment

Past-tense reflection is acceptable when the client describes a past event.

D. It repeats the client's own words too closely to add anything

The counselor substituted a new and much stronger word, so this is not parroting.

Rationale: An accurate reflection of feeling matches both the type and the intensity of the emotion the client expresses. Overshooting the intensity can leave the client feeling misunderstood or pushed, while undershooting can minimize the experience. A better response might be "It bothered you that it happened again."

References: Ivey et al. (Intentional Interviewing) — Reflecting feelings: matching the type and intensity of emotion

☐ Approve ☐ Revise ☐ Retire Notes:

A practicum student wants to use supportive confrontation with a client whose words and actions seem to conflict. According to Ivey's model, what should the student do FIRST?

A. Name the contradiction directly so the client cannot avoid looking at it

Pointing out the discrepancy is the second step and should follow careful listening.

B. Listen and observe closely to identify the mixed messages and conflicts ✓ **KEY**

Supportive confrontation begins by listening, observing and clearly identifying the discrepancy.

C. Rate the client's response on the Client Change Scale for the session

Evaluating change with the Client Change Scale comes after the discrepancy has been pointed out.

D. Offer an interpretation of the unconscious reasons behind the conflict

Interpretation is a separate influencing skill and is not the first step of confrontation.

Rationale: Ivey describes supportive confrontation as a sequence. The counselor first listens and observes to identify incongruities, mixed messages and conflicts; next points them out and helps the client work through them in a nonjudgmental way; and then evaluates the client's response, for example with the Client Change Scale. Starting with listening keeps the confrontation accurate and grounded in the relationship.

References: Ivey et al. (Intentional Interviewing) — Confrontation: the three steps of supportive confrontation

☐ Approve ☐ Revise ☐ Retire Notes:

A client has been describing a conflict with a coworker. Which counselor statement shows the HIGHEST level of immediacy?

A. "Last month you also had trouble speaking up with your brother."

This links past events and stays in the past tense, so it has little immediacy.

B. "Right now, as you talk about him, I notice your voice getting quiet." ✓ KEY

Present-tense attention to what is happening between counselor and client in the moment is immediacy.

C. "Next week you could try telling him how his comments affect you."

This is a future-oriented suggestion, not a here-and-now response.

D. "Your coworker sounds like he has been difficult for a long time."

This comments on a third party outside the session and stays in the past.

Rationale: Immediacy refers to the counselor's present-tense, here-and-now responses about what is happening in the session or between counselor and client. Statements about the past or future have less immediacy than those that name what is occurring right now. Used well, immediacy brings hidden feelings and relational patterns into the room where they can be explored.

References: Ivey et al. (Intentional Interviewing) — Immediacy: past, present and future tense in counselor responses

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor in an employee assistance program offers each client up to six sessions. When is it BEST to begin discussing the end of counseling with a new client?

A. At the fifth session, so the client is not distracted earlier

Waiting until the next-to-last session leaves little time to plan and may feel abrupt.

B. Only after the client reports that the main goal has been met

With a fixed session limit, the end date does not depend on when goals are reached.

C. In the first session, as part of setting goals and expectations ✓ KEY

In time-limited work, raising the ending at the start shapes goals and helps clients use each session well.

D. When the counselor notices the client becoming too dependent

Termination planning should not wait for a problem; in brief work it is built in from the outset.

Rationale: In brief and time-limited counseling, the ending is known from the start, so it should be discussed during informed consent and goal setting. This lets the client and counselor choose realistic goals, track progress and prepare for the end together. Raising termination early also reduces the chance that the final session feels like an abrupt loss.

References: Gladding (Counseling: A Comprehensive Profession) — Termination in counseling: timing and preparation for ending

☐ Approve ☐ Revise ☐ Retire Notes:

In the next-to-last session of CBT for depression, a 41-year-old client says, "You fixed me. I don't know what I'll do without you telling me what to do." Which counselor response BEST supports the gains lasting after counseling ends?

A. Review the specific skills the client used and credit them for the change ✓ KEY

Helping clients attribute progress to their own efforts builds self-efficacy and supports self-therapy after ending.

B. Accept the thanks graciously and add more sessions to ease the transition

Extending sessions because of dependence reinforces the belief that the client cannot cope alone.

C. Explain that relapse is unlikely now that the depression has fully lifted

Depression often recurs; relapse prevention prepares clients for setbacks rather than denying them.

D. Suggest the client call the counselor whenever a hard decision comes up

This keeps the counselor in charge of decisions and works against the goal of client independence.

Rationale: J. Beck describes preparing for termination from the start of CBT, with the aim of clients becoming their own therapists. Near the end, the counselor reviews which skills the client used and helps them attribute improvement to their own work rather than to the counselor. This attribution strengthens self-efficacy and supports relapse prevention and booster planning.

References: J. Beck (CBT: Basics and Beyond) — Termination and relapse prevention: attributing progress to the client

☐ Approve ☐ Revise ☐ Retire Notes:

A community clinic finds that many new clients drop out after one or two sessions without saying why. The director asks counselors for one change most likely to reduce this premature termination. Which change is BEST supported?

A. Assigning homework from the first session so that clients feel more invested

Homework can help later, but early demands without shared understanding can increase dropout.

B. Scheduling first sessions less often so clients who stay are more motivated

Longer gaps early in counseling weaken engagement and tend to increase dropout.

C. Moving clients who miss a session to a waitlist until they recommit to care

This penalizes clients who are already disengaging and makes premature termination more likely.

D. Clarifying expectations about counseling and agreeing on goals at the start ✓ KEY

Role induction and early agreement on goals and methods address mismatched expectations, a key cause of dropout.

Rationale: Premature termination is often linked to a mismatch between what clients expect and what counseling offers, along with a weak early alliance. Preparing clients for counseling (role induction), discussing how long it may take and agreeing on goals and tasks early reduce dropout. Monitoring the alliance and progress in early sessions also helps counselors notice clients at risk of leaving.

References: Gladding (Counseling: A Comprehensive Profession) — Termination in counseling: premature termination and how to reduce it

☐ Approve ☐ Revise ☐ Retire Notes:

A school counselor leads a parent-teacher committee planning a new homework policy. One parent objects to every proposal with a different reason each week. The counselor later learns this parent's child was recently suspended and the parent fears the policy will single the child out. What group dynamic is operating?

A. Scapegoating, in which the group blames one member for its lack of progress

Scapegoating is the group turning on one member; here one member is blocking the group.

B. A hidden agenda, an unstated personal goal that steers a member's behavior ✓ KEY

The parent's unspoken concern about the child is a hidden agenda that drives the repeated objections.

C. Subgrouping, in which several members form an alliance against the group

Subgrouping involves an alliance of several members, not one member acting alone.

D. Groupthink, in which members suppress dissent to reach quick agreement

Groupthink suppresses disagreement; this parent is expressing a great deal of it.

Rationale: A hidden agenda is a goal or concern a member holds but does not state, which shapes how that member acts in the group. Hidden agendas are common in task groups and can stall progress because the stated arguments are not the real issue. Leaders help by attending to process and creating safe ways for members to voice underlying concerns.

References: Gladding (Counseling: A Comprehensive Profession) — Groups in counseling: group dynamics and hidden agendas

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor is forming an eight-week group for new parents adjusting after the birth of their first child. She decides to include only members who share this life transition. According to Yalom, what is the main advantage of this homogeneous composition?

A. Deeper long-term character change through exposure to many conflict styles

Yalom links deep interpersonal change to heterogeneous, long-term groups, not homogeneous ones.

B. Less need for screening, since shared problems make members interchangeable

Screening is still needed; a shared concern does not make members alike in readiness or fit.

C. Faster cohesion and mutual support, which suits a brief group's aims well ✓ KEY

Homogeneous groups gel quickly, offer immediate support and suit short-term, focused goals.

D. More conflict in early sessions, which speeds up the group's working stage

Homogeneous groups tend to have less conflict, not more.

Rationale: Yalom notes that homogeneous groups become cohesive quickly, provide immediate support, have better attendance and less conflict, and produce faster symptom relief. Their drawback is that they may stay at a more superficial level. Heterogeneous composition is preferred for long-term interpersonal groups that aim at deeper change.

References: Yalom & Leszcz (Group Psychotherapy) — Composition of therapy groups: homogeneous versus heterogeneous groups

☐ Approve ☐ Revise ☐ Retire Notes:

In a partial hospitalization program, a counselor leads a long-term group for adults with severe, long-standing personality and mood problems. The aim is in-depth personality change and remediation of serious dysfunction. Under the ASGW classification, which type of group is this?

A. Task and work group

Task groups exist to complete a work product, such as a committee goal.

B. Psychoeducation group

Psychoeducation groups teach skills and information, often to prevent problems.

C. Counseling group

Counseling groups address normal-range personal and interpersonal problems of living.

D. Psychotherapy group ✓ KEY

Psychotherapy groups treat serious, long-standing problems and aim at in-depth personality change.

Rationale: ASGW describes four group work specializations: task and work, psychoeducation, counseling and psychotherapy. Psychotherapy groups serve members with serious or chronic psychological problems and aim at reconstructive, in-depth change, often in clinical settings. Counseling groups, by contrast, address problems of living for members who function in the normal range.

References: Gladding (Counseling: A Comprehensive Profession) — Group work: ASGW types of groups (psychotherapy groups)

☐ Approve ☐ Revise ☐ Retire Notes:

In an assertiveness group, a 26-year-old member watches another member calmly decline an unfair request in a role-play. The next week, she reports using the same words and tone with her landlord. Which of Yalom's therapeutic factors does this BEST illustrate?

A. Imitative behavior ✓ KEY

Adopting behaviors modeled by other members or the leader is imitative behavior.

B. Universality

Universality is the relief of learning that others share one's problems.

C. Catharsis

Catharsis is the release of strong emotion, not the copying of a skill.

D. Altruism

Altruism is the benefit of helping other members, not learning from them.

Rationale: Yalom lists imitative behavior among the therapeutic factors of groups. Members learn by watching how others, including the leader, handle situations and then try those behaviors themselves. Even when imitation is brief, it can help members experiment with new ways of acting.

References: Yalom & Leszcz (Group Psychotherapy) — Ch. 1: The therapeutic factors, imitative behavior

☐ Approve ☐ Revise ☐ Retire Notes:

In a long-term interpersonal group, several members tell Kofi that his habit of joking whenever someone becomes emotional leaves them feeling brushed off. Kofi is surprised and says, "I thought I was helping." Which therapeutic factor is MOST directly at work?

A. Instillation of hope from watching other members improve

Hope comes from seeing others get better; Kofi is learning how he affects people.

B. Imparting information through direct advice and teaching

Members are describing their reactions to him, not offering advice or instruction.

C. Interpersonal learning from feedback on his effect on others ✓ KEY

Learning how one is seen and how one's behavior lands on others is interpersonal learning.

D. Existential factors as he faces full responsibility for his life

Existential factors involve life's givens, such as death and ultimate responsibility.

Rationale: Yalom considers interpersonal learning one of the most powerful therapeutic factors in long-term groups. Through here-and-now feedback, members discover how their behavior affects others and how they are perceived, which they often cannot see on their own. They can then try new ways of relating within the safety of the group.

References: Yalom & Leszcz (Group Psychotherapy) — Interpersonal learning: feedback and the group as social microcosm

☐ Approve ☐ Revise ☐ Retire Notes:

In the first sessions of a new outpatient group, the leader often asks members to respond to one another rather than to her, thanks members who give honest feedback and models nonjudgmental acceptance herself. Which of Yalom's basic tasks of the group therapist is she MAINLY carrying out?

A. Creating and maintaining the group by selecting members and preventing dropout

This task covers forming the group and keeping it intact, such as screening and handling absences.

B. Culture building, shaping the norms that let the group act as a helping agent ✓ KEY

Shaping norms such as member-to-member interaction and honest feedback, explicitly and by modeling, is culture building.

C. Process illumination, explaining the meaning of an interaction just completed

Process illumination reflects on an interaction after it occurs; she is mainly setting norms.

D. Interpretation of transference, linking members' reactions to their families

Yalom's group therapist emphasizes here-and-now norms, and she is not interpreting family patterns.

Rationale: Yalom describes three basic tasks of the group therapist: creating and maintaining the group, culture building, and activating and illuminating the here-and-now. In culture building, the leader shapes norms such as self-disclosure, direct interaction among members and nonjudgmental acceptance, acting both as a technical expert and as a model-setting participant. These norms make the group itself the main agent of change.

References: Yalom & Leszcz (Group Psychotherapy) — The therapist: basic tasks, culture building and norm shaping

☐ Approve ☐ Revise ☐ Retire Notes:

Super's Archway Model shows personal factors such as needs, values and aptitudes in one pillar and societal factors such as family, school and the labor market in the other, joined by an arch of developmental stages. What sits in the keystone of the arch?

A. The labor market, which sets the jobs available to the person

The labor market is part of the societal pillar, not the keystone.

B. Career maturity, the readiness to make decisions at each age

Career maturity is a separate construct in Super's theory, not the keystone of the archway.

C. Interests, which link a person's values with available jobs

Interests sit in the personal pillar alongside needs, values and aptitudes.

D. The self, which integrates personal and societal influences ✓ KEY

Super placed the self, the person as decision maker and self-concept, in the keystone that unites both pillars.

Rationale: Super's Archway Model is a visual summary of the factors in career development. One pillar holds personal characteristics such as needs, values, interests and aptitudes, and the other holds societal influences such as the economy, community, school, family and labor market. The arch represents developmental stages and role self-concepts, and the keystone is the self, which brings these influences together in career decisions.

References: Sharf (Career Development Theory) — Super's life-span theory: the Archway Model

☐ Approve ☐ Revise ☐ Retire Notes:

As a child, Mateo spent many happy weekends at his aunt's bakery, where the smell of bread came to mean warmth and family. Now 19, he feels drawn to culinary school without ever having been praised for cooking. In Krumboltz's social learning theory, this influence is BEST described as:

A. an associative learning experience pairing the setting with positive feelings ✓ KEY

Associative learning links a previously neutral stimulus, here the bakery, with positive emotional experiences.

B. an instrumental learning experience shaped by rewards for his own cooking

Instrumental learning follows consequences of one's own actions; he was never rewarded for cooking.

C. a genetic endowment, an inborn special ability that limits career choice

Genetic endowment refers to inherited qualities and abilities, not emotional associations.

D. a task-approach skill he uses to gather information and make a decision

Task-approach skills are decision-making and work habits, not emotional associations.

Rationale: Krumboltz identified four influences on career decision making: genetic endowment and special abilities, environmental conditions and events, learning experiences and task-approach skills. Learning experiences are instrumental, when behavior is shaped by its consequences, or associative, when a neutral stimulus becomes paired with positive or negative experiences. Mateo's attraction comes from pairing the bakery with warmth, an associative experience.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory: instrumental and associative learning experiences

☐ Approve ☐ Revise ☐ Retire Notes:

A 15-year-old had little interest in computer science until a summer coding camp, where she built a working app and learned that coding jobs pay well and help people. Her interest is now strong. According to the interest model in social cognitive career theory (SCCT), what MOST directly produced this interest?

A. Her Holland code, which fixed her interests before she attended the camp
SCCT treats interests as learned and changeable, not set in advance by a personality type.

B. Her career maturity, which rose as she moved through the growth stage
Career maturity is Super's construct and does not explain this sudden rise in interest.

C. Rising self-efficacy and positive outcome expectations for the field ✓ **KEY**
SCCT holds that interests grow where people feel capable and expect valued outcomes.

D. Chance events alone, which shape interests apart from beliefs and skills
The camp mattered because it changed her beliefs about her ability and the work's rewards.

Rationale: In Lent, Brown and Hackett's social cognitive career theory, interests develop from self-efficacy beliefs and outcome expectations. When learning experiences lead people to feel capable at an activity and to expect valued results, interest grows, which then shapes goals and actions. This makes interests a target for intervention through mastery experiences and accurate information.

References: Sharf (Career Development Theory) — Social cognitive career theory: the interest model (self-efficacy and outcome expectations)

☐ Approve ☐ Revise ☐ Retire Notes:

A 34-year-old client describes herself as ambitious, persuasive and energetic. She enjoys leading teams, selling ideas and taking business risks, and dislikes detailed scientific work. Which Holland type is MOST dominant for her?

A. Social
Social types prefer helping, teaching and caring for others more than persuading or selling.

B. Enterprising ✓ **KEY**
Enterprising types like leading, persuading, selling and taking risks to reach organizational goals.

C. Conventional
Conventional types prefer orderly, detailed work with data and clear procedures.

D. Investigative
Investigative types enjoy scientific, analytical work, which she dislikes.

Rationale: Holland's six types are Realistic, Investigative, Artistic, Social, Enterprising and Conventional. Enterprising people tend to be ambitious, sociable and persuasive and are drawn to leading, managing and selling. Investigative work sits opposite Enterprising on the hexagon, which fits her dislike of scientific tasks.

References: Sharf (Career Development Theory) — Holland's theory of types: the Enterprising type

☐ Approve ☐ Revise ☐ Retire Notes:

A college junior refuses to consider any business major, saying, "I'm just not a numbers person. I've always been bad at math." Her transcripts show solid math grades. In Krumboltz's social learning theory, her statement is BEST described as:

A. a self-observation generalization that may be inaccurate ✓ KEY

Self-observation generalizations are learned self-appraisals of ability or interest, which can be mistaken.

B. an environmental condition that blocks her from the field

Environmental conditions are outside events such as the job market, not her beliefs about herself.

C. a genetic endowment that sets a ceiling on her math skills

Her good grades contradict an inborn limit; the barrier is her learned belief.

D. a task-approach skill she uses to rule out career options

Task-approach skills are decision-making and work skills, not self-evaluations.

Rationale: In Krumboltz's theory, learning experiences produce self-observation generalizations, which are overt or covert statements people make about their own abilities, interests and values. These generalizations guide career choices but can be inaccurate or overgeneralized. Counselors help clients examine such beliefs against evidence and test them through new learning experiences.

References: Sharf (Career Development Theory) — Krumboltz's social learning theory: self-observation generalizations

☐ Approve ☐ Revise ☐ Retire Notes:

Q176 of 200 · nce-s-cou-352 · Human Growth and Development · human development theories (Erikson, Piaget, Kohlberg) · medium · In review

An 81-year-old retired violinist has given up teaching and touring to focus on a few favorite pieces. She practices them more often than before, and she plays slower passages just before fast ones so the fast ones seem quicker by contrast. Which developmental model BEST explains her approach?

A. Baltes's selective optimization with compensation ✓ KEY

She selects fewer goals, optimizes them with practice and compensates for losses with new strategies.

B. Cumming and Henry's disengagement theory of aging

Disengagement describes mutual withdrawal from society, not active strategies to maintain skill.

C. Erikson's stage of generativity versus stagnation

Generativity concerns guiding the next generation in midlife, which is not the focus here.

D. Levinson's model of the midlife transition period

Levinson's midlife transition concerns reappraisal around age 40, not late-life adaptation.

Rationale: Baltes proposed that successful aging involves selective optimization with compensation. Older adults select a smaller set of personally valued goals, optimize their resources and practice in those areas, and compensate for losses with new means of reaching the same ends. The model views late life as a time of active adaptation rather than simple decline.

References: Berk (Development Through the Lifespan) — Late adulthood: selective optimization with compensation (Baltes)

☐ Approve ☐ Revise ☐ Retire Notes:

A 9-month-old watches a parent hide a toy under a blue cloth and finds it several times. The parent then hides it under a red cloth while the baby watches, and the baby searches under the blue cloth again. According to Piaget, what does this error indicate?

- A. The infant has no sense that hidden objects continue to exist
Searching for the toy at all shows that some object permanence is already present.
- B. The infant has a vision problem and needs to be screened soon
The A-not-B error is typical at this age and does not suggest a sensory problem.
- C. Object permanence is emerging but is not yet fully developed ✓ KEY
Searching for hidden objects but at the old location (A-not-B) marks incomplete object permanence.
- D. The infant has reached mental representation and pretend play
Mental representation emerges near the end of the sensorimotor stage, around 18 to 24 months.

Rationale: In Piaget's sensorimotor stage, infants of about 8 to 12 months begin to search for hidden objects, showing emerging object permanence. They often make the A-not-B search error, looking where the object was previously found rather than where they saw it hidden last. This error fades as object permanence becomes more complete in later sensorimotor substages.

References: Berk (Development Through the Lifespan) — Piaget's sensorimotor stage: object permanence and the A-not-B search error

☐ Approve ☐ Revise ☐ Retire Notes:

A 52-year-old electrician has begun training apprentices and volunteering with a youth program. He says, "I want to leave something behind that helps the next generation." Which of Erikson's psychosocial stages does this concern reflect?

- A. Intimacy versus isolation
This early adult stage centers on forming close, committed relationships.
- B. Integrity versus despair
This late-life stage involves reviewing one's life and accepting it as meaningful.
- C. Industry versus inferiority
This school-age stage concerns competence at tasks and comparisons with peers.
- D. Generativity versus stagnation ✓ KEY
Midlife generativity centers on guiding and contributing to the next generation.

Rationale: Erikson described generativity versus stagnation as the central task of middle adulthood. Generativity involves caring for and guiding the next generation through parenting, mentoring, teaching or community contribution. Adults who do not find such outlets may feel stagnant or self-absorbed.

References: Berk (Development Through the Lifespan) — Middle adulthood: Erikson's generativity versus stagnation

☐ Approve ☐ Revise ☐ Retire Notes:

A 10-month-old cries and crawls after his father when he leaves the room, and when the father returns the baby explores the playroom while checking back with him. In Bowlby's phases of attachment development, this infant is MOST likely in which phase?

A. Preattachment

In this phase, from birth to about 6 weeks, infants do not yet object to being left with strangers.

B. Clear-cut attachment ✓ KEY

From about 6 to 8 months, infants show separation anxiety and use the caregiver as a secure base.

C. Attachment in the making

Infants in this phase respond differently to familiar people but do not yet protest separation.

D. Reciprocal relationship

This later phase, from about 18 to 24 months, involves language and negotiating with the caregiver.

Rationale: Bowlby described four phases: preattachment, attachment in the making, clear-cut attachment and formation of a reciprocal relationship. In the clear-cut phase, beginning around 6 to 8 months, infants show separation anxiety and use the familiar caregiver as a secure base for exploration. As language and representation grow, toddlers enter the reciprocal phase and can negotiate with caregivers about comings and goings.

References: Berk (Development Through the Lifespan) — Bowlby's ethological theory: phases of attachment (clear-cut attachment)

☐ Approve ☐ Revise ☐ Retire Notes:

In the Adult Attachment Interview, a father describes his parents as "great, totally normal, very loving." When asked for specific memories, he says he cannot recall any, then mentions being sent to his room alone whenever he cried. He adds that childhood "doesn't really affect who you are." How is this interview MOST likely classified?

A. Dismissing ✓ KEY

Idealized but unsupported descriptions, poor recall and downplaying attachment's importance mark a dismissing interview.

B. Preoccupied

Preoccupied interviews are long, entangled and angry or vague, not brief and idealizing.

C. Autonomous

Autonomous interviews are coherent and balanced, with specific memories that support general descriptions.

D. Unresolved

Unresolved interviews show lapses in reasoning when the speaker discusses loss or trauma.

Rationale: The Adult Attachment Interview classifies adults by the coherence of their account of childhood. Dismissing adults idealize their parents in general terms, cannot supply supporting memories or offer memories that contradict the ideal, and minimize the importance of attachment. Their infants are more likely to show avoidant attachment in the Strange Situation.

References: Berk (Development Through the Lifespan) — Attachment: continuity across generations and the Adult Attachment Interview

☐ Approve ☐ Revise ☐ Retire Notes:

A Korean American client born and raised in Ohio tells her counselor that coworkers often ask, "But where are you really from?" and praise her "excellent English." In Sue's taxonomy of racial microaggressions, these comments are BEST classified as which type?

A. Microassault, an explicit and deliberate racial attack

Microassaults are conscious, overt attacks such as slurs; these comments are not deliberate insults.

B. Microinsult, conveying that she must be unintelligent

Microinsults demean a person's heritage or ability; these comments mainly deny that she belongs.

C. Macroaggression, a policy that excludes her from work

The comments are interpersonal remarks, not an institutional policy or system.

D. Microinvalidation, treating her as foreign in her own land ✓ KEY

"Alien in own land" remarks negate her experience as an American and are microinvalidations.

Rationale: Sue described three types of microaggression: microassaults, microinsults and microinvalidations. Microinvalidations exclude, negate or nullify the thoughts, feelings or reality of people of color. Treating Asian Americans and Latinx Americans as perpetual foreigners, the "alien in own land" theme, is a common microinvalidation.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial microaggressions: microinvalidations and the "alien in own land" theme

☐ Approve ☐ Revise ☐ Retire Notes:

A counselor believes that healthy adults should make decisions independently of their families and judges clients who consult elders as "enmeshed." He assumes his own training's view of mental health applies equally to every client and sees no need to examine it. Which term BEST describes his stance?

A. Cultural humility

Cultural humility involves ongoing self-reflection and openness, the opposite of his stance.

B. Acculturation

Acculturation is the change that occurs when cultures come into contact, not a counselor's bias.

C. Cultural encapsulation ✓ KEY

Wrenn's cultural encapsulation means treating one's own cultural assumptions as universal truths.

D. Cultural relativism

Cultural relativism judges behavior within its own cultural context, which he fails to do.

Rationale: Wrenn introduced the term cultural encapsulation to describe counselors who substitute their own cultural assumptions for reality and ignore cultural variation among clients. Encapsulated counselors define normal behavior by their own culture and may pathologize clients whose values differ, such as collectivistic decision making. Developing awareness of one's own cultural values and biases is the first multicultural competency for this reason.

References: Sue & Sue (Counseling the Culturally Diverse) — Multicultural counseling competence: cultural encapsulation of the counselor (Wrenn)

☐ Approve ☐ Revise ☐ Retire Notes:

After a diversity workshop left him feeling guilty and confused, a White graduate student now says that people of color "bring most of their problems on themselves" and that White people are unfairly blamed. He avoids the classmates of color he had begun to befriend. In Helms's White racial identity model, which status does this reflect?

A. Contact

Contact involves naïve obliviousness to race, before the guilt and confusion he has experienced.

B. Reintegration ✓ KEY

Retreating from guilt into idealizing Whites and blaming people of color marks reintegration.

C. Pseudo-independence

Pseudo-independence involves an intellectual rejection of racism, not blaming people of color.

D. Autonomy

Autonomy reflects a secure, nonracist White identity and openness to difference.

Rationale: Helms's statuses are contact, disintegration, reintegration, pseudo-independence, immersion/emersion and autonomy. In reintegration, a person resolves the discomfort of disintegration by retreating into beliefs that favor Whites and blame people of color for their problems. Counselors can help by gently challenging these beliefs while supporting the person through the guilt that triggered them.

References: Sue & Sue (Counseling the Culturally Diverse) — White racial identity development: Helms's reintegration status

☐ Approve ☐ Revise ☐ Retire Notes:

A 16-year-old with a Filipino mother and a Black father decided last year to identify only as Black among friends. She now tells her counselor she feels guilty and disloyal when she avoids her mother's relatives and hides her Filipino heritage at school. According to Poston's biracial identity model, which stage is she in?

A. Personal identity

Personal identity is the early childhood stage, when identity rests on personal traits rather than group.

B. Appreciation

In appreciation, she would begin to value and explore both heritages instead of hiding one.

C. Integration

Integration involves recognizing and valuing all of her racial and ethnic identities.

D. Enmeshment/denial ✓ KEY

Guilt, confusion and disloyalty after choosing one heritage over the other define enmeshment/denial.

Rationale: Poston's model of biracial identity runs from personal identity through choice of group categorization, enmeshment/denial and appreciation to integration. In enmeshment/denial, a person who has chosen one group feels guilt, confusion or self-hatred about not being able to express the other part of their heritage. Counselors can help resolve this guilt so the person moves toward appreciating and integrating both identities.

References: Poston (Biracial Identity) — Stage 3, enmeshment/denial · Sue & Sue (Counseling the Culturally Diverse) — Racial/cultural identity development: biracial and multiracial identity (Poston's model)

☐ Approve ☐ Revise ☐ Retire Notes:

Core Counseling Attributes

16 items · NCE weight 8% (13 of 160 scored items)

Q185 of 200 · nce-s-cor-081 · Counseling and Helping Relationships · countertransference · hard · In review

A supervisor notes that a counselor stays steady with highly distressed clients because she has a sound, intact character structure and keeps clear boundaries between her own needs and her clients' needs. In Gelso and Hayes's account of countertransference management, this capacity is called

A. self-integration ✓ **KEY**

Self-integration refers to an intact, healthy character structure and the ability to differentiate self from client.

B. self-insight

Self-insight is awareness of one's own feelings and their origins, not the stability of one's character structure.

C. anxiety management

Anxiety management is the ability to tolerate and regulate one's anxiety so it does not drive behavior in session.

D. conceptualizing skill

Conceptualizing skill is the use of theory to understand the client and the relationship, including one's reactions.

Rationale: Gelso and Hayes describe five interrelated counselor qualities that help manage countertransference: self-insight, self-integration, anxiety management, empathy and conceptualizing skills. Self-integration is the most basic of these: a healthy, well-differentiated sense of self that lets the counselor recognize where they end and the client begins. Without it, the counselor's unresolved needs are more likely to spill into the work.

References: Gelso & Hayes (Countertransference) — Managing countertransference: the five factors (self-insight, self-integration, anxiety management, empathy, conceptualizing skills)

☐ Approve ☐ Revise ☐ Retire Notes:

A 39-year-old client says early on that everyone eventually gives up on him. He arrives late, dismisses suggestions, and mocks the counselor's questions. The counselor notices she has become curt and is privately hoping he will drop out, much as his family and past employers did. Which view of countertransference BEST fits understanding her reaction as a response pulled by the client's interpersonal style?

A. The classical view

The classical view limits countertransference to the counselor's unconscious, unresolved conflicts reacting to the client's transference.

B. The totalistic view

The totalistic view counts every counselor reaction as countertransference, without the specific focus on interpersonal pull.

C. The complementary view ✓ KEY

The complementary view sees the counselor's reaction as the response the client's style tends to evoke from others, here rejection.

D. The intersubjective view

This is not one of the four conceptions Gelso and Hayes outline, and it does not specifically name the interpersonal pull.

Rationale: Gelso and Hayes describe classical, totalistic, complementary and integrative conceptions of countertransference. The complementary view holds that clients' characteristic interpersonal styles pull predictable responses from others, including the counselor. Recognizing the pull lets the counselor avoid repeating the rejection the client expects and instead use it to understand his relational pattern.

References: Gelso & Hayes (Countertransference) — Ch. 1: classical, totalistic, complementary and integrative conceptions; complementary view and interpersonal pull

☐ Approve ☐ Revise ☐ Retire Notes:

A leader of an adult grief group feels mounting irritation each time one member tells long stories that fill the session. She reminds herself that monopolizing often shields a member from painful feelings and from closeness with others, and her irritation turns to curiosity. Which countertransference management factor described by Gelso and Hayes is she MOST clearly using?

A. Self-integration, keeping a stable sense of self apart from members

Self-integration concerns an intact character structure; the leader's shift came from applying theory.

B. Anxiety management, lowering her own tension so it does not leak out

Regulating anxiety helps, but what changed her reaction here was a theoretical understanding of the member.

C. Empathy, feeling the member's grief as though it were her own grief

Empathy keeps the "as if" quality; merging with the member's grief is not what she did or what empathy means.

D. Conceptualizing skills, using theory to make sense of the member's role ✓ KEY

She drew on a theoretical understanding of monopolizing to interpret the behavior, which defused her reaction.

Rationale: Conceptualizing skill is the counselor's ability to draw on theory to understand the client's dynamics and the relationship, including the counselor's own reactions. Framing a behavior that irritates the leader as a meaningful defense helps her respond therapeutically instead of acting out irritation. In groups, this lets the leader address the monopolizing in a way that serves both the member and the group.

References: Gelso & Hayes (Countertransference) — Managing countertransference: conceptualizing skills as a management factor · Yalom & Leszcz (Group Psychotherapy) — Problem group members: the monopolist

☐ Approve ☐ Revise ☐ Retire Notes:

A 28-year-old client with contamination fears says she likes her counselor and agrees that being able to ride the subway again is the right goal. When the counselor proposes exposure exercises, she says, "Touching handrails on purpose makes no sense to me." In Bordin's model, which part of the working alliance is MOST strained?

A. The goals, since she does not share the aim of returning to the subway

She explicitly agrees with the subway goal, so agreement on goals is intact.

B. The tasks, since she does not accept the methods proposed to get there ✓ **KEY**

Her objection is to the exposure exercises themselves, which is a disagreement about tasks.

C. The bond, since her objection shows she does not trust the counselor

She says she likes the counselor; the objection concerns the method, not the emotional bond.

D. Readiness for change, which Bordin listed as a fourth alliance component

Bordin's model has three components (goals, tasks, bond); readiness for change is not one of them.

Rationale: Bordin defined the working alliance as agreement on goals, agreement on tasks and an emotional bond. A client can share the goal and like the counselor yet reject the methods, which strains the tasks component. Exploring the rationale for exposure collaboratively, and adjusting the steps with the client, helps restore agreement.

References: Gladding (Counseling: A Comprehensive Profession) — Working alliance: goals, tasks and bond (Bordin)

☐ Approve ☐ Revise ☐ Retire Notes:

In a first family session, a father says the only problem is his 14-year-old's defiance. The mother says the real issue is the father's drinking, and the teen says nothing needs fixing. Each member is warm toward the counselor but dismisses the others' concerns. Using Friedlander and colleagues' SOFTA framework, which alliance dimension is MOST in need of attention?

A. Emotional connection to the counselor

Each member is warm toward the counselor, so this dimension is not the main concern.

B. Engagement in the therapy process

Engagement concerns involvement in the work; the defining problem here is disagreement among members.

C. Safety within the therapy system

Nothing indicates members feel unsafe in session; the issue is a lack of shared aims.

D. Shared purpose within the family ✓ **KEY**

Members disagree about what the problem is and whether change is needed, which defines a weak within-family alliance.

Rationale: The System for Observing Family Therapy Alliances (SOFTA) has four dimensions: engagement in the therapeutic process, emotional connection to the therapist, safety within the therapeutic system and shared sense of purpose within the family. Shared purpose is unique to conjoint work because it concerns the alliance among family members, not just with the counselor. Building a common definition of the problem is often a priority before change work can begin.

References: Friedlander et al. (Alliances in Couple & Family Therapy) — SOFTA dimensions: engagement, emotional connection, safety, shared sense of purpose within the family

☐ Approve ☐ Revise ☐ Retire Notes:

As a client describes a violent assault, a new counselor feels her own heart racing and a strong urge to change the subject so that she can feel calmer. In Davis's multidimensional model of empathy, this self-focused discomfort is BEST described as

A. personal distress ✓ KEY

Personal distress is self-oriented anxiety and unease in response to another's suffering, which can push the helper to escape.

B. empathic concern

Empathic concern is other-oriented warmth and compassion for the person suffering, not anxiety about oneself.

C. perspective taking

Perspective taking is the cognitive ability to adopt another's viewpoint, not an emotional reaction.

D. fantasy

The fantasy dimension is the tendency to identify with fictional characters in books or films.

Rationale: Davis measured empathy with four dimensions: perspective taking, fantasy, empathic concern and personal distress. Empathic concern points the helper toward the other person, while personal distress is focused on the helper's own discomfort and can lead to avoidance. Counselors who notice personal distress can ground themselves and return attention to the client.

References: Davis (Multidimensional Empathy) — Four dimensions: perspective taking, fantasy, empathic concern and personal distress

☐ Approve ☐ Revise ☐ Retire Notes:

A 20-year-old college student who uses a wheelchair says, "The elevator in my science building broke again, so I missed another lab. My professor just said to get the notes from someone." Which counselor response BEST conveys accurate empathy?

A. Elevators break down sometimes. Have you thought about asking a classmate for notes?

This minimizes the barrier and jumps to advice, so it misses the client's feelings.

B. You should file a complaint with disability services right away about that building.

Advocacy may help later, but directing action first skips understanding the client's experience.

C. You're frustrated, missing labs over something you can't control, and then brushed off. ✓ KEY

This accurately reflects both the frustration and the sense of being dismissed by the professor.

D. I know exactly how you feel. I hated it whenever I had to miss a class in college too.

Claiming identical experience shifts focus to the counselor and overlooks the access barrier.

Rationale: Accurate empathy captures both the client's feeling and the situation that gives rise to it, in the counselor's own words. Here the client's frustration stems from repeated access barriers and a dismissive response, and an empathic reflection names both. Advice, minimizing and "I know exactly how you feel" statements pull away from the client's frame of reference.

References: Ivey et al. (Intentional Interviewing) — Reflection of feeling and empathic understanding

☐ Approve ☐ Revise ☐ Retire Notes:

When a client begins describing his sexual history, the counselor says, "I'm completely comfortable with this topic." As she speaks, her voice tightens, she leans back, and she starts taking notes rapidly. The client grows guarded. Which core condition is MOST clearly lacking?

A. Unconditional positive regard

Nothing shows she values him less; the problem is the mismatch between her words and her state.

B. Congruence ✓ KEY

Her words deny discomfort that her voice and posture reveal, a clear mismatch between inner experience and expression.

C. Concreteness

Concreteness is helping a client be specific; it does not explain the client's guardedness here.

D. Accurate empathy

She has not yet misread his feelings; the breakdown lies in her own genuineness.

Rationale: Congruence means the counselor's outward communication matches inner experience. Clients often read nonverbal cues more readily than words, so a counselor who claims comfort while showing tension can seem untrustworthy. A congruent counselor notices the discomfort, manages it, and, if relevant, acknowledges it honestly rather than covering it with a facade.

References: Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

In week five of a parenting skills group, three members again arrive 15 minutes late, and the leader has felt frustrated for weeks. One member says, "You seem annoyed with us today. Are you?" Which leader response BEST demonstrates congruence?

A. Not at all. I'm here to support all of you, so let's get back to the topic for today.

Denying a feeling the members can already sense is incongruent and models avoidance.

B. Yes. Some of you clearly don't respect this group, and that needs to change right now.

This voices the feeling as blame and judgment, not as an owned reaction the group can explore.

C. Yes, I've felt frustrated about the late arrivals. I'd like us to talk about it together. ✓ KEY

The leader owns the feeling honestly and turns it into a here-and-now discussion that serves the group.

D. What makes you ask? I'm curious what you're noticing, and how others see me today.

Deflecting the direct question keeps the leader's real reaction hidden behind a technique.

Rationale: A congruent leader's words match their inner experience, and they share relevant feelings in an owned, nonblaming way when it helps the work. Acknowledging frustration and inviting discussion models honest communication and lets members examine a pattern that affects the whole group. Denial or deflection teaches members that real reactions are not welcome.

References: Yalom & Leszcz (Group Psychotherapy) — Therapist transparency and here-and-now focus · Corey (Theory & Practice) — Person-centered therapy: congruence or genuineness

☐ Approve ☐ Revise ☐ Retire Notes:

A 31-year-old mother whose children were placed in foster care for neglect a year ago has completed her case plan and has regained custody. She says, "Everyone still looks at me like I'm a bad mother. I bet you do too." Which counselor response BEST conveys unconditional positive regard?

A. You did neglect them before, but what really matters most now is that you've changed.

This makes her worth depend on having changed, which is conditional regard.

B. I don't see you that way. I value you as a person, including the hard parts of your story. ✓ KEY

This affirms her worth without conditions and without denying or condemning her history.

C. Other people's opinions shouldn't matter, so try not to worry about how they look at you.

This dismisses her pain and sidesteps her question about how the counselor sees her.

D. You've done everything the court asked of you, so you have earned some respect from me.

Respect framed as earned by compliance is conditional, the opposite of unconditional regard.

Rationale: Unconditional positive regard is the counselor's acceptance and valuing of the client as a person, not contingent on behavior or progress. It does not require approving of past actions, but it separates the client's worth from them. For clients who carry shame and expect judgment, this acceptance helps them examine their experience openly.

References: Corey (Theory & Practice) — Person-centered therapy: unconditional positive regard and acceptance

☐ Approve ☐ Revise ☐ Retire Notes:

During a telehealth session, a client's 4-year-old wakes from a nap crying, and the client says, "I'm so sorry, I have to go check on her." Which counselor response BEST conveys warmth?

A. Please try to arrange childcare for our sessions so we're not interrupted again.

This focuses on the inconvenience and adds pressure at a stressful moment.

B. That's fine, but we'll need to end on time even if we lose some minutes now.

Leading with the time limit sounds cool and rule-focused rather than caring.

C. We can just reschedule the rest. Message me later whenever things calm down.

Ending abruptly may feel like a dismissal and gives up the session without asking.

D. Of course, no need to apologize. Go take care of her; I'll be right here. ✓ KEY

This relieves her guilt, accepts her parenting role, and signals the counselor's steady availability.

Rationale: Warmth is conveyed through acceptance, kindness and a relaxed, caring manner, especially when a client feels embarrassed. Telehealth brings clients' home lives into the session, and responding to interruptions with understanding protects the relationship. Logistics can be discussed later if interruptions become a pattern.

References: Ivey et al. (Intentional Interviewing) — Empathic relationship: warmth and positive regard

☐ Approve ☐ Revise ☐ Retire Notes:

A 38-year-old client says, "My partner and I decided we're never having kids. My parents keep saying I'll regret it and that it's selfish." Which counselor response BEST reflects a nonjudgmental stance?

A. It sounds like you've made your decision, and their comments are weighing on you. ✓ KEY

This accepts the decision as the client's and focuses on the client's experience of the pressure.

B. Your parents might have a point. A lot of people do change their minds about this.

This sides with the parents and implies the client's choice is likely mistaken.

C. Have you and your partner fully thought through what you might miss out on later?

This questions the soundness of the decision rather than exploring what the client brought.

D. Good for you both. Honestly, having children isn't the right choice for most people.

Approval is still judgment, and it imposes the counselor's own view about having children.

Rationale: A nonjudgmental stance means the counselor neither condemns nor endorses the client's life choices, staying with the client's own meaning and feelings. Both disapproval and enthusiastic approval insert the counselor's values. Here the client's concern is family pressure, so the counselor reflects that experience.

References: ACA Code of Ethics — A.4.b Personal Values

☐ Approve ☐ Revise ☐ Retire Notes: _____

A hearing counselor working with a 34-year-old Deaf client through an ASL interpreter suggests they explore "grief about your hearing loss." The client signs that she was born Deaf, sees Deafness as her culture, and feels no loss about it. Which next step BEST reflects cultural humility?

A. Explain that many people do grieve hearing loss, so it may still be worth exploring later on

This holds onto the counselor's frame and overrides the client's own view of her identity.

B. Ask the client to spend the next session teaching her everything she should know about Deaf culture

This shifts the counselor's learning burden onto the client and takes over the client's session.

C. Own the assumption, follow the client's view of Deafness, and learn about Deaf culture herself ✓ KEY

This acknowledges the error, centers the client's identity, and accepts responsibility for ongoing learning.

D. Refer the client to a Deaf counselor, since a hearing counselor cannot understand her experience

Referral may be an option the client chooses, but reflexive referral avoids the repair and the relationship.

Rationale: Cultural humility involves an accurate view of one's own limits, openness to the client's perspective, and ongoing self-directed learning. Many Deaf people view Deafness as a linguistic and cultural identity rather than a medical loss, and a hearing counselor's deficit framing can feel invalidating. Owning the assumption repairs the relationship, while learning on one's own time avoids making the client the teacher.

References: Sue & Sue (Counseling the Culturally Diverse) — Counseling individuals with disabilities; Deaf culture and the medical vs cultural model

☐ Approve ☐ Revise ☐ Retire Notes: _____

A 27-year-old Mexican American client describes being followed by store security while shopping. Hoping to reassure her, the counselor says, "I don't really see color. To me, everyone is just a person." The client changes the subject. In Sue's taxonomy of microaggressions, the counselor's comment is BEST classified as a

A. microassault

Microassaults are explicit, usually deliberate attacks; the counselor meant to be reassuring.

B. microinsult

Microinsults demean a person's heritage or ability, such as implying a client is less capable.

C. microaffirmation

Microaffirmations are small acts of inclusion, not one of Sue's three forms of microaggression.

D. microinvalidation ✓ KEY

Color-blind statements negate the client's racialized experience and reality, the hallmark of a microinvalidation.

Rationale: Sue described three forms of microaggression: microassaults, microinsults and microinvalidations. Color-blind remarks are a classic microinvalidation because they deny the relevance of race just as the client reports a racialized experience. Counselors' good intentions do not cancel the impact, which is why self-awareness of such reflexive reassurances matters.

References: Sue & Sue (Counseling the Culturally Diverse) — Racial microaggressions: microinvalidations and color blindness

☐ Approve ☐ Revise ☐ Retire Notes:

A 74-year-old widow tells her counselor she has a new partner and wants to talk about sexual intimacy, which feels different than it did decades ago. The counselor, in her thirties, finds herself steering back to the client's health appointments. What is the MOST appropriate response to this pattern?

A. Notice her possible age-related assumptions and return to the concern the client raised ✓ KEY

Recognizing the bias and refocusing on the client's stated goal is the core of self-awareness.

B. Refer the client to a medical provider, since sexual concerns in later life are medical

This treats the concern as purely medical and avoids the topic the client chose to bring to counseling.

C. Keep the focus on health care, because that is more likely to be the client's real priority

This substitutes the counselor's assumption about older adults for the client's own stated priority.

D. Tell the client she feels awkward about the subject and ask her to choose a different topic

Placing the counselor's discomfort on the client restricts the work instead of managing the reaction.

Rationale: Ageist assumptions, such as viewing older adults as asexual, can lead counselors to avoid topics clients want to address. Self-awareness means noticing when one's own discomfort or stereotypes are steering the session, exploring that in supervision, and following the client's agenda. Sexuality remains an important part of well-being across the lifespan.

References: ACA Code of Ethics — A.4.b Personal Values · Berk (Development Through the Lifespan) — Late adulthood: sexuality and intimate relationships

☐ Approve ☐ Revise ☐ Retire Notes:

A heterosexual counselor at a rural clinic realizes that the agency's intake form offers only "husband" or "wife" for partner status. A gay client mentions he left the field blank and wondered whether he would be welcome. Which action BEST reflects awareness of privilege in the counseling relationship?

- A. Reassure the client that the form is just paperwork and has no effect on how he is treated
This dismisses the message the form sends and the client's reasonable concern about safety.
- B. Acknowledge what the form signals, ask how he describes his relationship, and push to revise it ✓ KEY
This names the impact, centers the client's terms and addresses the institutional barrier.
- C. Fill in "wife" so that the record is complete, and then move on to the presenting concern
This erases the client's identity and repeats the heteronormative assumption in the record.
- D. Avoid asking about his relationship in the future so that he is not made to feel uncomfortable
Avoidance withholds attention from a relevant part of his life and implies the topic is unwelcome.

Rationale: Heterosexual privilege includes not having to notice that forms and routines assume one's own identity. Counselors who hold privilege can acknowledge how such systems affect clients, invite clients to describe themselves in their own terms, and advocate to change exclusionary practices. The MSJCC frame this as awareness of how privileged and marginalized statuses shape the relationship and the setting.

References: MSJCC (Ratts et al., 2016) — Counseling relationship and advocacy: privileged and marginalized statuses and institutional barriers · Sue & Sue (Counseling the Culturally Diverse) — Counseling LGBTQ clients: heterosexism and heterosexual privilege

☐ Approve ☐ Revise ☐ Retire Notes: